

Sources Sought Notice

SUBJECT* Lincoln County, OR Community Based Outpatient Clinic
Estimated POP: 12/01/2026 - 11/30/2036

GENERAL INFORMATION

CONTRACTING OFFICE'S ZIP CODE*	98661
SOLICITATION NUMBER*	36C26026Q0422
RESPONSE DATE/TIME/ZONE	03-17-2026 12:00 PACIFIC TIME, LOS ANGELES, USA
ARCHIVE	30 DAYS AFTER THE RESPONSE DATE
RECOVERY ACT FUNDS	N
SET-ASIDE	N
PRODUCT SERVICE CODE*	Q201
NAICS CODE*	621498
CONTRACTING OFFICE ADDRESS	NCO 20 Network Contracting Office ATTN: V4CONT 1601 E. Fourth Plain Blvd Bldg.17, Suite B428 Vancouver WA 98661
POINT OF CONTACT*	Senior Contract Specialist Scott Reed scott.reed2@va.gov NO PHONE CALLS ACCEPTED

PLACE OF PERFORMANCE

ADDRESS	See attached document: Sources Sought Notice
POSTAL CODE	
COUNTRY	

ADDITIONAL INFORMATION

AGENCY'S URL	
URL DESCRIPTION	
AGENCY CONTACT'S EMAIL ADDRESS	
EMAIL DESCRIPTION	

Sources Sought Notice

DISCLAIMER

This RFI is issued solely for information and planning purposes only and does not constitute a solicitation. All information received in response to this RFI that is marked as proprietary will be handled accordingly. In accordance with FAR 15.201(e), responses to this notice are not offers and cannot be accepted by the Government to form a binding contract. Responders are solely responsible for all expenses associated with responding to this RFI.

DESCRIPTION

The U.S. Department of Veterans Affairs, Network Contracting Office 20, in support of the VISN 20 Northwest Network, is currently requesting information to determine the existence of vendors capable of providing Community Based Outpatient Clinic (CBOC) services to the VA Portland Health Care System (VAPORHCS). It is anticipated that a single Indefinite-Delivery Indefinite-Quantity (IDIQ) contract will result from the solicitation. The intended period of performance is 10 years.

Description of Services

VAPORHCS requires clinic space, equipment, supplies and staff as detailed in the Performance Work Statement (PWS) for Primary Care services for up to 1600 patients in the Lincoln County, OR area. The requirement is a physical location within Lincoln County, OR. Mental Health services will be provided physically on site at the Salem CBOC, or virtually from the coastal clinics if Veterans have no other means of communication.

Contractor performing services under a resultant contract shall provide a continuum of care from prevention to diagnosis and treatment, and to appropriate referral and follow-up. Contractor's outpatient site of care shall have the necessary professional medical staff, diagnostic testing and treatment capability, and referral arrangements needed to ensure continuity of health care. The contractor shall provide services solely dedicated to Veterans regardless of gender or age. Those patients needing specialty care shall be referred to VA.

Alternatively, the contractor may provide clinic space, equipment, supplies and staff, except physicians, which may be provided by VAPORHCS.

The NAICS code assigned is: 621498, All Other Outpatient Care Centers the small business size standard for this NAICS code is \$25.5 million.

The Government is not currently soliciting offers. This is a Sources Sought Notice to gain knowledge of potential qualified sources. A solicitation is not currently available.

Responses to this request will be considered for the purpose of determining how the requirements will be stated and how the procurement will be conducted. This Sources Sought Notice is not to be construed as a commitment by the Government, implied or otherwise, to issue a solicitation or to award a contract. No reimbursement will be made for any costs associated with providing information in response to this notice or any follow-up information requests. If your business is interested and capable of providing the required services, please complete the Sources Sought Questionnaire following the PWS and email your response to scott.reed2@va.gov no later than 12:00 PM PT on March 16, 2026. Please include a Capabilities Statement. Phone calls will not be accepted. Include any comments or questions you may have.

PERFORMANCE WORK STATEMENT (PWS)

1. GENERAL:

- 1.1 Services Required:** The VA Portland Health Care System; requires the following services to be provided in a private hospital, office or clinic environment to Veterans, primarily residing in Lincoln County, Oregon area.
- 1.2 Site Classification:** This site is classified as a [X] Primary Care CBOC. VA Portland Health Care System (VAPORHCS) requires clinic space, equipment, supplies, and staff as detailed in this solicitation Primary Care services to up to 1600 patients in the Lincoln County area. The requirement to be met is a physical location within Lincoln County, Oregon. Mental Health services will be provided physically onsite at the Salem CBOC, or virtually from the coastal clinics if Veterans have no other means of communication.
- 1.3 Place of Performance:** Services shall be performed by contractor at a facility in Lincoln County on behalf of the VA Portland Health Care System 3710 SW US Veterans Hospital Rd. Portland, OR 97239
- 1.4 Authority:** Title 38 United States Code (USC) 8153 and FAR 12 in combination with 15 and/or VAAR 873.
- 1.5 Policy and Regulations:** The contractor is required to meet VHA performance and quality criteria, policy and standards including, but not limited to, access, quality of care, Veteran satisfaction, coordination of care, and clinical guidelines. Performance and quality standards may change during the contract. New or revised quality/performance criteria or standards will be provided to the contractor before implementation date and throughout the life of the contract through unilateral or bi-lateral modifications, as applicable. Copies of current VA and VHA publications can be located at <http://www.va.gov/vapubs/> or at <http://www.va.gov/vhapublications/>. Some VA policy documents include Standard Operating Procedures (SOPs) located on internal VA websites that are not available to the public. SOPs relevant to this requirement are provided as attachments under Section D. Compliance with mandated performance is required as a condition of this contract. Contractor shall comply with all relevant VA policies and procedures, including those related to quality, access, patient safety and performance, including, but not limited to the policies listed on the Contracted Clinic Policy Document located at <https://www.patientcare.va.gov/primarycare/docs/List-of-contracted-clinic-policy-documents.xlsx#>. A link to the VA Qualifications Standards is also included in this document.
- 1.6 Acronyms/Definitions:**
- ABMS:** American Board of Medical Specialties
- ACGME:** Accreditation Council for Graduate Medical Education
- ACIP:** Advisory Committee on Immunization Practices
- ACLS:** Advanced Cardiac Life Support
- ACO:** Administrative Contracting Officer
- ACPE:** American Council on Pharmaceutical Education
- ADE:** Adverse Drug Events
- AED:** Automatic External Defibrillator
- AIS:** Automated Information Security
- ANA:** American Nurses Association
- AOA:** American Osteopathic Association
- APRN:** Advanced Practice Registered Nurse
- ARRT:** American Registry of Radiologic Technology
- ASC:** Ambulatory Surgery Center
- Assigned:** A Veteran is “assigned” to an outpatient clinic via PCMM (i.e., CBOC) where the patient receives their primary care after the patient’s eligibility is determined through registration and enrollment.
- Asynchronous Care:** Care provided outside of bookable time and includes but is not limited to the following activities: refilling prescriptions, reviewing outside records, huddles and case management, team meetings, processing view alerts, secure messaging and training.
- BAA:** Business Associate Agreement
- BI-RADS:** Breast Imaging-Reporting and Data System; a quality assurance tool designed to standardize mammography reporting.
- BLS:** Basic Life Support

BOS: Bureau of Osteopathic Specialists
CAHEA: Committee on Allied Health Education and Accreditation
CAP: College of American Pathologists
CARF: Commission on Accreditation of Rehabilitation Facilities
CBO: VA Central Billing Office.
CCNE: Commission on Collegiate Nursing Education
CDC: Centers for Disease Control and Prevention
CEU: Certified Education Unit
CLIA: Clinical Laboratory Improvement Amendments
CME: Continuing Medical Education
CMS: Center for Medicare and Medicaid Services
CNP: Certified Nurse Practitioner
CNS: Clinical Nurse Specialist
CO: Contracting Officer
COPD: Chronic Obstructive Pulmonary Disease
COR: Contracting Officer's Representative
COS: Chief of Staff
CPA: Collaborative Practice Agreement
CPARS: Contractor Performance Assessment Reporting System
CPP: Clinical Pharmacist Practitioner
CPT: Current Procedural Terminology
CSWE: The Council on Social Work Education the CSWE website is <http://www.cswe.org>.
CVT: Clinical Video Telehealth
DICOM: Digital Image and Communication in Medicine
DIGMA: Drop-in Group Medical Appointment
DRG: Diagnostic Related Group
DSS: Decision Support System
ECC: Extended Care Center
Enrollment: The process of establishing eligibility for VA's "Medical Benefits Package." Most Veterans are required to "enroll" into the VA Health Care System to be eligible for VA health care and to be assigned to an outpatient clinic like a CBOC, however some can still receive care without enrolling. Applicants are only required to "enroll" once for VA health care unless they are determined ineligible for care at time of application or they have disenrolled.
EPRP: External Peer Review Program
FDA: Food and Drug Administration
FSMB: Federation of State Medical Boards
FTE: Full Time Equivalent
HCC: Health Care Center - A HCC is a VA-owned, VA-leased, contract, or shared clinic operated at least 5 days per week that provides primary care, mental health care, on site specialty services, and performs ambulatory surgery and/or invasive procedures which may require moderate sedation or general anesthesia.
HCFA: Health Care Financing Administration
HHS: Department of Health and Human Services
HICPAC: Healthcare Infection Control Practices Advisory Committee - a federal advisory committee made up of 14 external infection control experts who provide advice and guidance to the CDC and the Secretary of HHS regarding the practice of health care infection control, strategies for surveillance and prevention and control of health care associated infections in United States health care facilities.
HPE: Health Professions Education
HPT: Health Professions Trainee
HT: Home Telehealth
ICAVL: Intersocietal Commission for the Accreditation of Vascular Laboratories
ICD: International Classification of Diseases
iMED: software used by VA for informed consent.
INR: International Normalized Ratio
LIP: Licensed Independent Practitioner
MCCR: Medical Care Cost Recovery
Mental Health Services: per VHA Directive 1160.01 is meant to include services for the evaluation, diagnosis, treatment, rehabilitation, suicide prevention and crisis intervention, and coordination of care for both substance use disorders and other mental disorders.

General mental health services include:

- (a) Diagnostic and treatment planning evaluations for the full range of mental health problems, including SUD
- (b) Treatment services using evidence-based pharmacotherapy, somatic treatments, or primary evidence-based Psychotherapy (EBP) for patients with mental health conditions and substance use disorders
- (c) Patient education
- (d) Family education, family consultation, family psychoeducation and couples/marriage and family counseling when it is associated with benefits to the Veterans
- (e) Referrals as needed to inpatient and residential care programs; and
- (f) Consultation about special emphasis problems including Post Traumatic Stress Disorder (PTSD) and Military Sexual Trauma (MST)
- (g) Suicide Prevention
- (h) Referrals and coordination of care between other levels of care and community treatment programs.

Specialty mental health services include:

- (a) Consultation and treatment services for the full range of mental health conditions
- (b) Evidence-based psychotherapy
- (c) Intensive Community Mental Health Integration (ICMHR)
- (d) Psychosocial Rehabilitation Services, including: PRRCs, family psycho-education, family education, skills training, peer support, and Compensated Work Therapy (CWT), supported employment and vocational assistance.
- (e) PTSD teams or specialists
- (f) MST special clinics
- (g) Homeless programs; and
- (h) Specialty substance abuse treatment services.

MQSA: Mammography Quality Standards Act

MSN: Master of Science in Nursing

NLN: National League for Nursing

NLNAC: National League for Nursing Accreditation Commission

NP: Nurse Practitioner

NSQIP/CICSP: National Surgical Quality Improvement Program/Continuous Improvement in Cardiac Surgical Program

OTC: Over the Counter

PA: Physician Assistant

PACS: Picture Archiving and Communications System

PACT: Patient Aligned Care Team Background & Introduction: VA has implemented a PCMH model at all VA Primary Care sites which is referred to as PACT. This initiative supports VHA's Universal Health Care Services Plan to redesign VHA healthcare delivery through increasing access, coordination, communication, and continuity of care. PACT provides accessible, coordinated, comprehensive, patient-centered care, in team-based environment including the active involvement of other clinical and non-clinical staff. PACT allows patients to have a more active role in their health care and is associated with increased quality improvement, patient satisfaction, and a decrease in hospital costs due to fewer hospital visits and readmissions.

PACT Support staff: PACT staff assisting providers in the actual delivery of care to patients. It consists of RNCMs, clinical associates, and administrative associates.

SYNCHRONOUS CARE: Bookable, real-time care provided in person or virtually (VA Video Connect, CVT, telephone) with a Veteran and involves clinical decision-making in a two-way conversation.

Parent Facility: VAMC responsible for performance monitoring and payment for contracted outpatient site of care services.

PBM: Pharmacy Benefits Management

PCMH: Patient-Centered Medical Home

PCMHI: Primary Care-Mental Health Integration

PCMM: Patient Centered Management Module - is an enterprise application that enables users to (1) set up and define health care teams, (2) assign staff and their associated full-time equivalent (FTE) staff to positions within each team, (3) assign patients to the team, and (4) assign patients to specific team members.

PCP: Primary Care Provider

Phar.D.: Doctor of Pharmacy

PMPM: Per Member Per Month-reimbursement methodology for services provided under the PC and MH LINs based on all-inclusive monthly rate per Veteran that meets qualifying encounter criteria in the previous twelve months of the contract period. This methodology also accounts for costs of unassigned/unenrolled visits based on estimates provided to contractors and it supports the PACT model in increasing coordination, quality, and continuity of care of Veterans receiving services at the Outpatient Site of Care.

POC: Point of Care Testing

Primary Care Visit: an episode of care furnished in a clinic or virtually that provides integrated, accessible health care services by clinicians who are accountable for addressing a large majority of personal health care needs, developing a sustained partnership with patients, and practicing in the context of family and community. Primary care includes, but is not limited to, diagnosis and management of acute and chronic biopsychosocial conditions, health promotion, disease prevention, overall care management, and patient and caregiver education. The VHA site classification defines primary care as those encounters that occur within the primary care class of encounters.

PWS: Performance Work Statement

QAPI: Quality Assessment and Performance Improvement

QASP: Quality Assurance Surveillance Plan

REACH VET: Recovery Engagement and Coordination for Health Veterans Enhanced Treatment-REACH VET summarizes information from the Veterans Health Record and uses predictive modeling to identify Veterans at statistical risk for suicide and other negative outcomes. This tool supplements clinical strategies used to identify at-risk Veterans and may help providers intervene prior to these negative outcomes occurring.

RME: Reusable Medical Equipment

SFT: Store and Forward Telehealth

SMA: Shared Medical Appointments

SOP (Clinical): Scope of Practice

SPD: Sterile Processing Division

SPE: Senior Procurement Executive

Specialty Care Visit: The VHA site classification defines specialty care as those encounters that occur within the geriatric medicine; allergy; cardiology; dermatology; emergency; employee health; endocrinology; gastroenterology; general medicine; hematology or oncology; infectious disease; nephrology; neurology; outreach; pulmonary or respiratory disease; rheumatology; amputation follow-up; amputation; anesthesia; cardio-thoracic; ear, nose, and throat (ENT); eye; general surgery; gynecology (GYN); neurosurgery; orthopedics; plastic surgery; urology; or vascular clinic stops.

STORM: Stratification Tool for Opioid Risk Monitoring

Synchronous Care: Bookable, real-time care provided in person or virtually (VA Video Connect, CVT, telephone) with a Veteran and involves clinical decision-making in a two way conversation.

TCT: Telehealth Clinical Technician

TIU: Text Integration Utility

TJC: The Joint Commission

TMS: Talent Management System

VA: Veterans Affairs

VA ECHO: VA Extension for Community Health Outcomes

VA EHR: VA electronic health record

VAMC: Veterans Affairs Medical Center

VAST: VHA Site Tracking database

VetPro: a federal web-based credentialing program for healthcare providers.

VHA: Veterans Health Administration

VSSC: VHA Support Service Center

VVC: VA Video Connect

WHEN: Weekend- Holiday- Every- Night

2. STAFFING AND QUALIFICATIONS:

2.1 Minimum Patient Aligned Care Team (PACT) Staffing Requirements:

PACTs comprise the patients, the patients' personal support persons, teamlets (Primary Care Providers (PCPs), Registered Nurse Care Managers (RNCMs), Clinical Associates, Administrative Associates), discipline specific team members. The VA will be providing (Clinical Pharmacist Practitioners (CPPs), Licensed Social Workers, Registered Dietitian/Nutritionists, and Primary Care-Mental Health Integration (PCMHI) staff). The contractor

shall provide PACT staffing in numbers and qualifications capable of fulfilling the standards outlined in the resultant contract. The contractor shall provide a sufficient number of primary care providers so that each primary care provider has a caseload ratio to meet VA standards. Current standards are 1200 active patients per full time physician and 900 active patients per full time midlevel provider. Actual panel sizes can be determined by the VA in accordance with VHA Directive 1406 Patient Centered Management Module (PCMM) for Primary Care, panel numbers may increase or decrease depending on the need of Primary Care Services within Lincoln County. The staffing standard for support staff shall be at least three full time equivalent (FTE) support staff (1 FTE RNCM, 1 FTE Clinical Associate, 1 FTE Administrative Associate) for each FTE Primary Care Provider. The contractor shall provide personnel in numbers and qualifications capable of fulfilling the standards outlined in the resultant contract. The contractor shall establish and implement contingency plans for ensuring patients receive continuity of and access to appropriate primary care during periods of inadequate resources, extended staff absences, staff turnover, understaffing, and nature-related events (e.g., extreme weather conditions, natural disasters). If the number of patients reaches 90% of maximum panel size assigned by the facility the contractor shall communicate to the VA, the contractor's future staffing plan to ensure staffing ratio standards remain in accordance with PCMM staffing standards. The total estimated patients assigned to site is: 1,345 for Lincoln County.

Physician Director (Mandatory for all sites): FTE Ratio Performance Standard: 0.05 FTE per PACT.

Responsible Party: Shall be provided by contractor. **Qualifications:** Contractor's Physicians (including subcontractors) providing physician director services under the resultant contract shall demonstrate evidence of education, training, and experience in Internal Medicine or Family Practice. Contractor's Physicians performing under this contract shall be board certified by the ABMS in Internal Medicine and/or Family Practice or the BOS in Internal Medicine and/or Family Practice. If proposed staff do not meet VA credentialing requirements, the contractor shall propose substitute acceptable personnel within five (5) calendar days. The physician director may also be credentialed and privileged to function as a PCP. (If so, authorization for prescriptive authority is required). Physicians shall be licensed in the state in which the outpatient site of care (i.e., CBOC) is located; otherwise, qualifications as per VA qualification standards for Physician located in the VA Qualification Standards. See the Policy and Regulations section of this PWS for VA Qualification Standards access information. **Position Responsibilities:** Serves as medical director to oversee and be responsible for the provision of covered services to assigned and unassigned patients presenting for care at the site.

2.2 Primary Care Teamlet Staffing (Mandatory for all sites): All primary care teamlet staffing shall be provided by contractor.

Teamlet Member 1: Primary Care Provider (PCP): FTE Ratio Performance Standard: Current standards are 1200 active patients per full time physician and 900 active patients per full time mid-level provider. Contractor shall propose quantity of FTE to meet standard FTE ratio to panel size. Actual panel sizes can be determined by the VA in accordance with VHA Directive 1406 Patient Centered Management Module (PCMM) for Primary Care. Contractor to propose mix of PCP from the options below.

Option 1: Physician (MD/DO): Qualifications: Physicians shall demonstrate evidence of education, training, and experience in Internal Medicine or Family Practice. Physicians performing under this contract shall be board certified or board eligible by the ABMS in Internal Medicine and/or Family Practice or the BOS in Internal Medicine and/or Family Practice. Authorization for prescriptive authority is required. If proposed staff do not meet VA credentialing requirements, the contractor shall propose substitute acceptable personnel within five (5) calendar days. Physicians shall be licensed in the state in which the outpatient site of care (i.e., CBOC) is located; otherwise, qualifications as per VA qualification standards for Physician located in the VA Qualification Standards. See the Policy and Regulations section of this PWS for VA Qualification Standards access information. **Position Responsibilities:** Responsible for the provision of covered services to assigned and unassigned patients presenting for care, providing health care commensurate to the PCP's licensure and clinical privileges or scope of practice, ensuring the patient's care plan contains medical recommendations for clinically indicated care, offering clinically indicated health care services to patients assigned to the PACT, and providing or arranging for care to which patients consent, providing leadership to the team including shared delegation of appropriate care and care processes to appropriate team members, reviewing available clinical and performance data with the team, and focusing on continuous improvement of critical team processes, ensuring the patient has same-day access for face-to-face and virtual care visits during regular clinic hours, and collaborating with PACT staff to develop personal health plans that incorporate care management and care coordination appropriate to the patient's needs. Responsible for meeting all clinical performance metrics.

Option 2: Advanced Practice Registered Nurse (APRN): Qualifications: APRN's (including subcontractors) shall possess a master's degree from a program accredited by the National League for Nursing Accreditation Commission (NLNAC) or the Commission on Collegiate Nursing Education (CCNE) and maintain full and current certification as a Certified Nurse Practitioner (CNP) from the American Nurses Association or another nationally recognized certifying body in either Adult Health or Family Practice. Authorization for prescriptive authority is required. CNPs shall prescribe controlled substances in accordance with limitations imposed by the Controlled Substances Act and the CNP's State licensure on the authority to prescribe or administer controlled substances. Three years of clinical nursing experience is required. A minimum of one (1) year clinical experience as a CNP is required (three (3) years preferred). Experience in outpatient care in a Family Medicine or Internal Medicine environment is preferred. CNP shall have current, full, active, and unrestricted license and registration as a graduate professional nurse in the state in which the outpatient site of care (i.e., CBOC) is located; otherwise, qualifications as per VA qualification standards for Advanced Practice Nurse located in the VA Qualification Standards. See the Policy and Regulations section of this PWS for VA Qualification Standards access information. **Position Responsibilities:** Responsible for the provision of covered services to assigned and unassigned patients presenting for care, providing health care commensurate to the PCP's licensure and clinical privileges or scope of practice, ensuring the patient's care plan contains medical recommendations for clinically indicated care, offering clinically indicated health care services to patients assigned to the PACT, and providing or arranging for care to which patients consent, providing leadership to the team including shared delegation of appropriate care and care processes to appropriate team members, reviewing available clinical and performance data with the team, and focusing on continuous improvement of critical team processes, ensuring the patient has same-day access for face-to-face and virtual care visits during regular clinic hours, and collaborating with PACT staff to develop personal health plans that incorporate care management and care coordination appropriate to the patient's needs.

Teamlet Member 2: Registered Nurse (RN) Care Manager: FTE Ratio Performance Standard: Current standard is 1.0 FTE RNCM per 1.0 FTE PCP **Qualifications:** Graduate of a school of professional nursing accredited by one of the following accrediting bodies at the time the program was completed by the applicant: The accreditation Commission for Education in Nursing (ACEN) or the commission on Collegiate Nursing Education (CCNE). Current, full, active, and unrestricted registration as a graduate professional nurse in the state in which the outpatient site of care (i.e., CBOC) is located; otherwise, qualifications as per VA qualification standards for Nurse located in the VA Qualification Standards. See the Policy and Regulations section of this PWS for VA Qualification Standards access information. **Position Responsibilities:** Responsible for the provision of covered services to assigned and unassigned patients presenting for care at the site, providing all aspects of professional nursing services consistent with licensure, certification, nursing professional standards of practice, and the clinician's Functional Statement with elements of practice, enhancing patient safety and quality of care by collaborating with PACT staff to develop, oversee, and manage care management plans and care coordination for patients assigned to PACTs, participating in modes of communication and care delivery including, but not limited to, secure messaging, virtual care, view alerts management, shared medical appointments, face to face visits, etc., as part of care management, identifying patient needs for involvement of discipline-specific team members and discussing nursing recommendations with the PCP, engaging relevant PACT staff to support nursing care, according to locally established informal and formal communication processes, including entering consultation requests to discipline-specific PACT members, if required for formal communications, assuming full accountability for the appropriateness of assignments made by the RNCM to clinical associates or administrative associates related to care management, care coordination, nursing services, and outcomes of care, entering orders in the VA EHR for tests per approved standardized RN care management protocols or PCP orders, ensuring the RNCM has same-day access for face-to-face and virtual care visits, using nursing expertise, evidence-based guidelines, standardized nursing protocols, and professionally accepted practice standards to promote patient engagement, self-care and wellness, and provide care to patients and determine care management requirements for individual patients or cohorts of patients. The RN collaborates for the improvement of patient care outcomes in the Patient Aligned Care Team. Promotes systems to improve access and continuity of care, uses advanced clinical knowledge and critical thinking skills to mentor staff in planning, implementing and evaluating interventions that improve patient outcomes, designs and provides age and population specific health promotion and risk reduction strategies, translates evidence-based research into practice to ensure that patients benefit from the latest innovations in nursing science, manages patients in transition between levels of care, serves as an expert resource to implement and teach skills, including motivational interviewing to promote patient self-management toward patient-driven holistic care plan for life.

Teamlet Member 3: Clinical Associate FTE Ratio Performance Standard: Current standard is 1.0 FTE clinical associate per 1.0 FTE PCP. Contractor to propose the mix of Clinical Associates from the options below.

Option 1: Licensed Practical Nurse (LPN) or Licensed Vocational Nurse (LVN): Qualifications: Current, full, active, and unrestricted license in the state in which the outpatient site of care (i.e., CBOC) is located; otherwise, qualifications as per VA qualification standards for Licensed Practical or Vocation nurse located in the VA Qualification Standards. See the Policy and Regulations section of this PWS for VA Qualification Standards access information. **Position Responsibilities:** Responsible for the provision of covered services to assigned and unassigned patients presenting for care at the site, providing evaluation and care consistent with licensure, certification, and functional statement with elements of practice, to patients assigned PACTs, collaborating with PACT staff to develop comprehensive health care plans and care management plans for patients assigned to patient panels, managing clinic workflow, ensuring patients are placed in examination rooms in a timely manner, and providing direction to patients as they move through the clinic environment. Duties include but are not limited to the ability to perform a variety of specialized clinical support skills, ability to perform basic patient care service, have knowledge of medical terminology, demonstrate skills in interpersonal communication, demonstrate knowledge of aseptic technique and infection control and knowledge of patient confidentiality, policies and procedures. Shall assist all health care providers in performing patient care services and duties pertaining to the effective and efficient delivery of patient centered care in all clinical areas.

Option 2: Unlicensed Assistive Personnel (Health Technician, Nursing Assistant). This staffing is part of the PACT Teamlet, not the primary telehealth clinical technician (TCT): Qualifications: Nursing Assistant qualifications per VA qualification standards for Nursing Assistant located in the VA Qualification Standards. See the Policy and Regulations section of this PWS for VA Qualification Standards access information. Health Technician qualifications per Health Aid and Technician Series 0640 located at: [Health Aid and Technician Series 0640 \(opm.gov\)](https://www.opm.gov/pers-data/pdm/jsp/positions/series.jsp?series=0640). **Position Responsibilities:** Responsible for the provision of covered services to assigned and unassigned patients presenting for care at the site, providing evaluation and care consistent with licensure, certification, and functional statement with elements of practice, to patients assigned PACTs, collaborating with PACT staff to develop comprehensive health care plans and care management plans for patients assigned to patient panels, managing clinic workflow, ensuring patients are placed in examination rooms in a timely manner, and providing direction to patients as they move through the clinic environment. Duties include but are not limited to the ability to perform a variety of specialized clinical support skills, ability to perform basic patient care service, have knowledge of medical terminology, demonstrate skills in interpersonal communication, demonstrate knowledge of aseptic technique and infection control and knowledge of patient confidentiality, policies, and procedures. Shall assist all health care providers in performing patient care services and duties pertaining to the effective and efficient delivery of patient centered care in all clinical areas.

Teamlet Member 4: Administrative Associate: FTE Ratio Performance Standard: Current standard is 1.0 FTE administrative associate per 1.0 FTE PCP. Qualifications: Required education and experience demonstrating skills and abilities to provide clerical support and administrative functions to support site operations. Completion of scheduling training per VA policies and guidance. See VA qualification standards for Medical Support Assistant located in the VA Qualification Standards for additional qualifications. See the Policy and Regulations section of this PWS for VA Qualification Standards access information. **Position Responsibilities:** Responsible for the provision of covered services to assigned and unassigned patients presenting for care. Provide clerical support and administrative functions to PACT staff and collaborate with PACT staff to incorporate the logistical elements of care coordination into comprehensive care management plans. Provide guidance and direction to patients and personal support persons for navigating the VA health care system and administrative functions in VA. Provide scheduling services in accordance with VA policies and guidance.

2.3 Discipline Specific PACT Team Member (Required for all sites): Discipline-specific team members are designated in PCMM for one or more PACT(s). Discipline-specific team members provide continuity of direct discipline-specific care to all patients assigned to PACT(s) for which the team member is designated.

The contractor shall place consults for the following disciplines referring the Veterans to the VA for Clinical Pharmacist Practitioner (CPP) Anti-Coagulation, registered Dietitian/Nutritionist, Primary Care Mental Health Integration (PCMHI), and Social Work Services if the services are required.

2.5 General and Specialty Mental Health Staffing Requirements and Options: VA will provide by referral or by telehealth as needed. If telehealth or VVC is needed, Veterans can report to Lincoln County CBOC to be seen one of the telehealth rooms by the VA virtually.

2.6 Telehealth Services Clinical Staff: Contractor will not be required to provide Telehealth staff. However, contract staff may be expected to perform some Telehealth support. All contractor staff who support and manage telehealth services must be working within permitted licensure and scope of practice. The only contracted roles at clinic are RN, LPN, and MSA.

License and Accreditation: All contract licensed providers assigned by the Contractor to perform the services covered by this contract shall have a current license to practice in the state where the outpatient clinic is located. If the state where the outpatient clinic is located recognizes multistate licensure through a compact and a contract licensed provider's home state is a party to such compact, and no other restrictions apply, then the provider's compact licensure status in the state where the outpatient clinic is located may be acceptable as long as the provider complies with all licensure requirements of the multistate compact for the entirety of the provider's performance of services under the contract.

All licenses held by the personnel working on this contract shall be full and unrestricted licenses. Contract providers who have current, full, and unrestricted licenses in one or more states, but who have, or ever had, a license restricted, suspended, revoked, voluntarily revoked, voluntarily surrendered pending action, or denied upon application will not be considered for the purposes of this contract.

Contractor staff shall be technically proficient in the skills necessary to fulfill the government's requirements, including the ability to speak, understand, read, and write English fluently.

The contractor shall ensure that all individuals who provide services and/or supervise services at the contractor's outpatient site of care, including individuals furnishing services under contract are qualified to provide or supervise such services.

Position specific competencies shall be completed for all staff annually.

Contractor staff qualifications, licenses, certifications shall be maintained throughout the contract period of performance. If contractor's staff is not directly employed by the treating facility, documentation shall be provided to the COR to ensure adequate certification. All actions required for maintaining certification shall be kept up to date always. Documentation verifying current licenses and certifications shall be provided by the contractor on an annual basis.

The contractor is responsible for assuring that all persons, whether they be staff, agents, subcontractors, providers, or anyone acting for or on behalf of the contractor, are properly licensed always under the applicable state law and/or regulations of the provider's license, and shall be subject to credentialing and privileging requirements by VA.

The contractor shall not permit any staff to begin work at an outpatient site of care prior to confirmation from the VA that the individual's background investigation has been reviewed and released to the Office of Personnel Management (OPM), by the Security and Investigations Center (SIC), and that credentialing and privileging requirements have been met. A copy of licenses shall be provided with offer and will be updated annually. Any changes related to the providers' licensing or credentials will be reported immediately to the VA Credentialing Office. In accordance with FAR 52.212-4 Contract Terms and Conditions – Commercial Products and Commercial Services failure to comply with these terms and conditions may result in all rights and remedies provided by law.

The contractor shall notify COR, CO, and the VA Chief of Staff when any provider furnishing services under this contract is reported to the National Practitioner Data Bank. This notification shall include the name, title, and specialty of the provider.

The contracted facility shall meet all or exceed VA health care accreditation requirements in VHA Directive 1100.16. Documentation verifying accreditation requirements shall be provided by the contractor on an annual basis.

- 2.9 Credentialing and Privileging:** Credentialing and privileging of all health care providers who are permitted by law and the facility to practice independently will be done in accordance with the provisions of VHA Directive 1100.21, Privileging. Credentialing for all health care professionals who claim licensure, certification, or registration, as applicable to the position and who are not currently credentialed in accordance with VHA Directive 1100.21, Privileging, shall be credentialed in accordance with VHA Directive 1100.20, Credentialing of Health Care Providers. VHA Credentialing and Privileging Office Standard Operating Procedures (SOPs) define processes from VHA Directives 1100.20 and 1100.21, to include: VHA policy concerning VetPro; the Temporary Appointment Process; credentialing during activation of the Facility Disaster Plan; requirements for querying the FSMB; credentialing and privileging requirements for telemedicine and remote health care; clarifications for the Summary Suspension of Privileges process in order to ensure both patient safety and practitioner rights; and the credentialing requirements for other required providers.

Contractor shall ensure that all health care professionals who are permitted by law and the facility to practice independently participate in the credentialing and privileging process through VHA's electronic credentialing system, "VetPro". No services are to be provided by any contract provider requiring credentialing and privileging until the parent VA facility Medical Executive Board and Medical Center Director (or designee) have granted approval. The contractor shall be provided copies of current requirements and updates as they are published.

Credentials and privileges shall require renewal annually in accordance with VA policies. Credentialed providers assigned by the contractor to work at the site shall be required to report specific patient outcome information, such as complications, to the VA. Quality improvement data provided by the contractor and/or collected by the VA will be used to analyze individual practice patterns. The Service Chief, Primary Care Service Line will utilize the data to formulate recommendations to the Medical Executive Board when clinical privileges are being considered for renewal.

Contractor shall ensure that all health care professionals who claim licensure, certification, or registration, as applicable to the position and who are not currently credentialed in accordance with VHA Directive 1100.21 participate in the credentialing process through VA's "VetPro," in accordance with VHA Directive 1100.20 and 1100.21. No services are to be provided by any contract provider requiring credentialing until the VA parent facility has granted approval. The contractor shall be provided copies of current requirements and updates as they are published. Since contracted nurse practitioners, clinical pharmacist practitioners, and physician assistants are not recognized by the VA as independent practitioners, they function under a VA Scope of Practice (not Clinical Privileges). The VA Scope of Practice shall adhere to applicable practice acts within that state. The credentials and scope of practice for nurse practitioners, clinical pharmacist practitioners, and physician assistants are reviewed at the time of the initial appointment and at least every three years thereafter by an appropriate VA discipline-specific Professional Standards Board.

- 2.10 Continuing Medical Education (CME)/Certified Education Unit (CEU):** Contractor staff registered or certified by national/medical associations shall continue to meet the minimum standards for CME to remain current. CME hours shall be reported to the credentialing office for tracking. These documents are required for both privileging and re privileging. Failure to provide will result in loss of privileges.
- 2.11 Training (VA Mandatory/ACLS/BLS):** Contractor staff shall complete VA mandatory training as requested and complete ACLS/BLS training and keep ACLS/BLS certifications current throughout the life of the contract. Certification refers to the successful completion of an American Heart Association (AHA) or Military Training Network (MTN) approved BLS or ACLS course. The AHA and MTN are the only programs acceptable by VA for obtaining required certification. Copies of current certifications shall be provided to the COR. ACLS certification is required for health care personnel that order, administer, monitor, or supervise moderate sedation, monitored anesthesia care, or general anesthesia, according to current VHA policy. The contractor shall coordinate, and cover contract staff travel expenses in accordance with General Services Administration (GSA) rates.
- 2.12 Access to Patient Information:** In performance of official duties, contractor's provider(s) have regular access to printed and electronic files containing sensitive data, which shall be protected under the provisions of the Privacy Act of 1974 (5 U.S.C. 552a), Health Insurance Portability and Accountability Act (HIPAA) of 1996 (45 CFR Part 160 and 164) and other applicable laws, federal regulations, Veterans Affairs statutes, and policies. Contractor's provider(s) are responsible for (1) protecting that data from unauthorized use,

release/disclosure or from loss, alteration, or unauthorized deletion and (2) following all applicable regulations and instructions regarding access to computerized files, release of access codes, etc., as set out in VA Rules of Behavior which contract provider(s) signs.

Contractor staff shall complete required VA Information Security and Privacy training, review and sign the VA Rules of Behavior prior to having access to the VA computer system. Contractor staff who have access to PHI and PII shall also complete VHA Privacy and HIPAA Focused training prior to having access to PHI and PII. Both VA Information Security and Privacy training and VHA Privacy and HIPAA Focused training will be completed **annually**. Upon completion of the training(s), contractor shall email or fax training certificates to the CO at [REDACTED]

In addition, if providing clinical services, contractor staff will attend VA EHR training prior to providing any patient care services. Contractor staff shall document patient care in the VA EHR to comply with all VA policies.

All contractor staff requiring access to PHI / Encrypted information shall obtain a PIV card to ensure secure communications to and from the VA (e.g., Clinical staff, contract billing staff requiring patient lists with PHI/PII and Contract management personnel requiring PHI/PII information).

- 2.13 Rules of Behavior for Automated Information Systems:** Contractor staff having access to VA Information Systems are required to read and sign a Rules of Behavior statement which outlines rules of behavior related to VA Automated Information Systems. The COR will provide, through the facility information security officer, the Rules of Behavior to the contractor for the respective facility. A copy of the Rules of Behavior may also be found in the attachments section.
- 2.14 Standard Infection Control Measures (PPD, immunizations, etc.):** Contractor shall adhere to current CDC Guidelines for Infection Prevention and Control in Health Care Settings (located at: [Guidelines and Guidance Library | Infection Control | CDC](#)). Contractor shall provide statement that all required infection control testing and immunizations for their personnel are current, and that the contractor is compliant with OSHA regulations concerning occupational exposure to blood borne pathogens and Tuberculosis . All clinic staff are required to receive annual influenza vaccination or obtain an exemption for medical or religious reasons. Staff who are granted an exemption are required to wear a face mask throughout the influenza season. (See VHA Directive 1192.01 Seasonal Influenza Vaccination Program for VHA Health Care Personnel for more information). The contractor shall also notify the VA of any significant communicable disease exposures and the VA will also notify the contractor of the same, as appropriate. Contractor shall adhere to CDC Hand Hygiene Guidelines for Healthcare settings (located at: [About Hand Hygiene for Patients in Healthcare Settings | Clean Hands | CDC](#)); and provide antimicrobial hand soap and alcohol rub that is at least 60% ethyl alcohol (ethanol) or 70% isopropanol for Hand hygiene.
- 2.15 National Provider Identification (NPI):** All contractors who provide billable healthcare services to VA or VHA, shall obtain a NPI as required by the Health Insurance Portability and Accountability Act (HIPAA) National Provider Identifier Final Rule, administered by the CMS. This rule establishes assignment of a 10-digit numeric identifier for contractor staff, intended to replace the many identifiers currently assigned by various health plans. Contractor staff needs only one NPI, valid for all employers and health plans. Contractor staff shall also designate their specialties/subspecialties by means of taxonomy codes on the NPI application. The NPI may be obtained via a secure website at: <https://nppes.cms.hhs.gov/NPPES>
- 2.16 Prescription Drug Monitoring Program:** Contractor's providers shall register with the state prescription drug monitoring program in accordance with VHA Directive 1306, Querying State Prescription Drug Monitoring Programs (PDMP).
- 2.17 Conflict of Interest:** The contractor and all contractor's personnel are responsible for identifying and communicating to the CO and COR conflicts of interest at the time of proposal and during the entirety of contract performance. At the time of proposal, the contractor shall provide a statement which describes, in a concise manner, all relevant facts concerning any past, present, or currently planned interest (financial, contractual, organizational, or otherwise) or actual or potential organizational conflicts of interest relating to the services to be provided. The contractor shall also provide statements containing the same information for any identified consultants or subcontractors who shall provide services. The contractor shall also provide relevant facts that show how its organizational and/or management system or other actions would avoid or

mitigate any actual or potential organizational conflicts of interest. These statements shall be in response to the VAAR provision 852.209-70 Organizational Conflicts of Interest and fully outlined in response to the subject attachment in Section D.

- 2.18 Citizenship Related Requirements:** The contractor certifies that the contractor shall comply with all legal provisions contained in the Immigration and Nationality Act of 1952 (as amended); its related laws and regulations that are enforced by the Department of Homeland Security and the Department of Labor as these may relate to undocumented noncitizens and/or non-immigrant foreign nationals working under contract or subcontract for the contractor while providing services to Department of Veterans Affairs.

While performing services for the Department of Veterans Affairs, the contractor shall not knowingly employ, contract, or subcontract with an undocumented noncitizen or foreign national non-immigrant who is in violation their status, because of their failure to maintain or comply with the terms and conditions of their admission into the United States. Additionally, the contractor is required to comply with all E-Verify requirements consistent with Executive Order 12989 and any related pertinent Amendments, as well as applicable Federal Acquisition Regulations (including FAR Subpart 22.18).

If the contractor fails to comply with any requirements outlined in the preceding paragraphs or above referenced laws and regulations, the Department of Veterans Affairs may, at its discretion, require that the undocumented noncitizen or foreign national who has failed to maintain their legal status in the United States or otherwise failed to comply with the requirements of the laws administered by the Department of Homeland Security and/or the Department of Labor, shall be prohibited from working at the contractor's place of business that services Department of Veterans Affairs patient referrals; or other place where the contractor provides services to Veterans who have been referred by the Department of Veterans Affairs; and may form the basis for termination of this contract for breach.

The contractor acknowledges that this certification concerns a matter within the jurisdiction of an agency of the United States. The contractor further acknowledges that this certification is subject to Title 18, United States Code, Section 1001, and, as such, a false, fictitious, or fraudulent certification may render the offeror subject to criminal, civil, or administrative penalties, including prosecution.

The contractor agrees to obtain a similar certification from its subcontractors. The certification shall be made as part of the offerors response to the RFP using the subject attachment in Section D.

- 2.19 Annual Office of Inspector General (OIG) Statement:** In accordance with The Health Insurance Portability and Accountability Act (HIPAA) and the Balanced Budget Act (BBA) of 1977, the VA OIG has established a list of parties and entities excluded from Federal health care programs. Specifically, the listed parties and entities may not receive Federal Health Care program payments due to fraud and/or abuse of the Medicare and Medicaid programs.

Therefore, all contractors shall review the OIG List of Excluded Individuals/Entities (LEIE) on the OIG web site at <http://oig.hhs.gov/exclusions/index.asp> to ensure that the proposed Contract staff and/or firm(s) are not listed. Contractors should note that any excluded individual or entity that submits a claim for reimbursement to a Federal health care program, or causes such a claim to be submitted, may be subject to a civil monetary penalty (CMP) for each item or service furnished during a period that the person or entity was excluded and may also be subject to treble damages for the amount claimed for each item or service. CMP's may also be imposed against the contract staff and entities that employ or enter contracts with excluded individuals or entities to provide items or services to Federal program beneficiaries.

By submitting their proposal, the contractor certifies that the OIG LEIE has been reviewed and that the contractor and its subcontractors are not listed as of the date the proposal was signed. In addition, contractor will review the OIG LEIE annually to verify compliance.

- 2.20 Non-Personal Services:** The parties agree that the contractor, contract staff, agents and sub-contractors shall not be considered VA employees for any purpose. All individuals that provide services under this resultant contract and are not staff of the contractor shall be regarded as subcontractors. The contractor shall be responsible and accountable for the quality of care delivered by all its subcontractors. The contractor shall be responsible for strict compliance of all contract terms and conditions without regard to who provides the service.

2.21 Contract Personnel: The contractor shall be responsible for protecting all contractor personnel furnishing services. To carry out this responsibility, the contractor shall provide or certify that the following is provided for all contract staff providing services under the resultant contract:

- Workers' compensation
- Professional liability insurance
- Health examinations
- Income tax withholding, and
- Social security payments

2.22 Inherently Governmental Functions Prohibited: This includes, but is not limited to, determination of agency policy, determination of Federal program priorities for budget requests, direction and control of government employees, selection or non-selection of individuals for federal Government employment including the interviewing of individuals for employment, approval of position descriptions and performance standards for federal employees, approving any contractual documents, approval of federal licensing actions and inspections, and/or determination of budget policy, guidance, and strategy.

2.23 Tort Liability: The Federal Tort Claims Act does not cover contract staff. When a contract staff member has been identified as a provider in a tort claim, The contractor's staff member shall notify the contractor's legal counsel and/or insurance carrier. Any settlement or judgment arising from a contractor's provider's action or non-action is the responsibility of the contractor and/or insurance carrier.

2.24 Ryan Haight Act: In support of providing Veterans access to comprehensive Telehealth services, including the provision of controlled substances in compliance with the Ryan Haight Act, contractor shall apply for registration if this option is available under state law. If DEA registration is not available under state law or the contractor is unable to obtain DEA registration, contractor shall ensure a DEA registered provider can be present in the room with the patient during discussions of controlled substances prescriptions, at telehealth visits in which controlled substances are prescribed, if the patient has not had at least, one prior in-person medical assessment with the prescribing provider.

2.25 Individual Provider Requests to Opt-Out of Care: Individual providers shall raise any requests to opt out of the provision of specific care required by this PWS, including those based on a Rights of Conscience (ROC) Claim or on a sincerely held religious belief, to the contractor. Contractor is responsible for evaluating the individual provider's request and deciding whether to provide an accommodation. Absent express, written approval by VA, contractor shall ensure all services included in this PWS are available to all patients in a timely manner based on the applicable standard of care even if an individual provider has requested to opt out of providing certain services to the patient.

3. HOURS OF OPERATION: The following outlines the required hours of operation:

3.1 Business Hours: Services shall be available from the contractor Monday – Friday, between the core hours of 8:00 am and 5:00 pm.

3.2 Evening Hours: Occasionally extended hours may be necessary. Extended hours would include any times beyond core hours.

3.3 Weekend Hours: Time beyond the core hours may be necessary but not indicated at this time.

3.4 Federal Holidays: The following holidays are observed by the Department of Veterans Affairs: New Year's Day, Washington's Birthday, Martin Luther King's Birthday, Memorial Day, Juneteenth, Independence Day, Labor Day, Columbus Day, Veterans Day, Thanksgiving, Christmas, any day specifically declared to be a national holiday.

4. CONTRACTOR RESPONSIBILITIES:

4.1 General: Contractor performing services under this contract shall provide a continuum of care from prevention to diagnosis and treatment, to appropriate referral and follow-up. Contractor's outpatient site of care shall have the necessary professional medical staff, diagnostic testing and treatment capability, and referral arrangements needed to ensure continuity of health care. The contractor shall provide services solely

dedicated to Veterans regardless of gender or age. Those patients needing specialty care shall be referred to VA.

NOTE: Patients referred for care outside the contractor's clinic (e.g., to VA Community Care, VA facility, or similar) for care that the contractor is required to provide under the terms of this contract will be removed from the applicable billable roster the month after the patient is referred for care.

4.2 Registration and Financial Assessments:

Registration and Enrollment: Registration is the adding of a Veteran or patient to the VHA Electronic Health Record (EHR) for the purpose of receiving VA sponsored health care or services at a VA medical facility. Enrollment is the acceptance of an eligible Veteran into the EHR and assignment to an enrollment priority group. Not all eligible Veterans are required to enroll in the EHR to receive health care services. Eligible and enrolled Veterans may seek care at any VA medical facility without being required or requested to reestablish eligibility for VA health care. Veterans who are enrolled in the EHR will remain continuously enrolled unless the Veteran submits a signed written request to be disenrolled. Veterans who are disenrolled, may reapply for enrollment at any time. However, they will be subject to new eligibility rules at that time. The contractor, or patient will call [REDACTED]

Financial Assessments: Financial assessment is the process known as a Means Test (MT) that is used by VA to assess the gross household income of Veterans who are not otherwise eligible for enrollment in the EHR. The MT determines a Veteran's copayment responsibilities, assists in determining enrollment priority group assignment, and assists in determining beneficiary travel benefits. The contractor, or patient will call [REDACTED]

4.3 Episodic Care for Unassigned/Unenrolled Patients:

Contractor shall provide at no additional cost approximately 7 nurse-only visits per month and 5 provider visits per month to Veterans who are not assigned for care at the contractor's outpatient site of care. This may occur when an unenrolled Veteran presents to contractor's site of care seeking care and when an enrolled Veteran not assigned to the contractor's site of care presents to the contractor's site of care seeking limited, episodic care. After initial enrollment occurs, Veterans are eligible to receive health care services at any facility where VA services are offered once registered at that facility. These services may be planned or unplanned episodes of care. The contractor shall ensure that unassigned and unenrolled Veterans are triaged by an appropriate clinical staff member and that any care that can be provided by the nurse and/or provider is provided. Contractor shall provide care for unassigned/traveling Veterans in accordance with VHA Handbook 1101.11(4), "Coordinated Care for Traveling Veterans". See the SOP and flow maps referenced in Handbook 1101.11(4) for specific requirements.

Telehealth support for patients not assigned at the outpatient site of care - At no additional cost the contractor shall provide approximately 10 telehealth visits with the VA parent facility for Veterans who are not assigned for care at the contractor's outpatient site of care per month. These visits occur when an unassigned Veteran eligible for VA health care requires a telehealth visit with the VA parent facility (vs. requiring the patient traveling to the VA parent facility). The contractor shall support the scheduling and visit management as per requirements and normal routine as defined in the PWS.

4.4 Standards of Practice: Contractor shall be responsible for meeting or exceeding VA policies and accreditation requirements in VHA Directive 1100.16.

4.5 Health Equity: Equitable access to high-quality care for all Veterans is a major tenet of the VA health care mission. To that end, the contractor shall provide appropriate individualized health care to each Veteran in a way that eliminates disparate health outcomes and assures health equity.

5. PATIENT CARE:

5.1 Primary Care Services:

Synchronous Care: It is the expectation that 80% of a provider's FTE be bookable time for synchronous care (30 hours per week for a full-time provider). These visits may be in person or virtual (VA Video Connect, CVT, telephone) with a Veteran and involve clinical decision-making in a two-way conversation.

Asynchronous Care: It is the expectation that 20% of a provider's FTE be dedicated to asynchronous care and includes but is not limited to the following activities: refilling prescriptions, reviewing outside records, huddles and case management, team meetings, processing view alerts, secure messaging, and training.

PACT Training: The PACT curricula include multiple training opportunities. While all PACT training sessions are recommended, all staff working in primary care are required to complete PACT 100. The PACT 100 curriculum provides foundational knowledge of the PACT model and tools/resources needed to succeed in application of the model in daily work. PACT 100 is offered as 8-1-hour interactive live virtual trainings or TMS on-demand courses. The contractor shall ensure all contractor staff working in primary care complete PACT 100 training within 90 calendar days of assignment to primary care.

Whole Health is an approach to care that focuses on the whole person and empowers and equips people in supporting their own health and well-being. It is an approach that is very aligned with the mission of primary care. Whole Health in primary care is a two-hour live virtual training designed to help PACT members provide care that is personalized, proactive, and patient driven. The contractor shall ensure all contractor staff working in primary care complete Whole Health in primary care within 90 calendar days of assignment to primary care.

Primary Care Tasks Summary: VA employs the patient centered medical home model of primary care, which in VA is called the Patient Aligned Care Team (PACT). VHA Handbook 1101.10 "Patient Aligned Care Team" outlines complete requirements for the PACT model. The PACT delivery model is predicated on a foundation of delivering care that is patient centered, team based and continuously striving for improvement. Important components of the model include patient centered care, access, care management and coordination as well as redesigning the team and work. Contractor shall provide all services in accordance with VHA Handbook 1101.10. Information provided below summarizes the PACT model and contractor requirements. See VHA Handbook 1101.10 for more comprehensive information and requirements.

Enhance Patient Centered Care (PCC): Establishing a patient centered practice environment and philosophy as a core principle of PACT requires a knowledgeable staff and an engaged, activated patient and family. Contractor staff shall be required to complete the following tasks to begin to implement PCC:

- Engage the patient/family in self-management and personal goal setting.
- Provide education pertinent to care needs and document the provision of that education.
- Provide support on site to enroll patients in My HealtheVet & secure messaging.
- Ensure staff is trained in self-management techniques, motivational interviewing, shared decision making as made available by VA.
- Provide access to complementary and integrative health care services in accordance with VHA Directive 1137 Provision of Complementary and Integrative Health.
- Clinic patients will be notified of all test results requiring action within 7 calendar days and all test results not requiring action within 14 calendar days. For critical results that represent an imminent danger to the patient, the contractor shall notify the patient immediately.

Enhance Access to Care: PACT strives for superb access to care in all venues including face to face and virtual care. Contractor is expected to enhance access to care by offering care in the following modalities:

- **Face to Face Visit Access:** Provide same day access for patients and increase (or establish) group visits and shared medical appointments.
- **Virtual Access:** The contractor shall provide the following virtual access:

Telephones: Phones should be answered by a “live” person with a focus on achieving first call resolution. First call resolution is taking care of the Veteran’s issue/request during that call. This approach requires thoughtful planning and strategy.

VA’s Patient Portal (e.g., My HealtheVet (MHV) or VA.gov): Provide training and support to enroll Veterans into VA’s patient portal and mobile applications to include face-to-face and online assistance for a premium account such as use of secure messaging, access to health records, refilling prescriptions, and appointment management.

Secure Messaging (SM): Encourage and educate patients and providers to use SM as VA’s approved non-synchronous mode of communication; promote SM as a communication method in clinic, and increase Veteran and provider participation and adoption of SM.

Mobile Applications: Educate patients about mobile applications such as Annie, Mental Health Assistant, My VA Images, VA Share My Health App, and others that may facilitate patient engagement and/or sharing of patient generated health data (PGHD) with the healthcare team for promotion of self-management of health.

- **Telemedicine & Telehealth:**

Improve access to scarce medical services via telemedicine.

Increase Veteran enrollment in telehealth modalities.

Provide telehealth services using VVC and other telehealth modalities.

Enhance Care Management & Coordination of Care: Improving systems and processes associated with critical patient transitions, managing populations of patients and patients at high risk has proven to have a positive impact on quality, patient satisfaction and utilization of high-cost services such as acute inpatient admissions, skilled nursing facility stays, and emergency department visits. Clinic staff shall take the following actions to achieve improvements:

Improve Critical Transitions Processes: Inpatient to Outpatient: develop systems to identify admitted primary care patients; provide follow up care either by face-to-face or virtual visit within 2 business days’ post discharge and document the follow up care in the VA EHR and communicate among the team.

Enhance Primary Care to Specialty Care Interface:

Participate in electronic virtual consults & VA ECHO as available.

Develop resource listing of specialty care points of contact for nursing and medical care.

Participate in VA parent facility sponsored medical educational activities to enhance networking with specialty staff.

Provide the annual Prevention of Amputation in Veterans Everywhere (PAVE) foot check as required in VHA Directive 1410 Prevention of Amputation in Veterans Everywhere which includes visual inspection, palpation of pulses, and sensory testing with monofilament. If estimated to be a foot risk score of 2 or 3 a referral to podiatry shall be made.

Enhance VA & Community Interfaces in Caring for Veterans.

Develop a list of community points of contact.

Develop mutually agreeable interface systems with community facilities and providers.

Improve Systems for Managing the Care of Patient Populations:

Enhance management of patients with chronic illness.

Identify patients with suboptimal chronic disease indices from VHA databases (registries).

Develop plans including staff roles and responsibilities in addressing care needs. Include all team members in delivering care as license allows. Use face to face or virtual care delivery methods such as pharmacy/nurse clinics, telephone clinic etc.

Enhance Health Promotion & Disease Prevention Focus in Care Delivery:

Identify patients with preventive care needs from VHA databases (registries).

Develop & implement plans including staff roles and responsibilities in addressing care needs. Include all team members in delivering care as license allows. Use face to face or virtual care delivery methods such as pharmacy/nurse clinics, telephone clinic etc.

Clinical PACT teamlet members (PCP, RNCM, and clinical associate) shall attend VA TEACH and Motivational Interviewing training within 1 year of assignment to the PACT.

Enhance Management of High-Risk Veterans: (such as Veterans with frequent emergency department visits, frequent inpatient admissions for ambulatory sensitive conditions, and severely injured/disabled, frail elderly).

Identify high risk patients from VHA databases (registries).

Develop plans including staff roles and responsibilities in addressing care needs. Include all team members in delivering care as license allows.

Use face to face or virtual care delivery methods such as pharmacy/nurse clinics, telephone clinic etc.

Improve Practice Design & Flow to Enhance Work Efficiency & Care Delivery:

Maximize functioning of all team members through role and task clarification for workflow processes. Ensure all team members work to their maximum ability/skill/license.

Develop a plan to improve workflow process for visit or virtual care.

Conduct daily teamlet huddles to focus on operational needs for that day.

Conduct weekly team meeting to focus on systems and process improvements, review and use data to monitor processes, etc.

Primary Care Services Scope of Care: Contractor shall use the PACT model to provide primary care services supporting a continuum of care from prevention to diagnosis and treatment, to appropriate referral and follow-up. The contractor shall be responsible for scheduling initial and/or follow-up appointments with primary care providers and other staff at the contractor's outpatient site of care (or via virtual modalities) for workload that can be appropriately managed in a primary care outpatient environment to include (but not limited to) care for: preventive screenings and procedures, hypertension, depression, ischemic heart disease, anxiety, substance use disorder, other mental health conditions, hypercholesterolemia, degenerative arthritis, congestive heart failure, respiratory infection, cerebral vascular disease, chronic obstructive pulmonary disease (COPD), peripheral vascular disease, urinary tract infection, diabetes mellitus, common dermatological conditions, acute and chronic pain, acute wound management, gastric disease, skin ulcers (stasis and dermal), anemia, genitourinary (GU) issues, stable chronic hepatic insufficiency, constipation, osteoporosis, common otic and optic conditions, basic diagnostic, evaluation, and tests for infertility, pharmacology in pregnancy & lactation, evaluation & treatment of vaginitis, amenorrhea/menstrual disorders, evaluation of abnormal uterine bleeding, menopause symptom management, diagnosis of pregnancy and initial screening tests, evaluation and management of acute and chronic pelvic pain, recognition and management of postpartum depression and postpartum blues, evaluation and management of breast symptoms, (mass, fibrocystic breast disease, mastalgia, nipple discharge, mastitis, galactorrhea, mastodynia), crisis intervention, evaluation of psychosocial, well-being and risks including issues regarding abuse, intimate partner violence screening, physical, emotional, verbal, and psychological abuse, preconception counseling and assessment of abnormal cervical pathology.

Extended Hours: The contractor shall meet the following requirements for Extended Hour Clinics if requested by the VA.

5.2 Women Veterans Health Care Services:

Comprehensive primary care and specialty women's health services: The contractor shall provide comprehensive primary care in accordance with VHA Directive 1330.01(7) Health Care Services for Women Veterans (or subsequent policy). Comprehensive primary care for women Veterans is defined as the availability of complete primary care from one primary care provider at one site. The primary care provider shall, in the context of a longitudinal relationship, fulfill all primary care needs, including acute and chronic illness, gender-specific, preventive, and mental health care. The full range of primary care needs for women Veterans includes care for acute and chronic illness such as routine detection and management of disease such as acute upper respiratory illness, cardiovascular disorders, cancer of the breast, cervix, colon, and lung, diabetes mellitus, osteoporosis, thyroid disease, COPD, mental health conditions, etc. Gender-specific primary care, delivered by the same provider, encompasses preconception care, sexuality, contraception- including same day access to emergency contraception-, pharmacologic issues related to pregnancy and lactation, management of menopause-related concerns, and the initial evaluation and treatment of gender-specific conditions such as pelvic and abdominal pain, abnormal vaginal bleeding, vaginal infections, infertility, pregnancy options counseling and abortions (when not excluded pursuant to 38 C.F.R. §17.38), etc. Preventive care includes services such as age-appropriate cancer screening, weight management counseling, smoking cessation, immunizations, etc. The same primary care provider shall screen and appropriately refer patients for military sexual trauma as well as evaluate and treat uncomplicated mental health disorders and substance use disorders.

When specialty care is necessary, the primary care provider shall coordinate this care and communicate with the specialty provider regarding the evaluation and treatment plan to ensure continuity of care.

NOTE: When a women Veteran requests a provider of a specific gender, accommodation shall be made. To satisfy the patient request, referral to parent VA facility or VA Community Care may be utilized. In cases of emergent delivery of care, the accommodation may not be possible, and this can be documented in the VA EHR as such.

Breast Cancer Screening: The contractor shall provide breast cancer screening in accordance with VHA Directive 1330.01(7) Health Care Services for Women Veterans (or subsequent policy) and VHA guidelines which follow the American Cancer Society recommendations.

Breast cancer imaging reports (written or electronic) received directly by the contractor from a VA Community Care provider shall be scanned into the VA EHR within 7 days. All reports must include the appropriate BI-RADS code including the FDA mandatory final assessment wording category. Veteran's VA PCP, ordering provider, and Community Nurse Care Manager Laura DaCorte RN CCM should be alerted on in the VA EHR. Any questions regarding Breast Cancer Screening should be directed to [REDACTED]

Screening, diagnostic mammograms, breast ultrasounds (US) and MRIs will be provided at the VA parent facility or through VA Community Care when Veterans meet the criteria for and elect to have these services provided through VA Community Care. The contractor's provider shall place an order into the VA EHR for services provided at the VA parent facility. If the Veteran meets the criteria for VA Community Care and elects to have these services through VA Community Care the contractor shall enter both an order and a VA Community Care consult in the VA EHR. There is an approved Standardized Episode of Care (SEOC) that authorizes a woman to have a screening mammogram, diagnostic mammogram, breast ultrasound, and breast biopsy, if medically indicated.

Breast cancer imaging reports (written or electronic) received directly by the contractor from a VA Community Care provider shall be scanned into the VA EHR.

Any questions or concerns regarding mammograms should be directed to [REDACTED]

Screening mammogram, diagnostic mammograms, breast ultrasound and MRIs will be provided through VA Community Care. The contractor's provider shall enter both an order and a VA Community Care consult into the VA EHR. There is an approved Standardized Episode of Care (SEOC) that authorizes a woman to have a screening mammogram, diagnostic mammogram, breast ultrasound, and breast biopsy, if medically indicated.

Breast cancer imaging reports (written or electronic) received by the contractor directly from a VA Community Care provider shall be scanned into the VA EHR.

The ordering provider or designee shall communicate the results of normal mammograms to the patient within 14-calendar days of receiving the results. All mammogram results requiring action shall be communicated by the ordering provider or designee to patients no later than 7-calendar days from the date the results are available to the ordering provider. Communication shall be documented in the VA EHR. If indicated, the VHA ordering provider is expected to also communicate and document a follow up diagnostic or treatment plan. The fact that an outside radiologist may discuss findings with the patient does not remove the obligation of the VHA ordering provider to discuss the findings and a follow-up plan with the patient. Significant abnormalities may require review and communication in shorter timeframes and 7 calendar days represents the outer acceptable limit. For abnormalities that require immediate attention communication needs to occur in the timeframe that minimizes risk to the patient.

Cervical Cancer Screening: The contractor shall provide cervical cancer screening in accordance with VHA Directive 1330.01(7) Health Care Services for Women Veterans (or subsequent policy). The cervical pathology report of normal results shall be communicated to the patient in terms easily understood by a layperson within 14-calendar days from the date of the pathology report and the Human Papilloma Virus (HPV) report becoming available to the ordering provider. Documentation of a letter and/or verbal communication with the patient shall be entered into VA EHR. If using the United States Postal Service, confirmation of the receipt of these results is not required. For any abnormal cervical pathology report, the results shall be communicated within 7-calendar days of the report (including both cytology and HPV) becoming available to the ordering provider.

Tracking and Care Coordination: The contractor shall follow the VA parent facility SOP for tracking and care coordination for breast and cervical cancer screening. The SOP is provided in the attachments section. Per VHA Directive 1330.01(2) Health Care Services for Women Veterans https://www.va.gov/vhapublications/ViewPublication.asp?pub_ID=5332 Each facility must have a process in place to ensure tracking and timely follow-up of findings from breast and cervical cancer screening. All Administrative Parents (Health Care Systems) must have in place standard operating procedures that specify the tracking process and assign breast, maternity, and gynecological care coordination duties to specific individuals. These duties may be assigned to individuals such as a WH-PACT RN Care Coordinator, Oncology Care Coordinators Mammogram Coordinators or Maternity Care Coordinators (see VHA Directive 1330.03). These duties should not be assigned to the WVPM who fills a leadership and management role for the Women's Health Program. See local tracking and care coordination SOP in attachments section for more information. Contractor must designate a women's health clinical liaison at each outpatient site of care to coordinate women's health services with the Women Veterans Program Manager at the main facility. The liaison is usually a nurse or social worker, but may be a provider. The role of the liaison is to be the point of contact who communicates with the WVPM about issues related to women's health care, environment of care and policy, and to communicate these messages to other staff at the clinic.

Assignment to a Women's Health -Primary Care Provider (WH-PCP): The contractor shall develop a plan to assign women to a proficient WH-PCP who has a sufficient number of women in their primary care panel to maintain competency in caring for those Veterans. The contractor shall develop a plan to assign women preferentially to the designated WH-PCP at the contracted site. All outpatient sites of care shall have at least two WH-PCPs. (Because of small populations of women at most CBOCs, CBOC WH-PCPs will usually have mixed gender panels). It is necessary to have two WH-PCPs to provide full coverage for women during sick leave and vacation. All newly enrolling women should be assigned to a WHPCP. Women in panels of non-women's health PCPs should be offered the opportunity to request transfer to a WH-PCP at the same site of care. It is recommended that women Veterans be clustered in teams where the provider and all team members have experience, knowledge, and established systems of care to provide equitable, high-quality care to women Veterans.

It is recommended that Women's Health PACT teamlets are assigned a panel size of at least 100 women Veterans, thus allowing all teamlet members to care for a volume of patients to support maintenance of expertise in the care of women.

To be initially designated as a WH-PCP, a provider shall have at least one of the following:

- Documentation of attendance at a Women's Health Mini-Residency within the previous 3 years; or
- Documentation of at least 20 hours of women's health continuing medical education (CME) or continuing education unit (CEU) within the previous 3 years; or
- Documentation of at least 3 years in a practice with at least 50% women patients within the previous 5 years; or
- Evidence of completion of an internal medicine or family practice residency; women's health fellowship; or women's health, adult or family practice NP or PA training within the previous 3 years; or
- Documentation of a current preceptorship arrangement with an experienced WH-PCP such as weekly meetings (for at least 6 months); or
- Evidence of being recognized as a known women's health leader and subject matter expert with experience practicing, teaching, and/or precepting women's health.

Renewal/retention of Women's Health PCPs status requires ten (10) hours of CME or CEU in women's health every 2 years. The contractor shall provide ongoing education and training to the Women's Health primary care providers to assure competency, proficiency, and expertise in providing care to women Veterans.

Special Considerations in the Delivery of Comprehensive Women's Health Primary Care:

- **Appointment Times/Duration and Panel Sizes:** Contractor shall follow all requirements for appointment times, appointment duration, and panel sizes per VHA Handbook 1101.10(1) Patient Aligned Care Team (PACT) Handbook and VHA Directive 1330.01(7) Health Care Services for Women Veterans, or subsequent policies).
- **Pregnancy Testing:** The contractor shall ensure access to stat pregnancy tests within 1 hour. Clinics that do not have access to onsite stat pregnancy testing shall use point of care pregnancy testing. (See mandated POC pregnancy testing in the Ancillary, Point of Care and Waived Testing Services section of this PWS).
- **Teratogenic Medication:** The contractor shall monitor the prescription of high or extreme risk teratogenic medications which could be prescribed to women who are pregnant, or with the potential to become pregnant, in the Veteran's EHR (Teratogenic Drugs Order Check system in CPRS, or the Discern Alert System in Cerner). An informed discussion with the Veteran shall cover the risks and benefits of such prescriptions and documentation of patient counseling shall be recorded in the EHR at the time of initial prescription. The contractor shall utilize the Pregnancy Intentions clinical reminder/recommendation and Teratogenic Order Checks in the EHR.

Emergency Contraception: Contractor shall ensure emergency contraception (e.g., Levonorgestrel (Plan B) etc.) is available to all women patients in a timely manner (same day of their appointment) even if a provider has requested to opt out from providing emergency contraception to the patient because of a Rights of Conscience (ROC) Claim.

Infertility Services: Consistent with VA's goal of improving Veterans' health and the quality of their lives, VA health care includes infertility evaluation and treatment. The contractor shall refer Veterans to Urology or Gynecology for infertility evaluation. Infertility services shall be provided on-site or through VA Community Care to the extent the Veteran is eligible in accordance with VHA Directive 1332(2) Fertility Evaluation and Treatment and VHA Directive 1334 In Vitro Fertilization Counseling and Services Available to Certain Eligible Veterans and Their Spouses (or subsequent policies).

Maternity and Newborn Care: VA is authorized to provide comprehensive pre-natal, intra-partum and post-partum care to eligible women Veterans. Newborn care is available for Veterans enrolled in VA care and receiving maternity benefits. Maternity benefits begin with the confirmation of pregnancy, preferably in the first trimester, and continue through the final post-partum visit, usually at 6-8 weeks after the delivery, when the Veteran is medically released from obstetric care. Providers shall initiate a VA Community Care Maternity Consult and notify [REDACTED] [REDACTED]

Pregnancy Options Counseling: Pregnancy options counseling (to include abortion counseling) is available for all eligible and enrolled Veterans. The goal of patient-centered pregnancy options counseling is to provide non-directive, individualized counseling to aid the pregnant person in understanding and exploring all possible pregnancy options so the individual can make an informed decision for their health and wellbeing that is consistent with their values and preferences for care. Providers should be knowledgeable about the different pregnancy options, relevant terms and be skilled in unbiased and effective patient-centered counseling techniques.

Abortion: Abortion may be provided to Veterans eligible for care under the medical benefits package (either in VA medical facilities or through VA Community Care) if determined needed by a health care professional when:

(a) The life or the health of the pregnant Veteran would be endangered if the pregnancy were carried to term; or
(b) The pregnancy is the result of an act of rape or incest. **NOTE:** Self-reporting from the pregnant Veteran constitutes sufficient evidence that an act of rape or incest occurred.

The decision of whether an abortion is covered under one of these exceptions is made on a case-by-case basis by the appropriate health care professional pursuant to the promote, preserve, restore standard in the medical benefits package (see 38 C.F.R. § 17.38(b)).

Abortion Counseling and Abortions in States where such Service is Prohibited by Law: Contract physician(s) and all other contract licensed providers assigned by the contractor to perform the services covered by this contract shall adhere to the state laws and regulations governing their professional license to practice. If a Veteran, assigned to, or being seen by, a contract provider, requires abortion counseling and/or an abortion in a state where such service is prohibited by law, the contract provider shall provide a warm hand off in a timeframe that maximizes access to all available options, to a VA employed provider who is able to coordinate care or provide care to the Veteran.

Abortion Counseling and Abortions in States where such Service is not Prohibited by Law: If a Veteran assigned to, or being seen by, a contract provider requires abortion counseling and/or an abortion in a state where such service is not prohibited by law, then that care shall be provided by the contract provider within their scope of practice and clinical privileges. If such care cannot be provided within the contract provider's scope of practice and clinical privileges, then contract provider shall provide a warm hand-off in a timeframe that maximizes access to all available options, to a VA provider who is able to coordinate care or provide care to the Veteran.

Chaperones: The contractor is responsible for the availability of chaperones. A chaperone shall be in the examination room during breast and pelvic exams, Pap smears, electrocardiograms, and any other procedure that exposes the breast, groin, or pubic area. The following staff shall function as chaperones: MDs, RNs, HTs, LPNs and other clinical personnel such as radiology technologists.

Women's Health Environment of Care: Contractor shall provide all equipment necessary to provide comprehensive women's health services. Examination rooms shall be set up in accordance with current VA standards to afford women with privacy (placement of examination tables in the room, privacy curtains, etc. See Space Requirements section for more details). Equipment such as privacy curtains, exam tables with stirrups and lights, bathrooms adjacent to where pelvic exams are performed, speculums, supplies, and equipment to perform Pap tests and point of care pregnancy testing should be on hand in the clinic area. Each designated women's health provider shall have an appropriate exam table to conduct the annual women's health exam. Women Veterans shall have women-only or unisex toilet rooms. Appropriately locking door hardware with locking mechanisms providing for privacy, safety, security, and utility (allowing staff members to have key or code access in case of emergency) are required for all toilets. Personal hygiene products (sanitary pads and tampons) shall be available in examination rooms, public female, unisex, and family toilet rooms at no charge. Diaper changing tables shall be available in designated public male, female, unisex, and family toilet rooms. Diaper changing tables shall be placed at least one per floor in male, female, and unisex toilet rooms, and no more than 300 feet from areas accessible to a patient. Rooms with changing table shall be identified, and toilet rooms without changing tables should include signage directing users to the nearest changing table.

Transgender Veterans: Patients will be addressed and referred to based on their self-identified gender. Room assignments and access to any facilities for which gender is normally a consideration will give preference to self-identified gender, or medical needs of the Veteran, irrespective of appearance and/or surgical history in a

manner that respects the privacy needs of transgender and non-transgender patients. For more information, please see VHA Directive 1341(3) Providing Health Care for Transgender and Intersex Veterans.

5.3 Pharmacy Services:

Prescription Fulfillment: The contractor will provide all medications that are to be administered to patients in the clinic (including vaccines in accordance with current and future Advisory Committee on Immunization Practices (ACIP)/CDC recommendations and guidance). Influenza (flu) vaccines will be offered and administered to enrolled Veterans being seen at this contractor's location, by scheduled appointments or as a walk in.

Routine prescriptions will be dispensed by the VA and mailed via VA Centralized Mail Out Pharmacy (CMOP) to the Veteran following appropriate contractor's provider order entry in the VA EHR. The VA will review all submitted non-formulary restricted, and prior authorization medication consults in a timely manner in accordance with VHA Directive 1108.08. VA Pharmacy Service will require contractor clinic management or designee to conduct routine inspections of medication storage areas monthly and provide documentation per local VA Pharmacy policy to the COR. Urgent/Emergent medications needed will be filled via contracted local pharmacy for up to a 10-day supply.

Prescriptions written for filling via a contracted local pharmacy, shall also be documented as non-VA medication in the EHR. All other prescribed medications will be via the VA EHR, faxed, or mailed to VA pharmacy for filling. The contractor shall abide by all VA policies on the storage, security, handling and disposal of all medications held in their clinic and comply with all monthly clinic inspections and the recommendations generated from those inspections.

The contractor shall be responsible for prescribing medications as medically indicated. Prior to prescribing any medications, the contractor shall review medication profiles in the VA EHR for duplicate therapy, drug-disease complications, drug-drug, drug-food, drug-lab interferences, appropriateness of dose, frequency and route of administration, drug allergy, clinical abuse/misuse, and documentation of medications obtained outside of the VA in the VA EHR "Non-VA" medications list, including over-the-counter and herbal agents and known allergies. The contractor shall also query state Prescription Drug Monitoring Programs (PDMPs) before prescribing controlled substances per VHA Directive 1306 Querying State Prescription Drug Monitoring Programs (PDMP) and review the Stratification Tool for Opioid Risk Monitoring (STORM) risk dashboard before prescribing controlled substances. Documentation for review of the PDMP database and review of the STORM dashboard will be done via templated VA EHR notes. iMED consents shall be obtained by clinic staff prior to prescribing or dispensing medications requiring consent including prescriptions for controlled substances to Veterans.

Medication orders for all medications, including controlled substances prescriptions shall be entered into the VA EHR (as per local policy) using the appropriate e-prescribing process (e.g., PIV card). In event of computer down-time, written prescriptions (on an authorized VA Form 10-2577F or other state or federally approved controlled substance order form) shall be used and shall be couriered, signature-confirmed, to the VA pharmacy-designated point of contact at the end of each business day. The VA will dispense controlled substances in accordance with Federal Law CFR Title 21 1300-end. It is fully expected that all providers will always maintain active PIV cards to comply with required prescribing guidelines on controlled substances, as applicable. Vaccines shall be documented in the appropriate immunization record in the VA EHR.

The contractor is required to utilize the VA National Formulary. The formulary is available electronically under Drug File Inquiry in the VA EHR physician package. Non-formulary, Restricted and prior authorization medications are marked "NF" or "Restricted" in the VA EHR drug file. Changes to the formulary affecting prescribing will be sent to the contractor electronically through Outlook messages. Non-formulary or restricted medications may be reviewed for approval with appropriate clinical justification by utilization of the electronic non-formulary/restricted medication consult request process in the VA EHR. The contractor is required to follow all national VA guidelines for the use of non-formulary, restricted, and prior authorization medications, and to support evidence-based VA cost savings initiatives undertaken by the local VA. These guidelines may be accessed in the VA EHR, Web links, PBM website or directly through the PBM website at [VA National Formulary](#). The contractor is also required to adhere to VHA Directive 1310, Medical Management of Enrolled Veterans Receiving Self-Directed Care from External Health Care Providers. **NOTE:** The contractor's providers shall enter documentation in the non-VA medication section of the VA EHR for any medication(s) patients are taking that are not issued by VA.

A patient's new allergy information shall be entered into the patient's record via the VA EHR. The specifics of the patient's allergy or adverse drug reaction, if known, shall be included in the documentation. VA Pharmacy is not permitted to dispense any prescriptions without documentation of a patient's allergies being listed in the chart (or documentation that no known allergies exist as appropriate).

The contractor shall be responsible for providing all necessary information for each provider with prescriptive authority to the VA Credentialing Office (or as designated by the VA parent facility).

New drug orders: The contractor shall ensure that at least 95% of all new drug order requests follow all VA National Formulary prescribing guidelines.

The contractor shall provide medication counseling to patients, family, or caregivers in accordance with state and federal laws and VHA requirements, including, but not limited to:

- Medication instructions regarding drug, dose, route, storage, what to do if dose is missed, self-monitoring drug therapy, precautions, common side effects, drug-food interactions, and medication reconciliation, and importance of maintaining an accurate and up-to-date list of all medications (including herbals and over-the-counter medications), along with any verbal and/or written instruction provided. Confirmation and documentation of patient/caregiver instruction and the patient's/caregiver's understanding of the instructions including telephone contacts shall be documented in the VA EHR Progress Notes.
- Instructions of VA refill process (VA patient handout).
- Instructions to Veterans and/or caregiver on the safe and appropriate use of medication-related equipment being supplied shall be documented in the Veteran's medical record.
- Instructions on VHA Handbook 1101.11(4) Coordinated Care for Traveling Veterans (or subsequent revisions thereto).
- Instructions on VHA Directive 1310 Medical Management of Enrolled Veterans Receiving Self-Directed Care from External Health Care Providers (or subsequent revisions thereto).

Medication Management: All medications and supplies used in the treatment of outpatients on premises are required to be stored and secured to meet compliance with VHA policies. Efforts should be made to limit the number of ward stock medications and supplies stored at the outpatient site of care. The contractor is responsible to ensure all medications are subject to routine inspection, as required by VA Pharmacy, proper storage (in a secure and locked location) and meet all VA policies for medication management. Contractor shall ensure all vaccines and medications requiring refrigeration or freezing are stored in pharmaceutical grade purpose-built refrigerators and freezers. Contractor will also ensure monitoring of the temperatures for medication refrigerators and freezers, United States Pharmacopoeia (USP), and VA standards. In addition, contractor shall ensure that vaccines and medications exposed to temperature excursions are not used for patient care.

The contractor shall inspect all medication storage sites to ensure that medications are being stored properly (e.g., under appropriate refrigeration, and USP800 compliant if required; externals separated from internals; expiration dates checked, etc.) and VA Medication Inspection Form (VA Form 10-0053) or local medical center form will be completed and signed by the contractor clinic management or designee and sent to the COR within 10 calendar days of the end of month. This information will be used in conjunction with the COR's quarterly evaluation of the contractor's performance. Follow-up on all recommendations identified and resolution of all identified discrepancies on the Medication Inspection Form will be completed in a timely manner by clinic personnel, as directed or requested by the VA Chief of Pharmacy.

Reports of Adverse Drug Events (ADEs) will be documented in the VA EHR, with the specifics of the event documented as outlined in local VA policy and forwarded to VA Pharmacy as they occur via encrypted E-mail to VA Chief, Pharmacy Service or Patient Advocate Office within two business days.

All medication errors and medication-related incidents shall be reported immediately to the Chief, Pharmacy Service or designee and submitted to the local VA Patient Safety on the local VA-approved Incident Report form.

Customer complaints regarding pharmacy services shall be addressed by the VA Pharmacy Service in collaboration with contract clinic management staff. Reports of such complaints shall be recorded and forwarded to the VA Chief, Pharmacy Service within two business days.

The contractor shall work in collaboration with VA Pharmacy Service when there are identified unique medication management needs of the patients and submit appropriate Nonformulary/Restricted/Prior Authorization consults in the VA EHR where appropriate for further review and completed within 96 hours. Examples of this include notification and management of patients that are taking medications that pose a medication safety concern or patients that are taking medications that require therapeutic substitution based on formulary or medication safety concerns. Contractor requirements will be further identified by VA governing bodies and VA Pharmacy.

The contractor shall provide the patient with an accurate, reconciled list of medication to include medications that the patient is receiving from the VA, medications that he takes from non-VA providers, and any OTC, herbal or alternative medications that the patient reports taking. The contractor shall meet all requirements of VHA Directive 1345 Medication Reconciliation (or subsequent revisions thereto) as well as any VA policy related to medication reconciliation. The contractor shall also provide monthly monitors on compliance with Medication Reconciliation per VHA Directive 2011-012 Medication Reconciliation (or subsequent revisions thereto), which is attached in Section D (attachments).

The contractor shall meet all requirements for anticoagulation management outlined in VHA Directive 1108.16 Anticoagulation Therapy Management (or subsequent revisions thereto) as well as local policy related to the management of patients on anticoagulation. Local policy is included in Section D (attachments).

The contractor shall provide quarterly, and annual anticoagulation quality assurance summaries as outlined by the local Pharmacy & Therapeutics Committee. For questions, please contact, POC, VA Anticoagulation Coordinator Program Manager and Anticoagulation Program Champion.

The contractor is required to enter all prescription orders using the VA EHR outpatient medication order entry option. The contractor shall include complete directions for the prescription ("PRN" alone is not acceptable), and shall include the indication for medication use, the appropriate quantity, and subsequent refills for the medication.

Clinical Pharmacy Services: CPS will be provided remotely from the Salem CBOC or from the Portland VA Pharmacy remotely.

5.4 Ancillary Support Services:

Radiology Services: Radiology will be provided either at the Portland VA Medical Center, Salem CBOC, or sent out by consult to Community Care providers.

Electrocardiogram (ECG) Services: ECGs shall be used that are interfaced with the VA EHR. MUSE-compatible EKGs shall be used which are interfaced with the VA EHR Imaging. This will be supplied by VA at cost to the contractor. EKGs are done by the Outpatient Site of Care and documentation will be sent electronically from the GE 5500 EKG machine directly into the VA EHR Imaging. When MUSE-compatible system is not available EKGs will be confirmed, interpreted and documented by the Contractor's licensed provider. The report will be scanned directly into the VA EHR Imaging by the Outpatient Site of Care. The EKGs will be confirmed and/or read by Contractor's providers.

Phlebotomist: The Contractor will provide staff members who is competent and authorized to draw labs as needed by direction of VA Licensed Provider (LP). Any contract site performing phlebotomy will have oversight by VA laboratory.

Laboratory Services: Contractor is responsible for: 1) Entering orders for laboratory tests into the VA EHR. Information concerning the laboratory tests are provided as an attachment in Section D. 2) Sending specimens to the VA Core Laboratory twice daily, prior to the Contractor's mid-day break period and after the close of business of the workday, except for those specified in this PWS. 3) Paying any costs of all lab work, except for lab work sent to the VA or emergency lab work sent to another site which has been authorized by the VA Communications Center and paying any costs associated with transportation of specimens to the VA and for arranging such transportation in a proper secure method and 4) ensuring that all courier service employees have completed VHA Privacy and HIPAA Focused Training 5) Ensuring the proper collection, collection supplies, and other preservation of specimens and providing appropriate collection containers that are compatible with the instrumentation and methodology used by the VA laboratory. Any actively enrolled Veteran will be able to use the contractor's laboratory.

Sample Collection: The contractor shall be responsible for entering laboratory orders into the VA EHR and sample collection. A listing of specimen collection requirements, including collection containers, are included in the Pathology and Laboratory Program Laboratory Information Manual provided in Section D (attachments).

The contractor shall provide all specimen collection materials and containers and may not purchase from the VA since the Federal Acquisition Regulation prohibits the purchase of supplies for resale.

Sample Processing/Preparation/Packaging: The contractor shall process, prepare, store, and package laboratory specimens in accordance with the requirements defined in the Pathology and Laboratory Program Laboratory Information Manual provided in Section D (attachments).

The contractor shall provide appropriate shipping containers and any packing material including wet and/or dry ice, if required. The contractor is responsible for any specimen preservatives required for specimen preservation. The contractor is responsible for packaging and transporting in a manner that ensures integrity of the specimen and proper safeguarding of protected health information. Frozen specimens shall be shipped on dry ice, if required.

The VA will not be responsible for the quality of laboratory test results obtained from specimens improperly collected, labeled, processed (centrifuged and aliquoted), stored, and/or transported by the contractor.

Specimen Transportation: The VA Portland Healthcare System has a courier contract that covers the specimen transportation for the VA Portland Healthcare System, including all the CBOCs. The contractor is responsible for proper safeguarding of protected health information. Specimens with a shipping manifest shall be delivered to the VA parent facility laboratory receiving area. The contractor shall be contacted to resolve any discrepancies identified on the shipping manifest.

The regulations governing the packaging and shipment of biological, infectious, and hazardous substances are primarily found in 49 CFR parts 100-185 and 397. The parts of 49 CFR specifically dealing with biological and infectious specimens are 107, 171, 172, and 173. All specimens, as well as any select agents and toxins that are identified, shall be packed, and shipped in accordance with the applicable guidance provided in 49 CFR and comply with all laws, regulations, and policies of federal, state, and local governments, the Centers for Disease Control (CDC), Department of Transportation (DOT), International Air Transport Association (IATA), and the Department of Veterans Affairs.

Hazardous Materials and Security Training: Contractor or courier service personnel transporting or shipping hazardous materials (hazmat) shall be aware of the potential use of hazmat for acts of terrorism and are required to have documented transportation security awareness training. As many biological specimens are now classified as hazmat, it is important for all personnel that are involved in the packing and shipping of laboratory specimens to have hazmat training. The contractor shall ensure that all personnel who deal with specimen transportation and shipment have the training and certification required by law and regulation.

Results: In accordance with VHA Directive 1088, Communicating Test Results to Providers and Patients, all test results requiring action must be communicated by the ordering provider, or designee, to patients no later than 7 calendar days from the date on which the results are available. For test results that require no action, results must be communicated by the ordering provider, or designee, to patients no later than 14 calendar days from the date on which the results are available. The Contractor shall provide the VA with the name, pager and telephone numbers of a LIP (physician, nurse practitioner, or physician assistant) at the Outpatient Site of Care to accept critical test results discovered on tests done by the VA. For critical results, the LIP must respond back to the VA within forty-five (45) minutes of the initial page or telephone call. The receiving LIP will document the results in the record and conduct a "read back" procedure to ensure accuracy of transmission and translation of all verbal results during normal business hours. The contractor shall determine a plan to fulfill critical test result procedures, per VA policy. VA will not be responsible for the failure of the Contractor to receive critically abnormal test results. Critical results must be reported to the clinician by the radiologist by telephone. Documentation of this notification, "who, when" must appear in the radiology report. For critical results that represent an imminent danger to the patient, the Contractor shall notify the patient immediately. See policy MCM 11-44 in section D (attachments) for additional requirements regarding communication of test results.

Ancillary, Point of Care, and Waived Testing Services:

Mandated POC testing includes: Urinalysis and Pregnancy testing. Outpatient Site of Care must have point of care or stat pregnancy testing at the same site of care.

The laboratory tests designated as waived under the Clinical Laboratory Improvement Amendments of 1988 and all amendments (CLIA'88, et al.), 42 CFR 493.15(b) and 493.15(c). In CLIA regulations, waived tests are categorized as "simple laboratory examinations and procedures that have an insignificant risk of an erroneous result." The Food and Drug Administration (FDA) determines which tests meet these criteria when it reviews manufacturer's applications for test system waiver. To perform waived tests, the contractor shall apply for and maintain a current CLIA Certificate. A copy of the CLIA certificate shall be provided to the VA Laboratory and National Enforcement Office. (A CLIA Certificate cannot be obtained through the VA.)

In addition, the Contractor must apply for and maintain a Oregon Department of Health Level II Clinical Laboratory Permit. In the performance of these tests, the Contractor must comply with the terms and requirements of the Ancillary Testing Policy, The Ancillary Testing Policy is included in Section D (attachments).

The Contractor must also adhere to VA standards/requirements as detailed in VA Handbook 1106.1 when performing ancillary laboratory tests. The results of all testing must be entered into the medical record through the laboratory software package in the VA EHR. The Contractor must take immediate action on any critical test result, immediately inform the VA, and document the action taken through the VA EHR. It is the Contractor's responsibility to maintain the test systems/instruments in proper working order. When necessary, the Contractor must send quality control records and test results to the Ancillary Testing staff for troubleshooting test system/instrument malfunction. The Contractor must address all questions concerning waived and point of care testing to the [REDACTED]

The contractor is required to use the same waived test systems/instruments; quality control and reagent used for ancillary testing performed at the VA. The VA parent facility will provide the test systems/instruments and reagents for contractor waived testing. See attachment in Section D for waived testing test systems/instruments, quality control, and reagent information. If the VA Ancillary Testing Program changes waived test systems/instruments, the VA parent facility will furnish the contractor with the new test systems/instruments to maintain the same standard of care. The contractor shall follow VA parent facility procedures regarding appropriate specimen collection and preservation guidelines, instrument calibration, quality control and remedial actions, equipment performance evaluation, temperature monitoring requirements, reporting of results, and performance of the test. See Section D for this information.

It is the contractor's responsibility to maintain the test systems/instruments in proper working order. When necessary, the contractor shall send quality control records and test results to the ancillary testing staff for troubleshooting test system/instrument malfunction. In the event of system/instrument failure, the ancillary testing coordinator will arrange for repair/maintenance. If required, the contractor shall provide a courier to transport instruments and/or reagents to and from the contractor to the VA.

The VA will purchase proficiency testing materials for the contractor, and the contractor shall comply with the Pathology and Laboratory Medicine, CAP and TJC requirements/regulations for testing proficiency materials and submitting results.

The contractor shall address all questions concerning the Ancillary Testing Program with the ancillary testing coordinator. The results of all testing shall be entered into the medical record through the laboratory software package in the VA EHR.

Mental Health Services:

Estimated Mental Health Workload:

It is estimated that approximately 10% of Veterans assigned to this clinic will require Primary Care Mental Health Integration services.

It is estimated that 30% of Veterans assigned to this clinic will require General or Specialized Mental Health services.

Mental Health Services Provided as Part of Primary Care:

Mental Health and Substance Use Screening and Care: As a part of standard primary care services, the contractor's staff shall provide screening and care for common mental health and substance use conditions, consistent with team member's clinical privileges, skills, scope of practice, position description, or functional statement. The contractor's staff shall:

- When due as alerted by the clinical reminder system, screen patients for depression, PTSD, suicide risk, alcohol use, and tobacco use.
- If primary screen for depression and/or PTSD is positive, the screener shall verbally communicate this to the PCP/LIP for further evaluation.
- If suicide risk screening is positive as determined by a Veteran's responses to the Columbia-Suicide Severity Rating Scale (C-SSRS), then timely completion of the Comprehensive Suicide Risk Evaluation (CSRE) is required within the same calendar day. Additional details related to the suicide risk and evaluation required processes are provided in the Suicide Prevention section of this document.
 - Provide counseling about smoking and tobacco use cessation. In addition to offering brief and intensive behavioral counseling for tobacco use treatment, all FDA-approved tobacco cessation medications shall be available for all adult patients using tobacco products. All patients seeking assistance with quitting tobacco use shall be offered a combination of pharmacotherapy and behavioral counseling and provided with information about the VA proactive quit line, Quit VET.
 - Provide brief alcohol counseling for positive screens on the Alcohol Use Disorders Identification Test (AUDIT-C).
 - Because population screening is not evidence-based for substance use conditions other than alcohol misuse and tobacco use contractor's staff will need to use targeted case-finding methods to identify patients who use illicit drugs or misuse prescription or over-the counter agents. These methods need to include evaluation of signs and symptoms of substance use in patients with other relevant conditions (e.g., other mental health disorders, hepatitis C, or HIV disease).
 - Provide care for patients with mild to moderate MH and SUD conditions, engaging Primary Care Mental Health Integration (PCMHI) providers, general and specialty MH providers, disease prevention specialists, substance use disorder (SUD) providers, or other providers as indicated.
 - For patients with SUD who decline referral to specialty SUD treatment, the contractor's staff shall continue to monitor patients and their substance use conditions. They are to utilize their interactions with the patient to address the substance use problems and to work with them to accept referrals. **NOTE:** Strategies that may enhance motivation to seek SUD specialty care include providing the patient easy-to-read information on the adverse consequences of drinking; having the patient identify problems that alcohol has caused; urging the patient to maintain a contemporaneous diary of alcohol use and the circumstances and consequences associated with it; and offer of frequent appointments with the patient. Interventions with SUD treatment-reluctant patients are always to be characterized by a high-degree of provider empathy.

In all MH services that the contractor provides, the contractor shall comply with VA policies. Non-compliance with these policies may result in the revocation of clinical privileges by the VA.

The contractor's mental health staff shall provide consultative advice to the contractor's primary care team, provide direct clinical care when indicated and maintain an ongoing relationship with the enrolled patient for the care and treatment of Veterans with mental health conditions. Services to be provided include screening and prevention for mental health and substance use conditions, psychopharmacology, referral for inpatient or residential care, direct care, case management, group therapy, individual therapy, medication management, outreach, telemental health or arranging and managing consultation for special emphasis and/or complex conditions (such as PTSD evidence-based psychotherapy, psychological testing, military sexual trauma). The contractor's mental health staff will also establish contact with VA if inpatient care is needed and serve as liaison between the VA and the contractor's primary care staff to coordinate primary care and general/specialty mental health treatment. The contractor's mental health staff shall assist the contractor's primary care team when a need for treatment or referral to community or VA resources is indicated for patients with mental health and substance use conditions.

Mental Health Same Day Access Requirements: The contractor shall provide same day MH access and same day MH evaluation. Any Veteran reporting or identified as being in crisis (including suicidality), will receive an immediate crisis response. Services shall be provided as follows:

- **Veterans New to MH:**

In Person: Any Veteran new to MH requesting or referred for care in person will be seen in person the same day by a Licensed Independent Provider (LIP) to screen for and address immediate care needs.

By Phone: Any Veteran new to MH calling to initiate care will be scheduled for an initial evaluation. Schedulers answering the phone will ask if the Veteran needs to speak with a provider immediately. If an urgent request is made or suggested, an immediate crisis response will be initiated and follow-up care will be provided, as needed. If an urgent response is not indicated, a LIP will call the Veteran back the same day or no later than the next calendar day.

- **Veterans Established in MH Care:**

In Person: Veterans established in mental health care self-identifying a need for attention will be seen in person the same day by a provider to address immediate care needs.

By Phone: Veterans established in mental health care may self-identify a need for urgent attention. Schedulers answering the phone will ask if the Veteran needs to speak with a provider immediately. If an urgent request is made or suggested, an immediate crisis response will be initiated, and follow-up care will be provided as needed. [If urgent response is not indicated, a provider will call the Veteran back the same day (or the next business day)].

Documentation should cover the requirements of the screening evaluation. This should include documentation of determination of urgency of mental health care needed and initiation of immediate crisis response if needed, identification of the appropriate setting for subsequent evaluation and treatment, treatment follow-up plan, provision of emergency contact information for mental health services (this can include the Veterans Crisis Line (VCL) Hotline number or a local facility contact number), and follow-up on any specific concerns or questions by the Veteran. A comprehensive MH diagnostic and treatment planning evaluation shall be completed within 30 calendar days of the same day evaluation.

MH Urgent/Emergent Services: If at any time a patient needs more intense services than those provided on site, the Contractor shall take steps to arrange transfer to VA; or if more urgent care is needed, to the nearest emergency room.

During normal business hours, transfer to VA can be arranged by calling [REDACTED] which is answered twenty-four (24) hours per day. The nurses or Administrative Officer of the Day will assist in arranging transfer to VA. If immediate consultation with a psychiatrist is needed, the staff can also call this number and request assistance. Patients with health-related questions may also be directed to call the [REDACTED] and follow the menu options.

VA Mental Health also maintains same day services. Patients shall be given specific directions to the location; the contractor shall call the location on the VA Campus and alert the personnel to expect the same day need. The patient shall be advised that they will be seen in the Initial Evaluation clinic the same day. The Contractor shall follow up to document that patient successfully arrived or did not arrive at the location and document the file accordingly. If the patient did not arrive, the Contractor shall make phone contact with the patient to determine if the patient requires further direction or assistance.

Workplace Violence Prevention: All contractor staff members shall meet current VA training requirements for VHA employees in prevention and management of disruptive behavior as outlined in VHA Directive 1160.0.8 VHA Workplace Violence Prevention Program. All contractor staff members shall report disruptive behavior events in the Disruptive Behavior Reporting System (DBRS).

Military Sexual Trauma-Related Care: VHA Directive 1115(1), Military Sexual Trauma (MST) Program (or subsequent revisions thereto) establishes VA requirements related to sexual harassment and assault experienced during military service, also known as "military sexual trauma" (MST.) The VA mandates screening every former Service member (not just those with Veteran status) seen for health care for experiences of MST. This involves asking the patient whether they experienced sexual harassment or sexual assault during military

service, using specified screening language. All former service members, of all genders, seen in contractor's sites shall be screened for experiences of MST. This shall be done using the MST screening language in the VA EHR and the patient's response shall be documented. Screening is to be conducted in appropriate clinical settings by providers with an appropriate level of clinical training, as described in VHA Directive 1115(1); screenings are not to be conducted by administrative associates. VA provides free treatment for all mental and physical health conditions related to a patient's experience of MST and eligibility for this care is expansive, as described in VHA Directive 1115(1). If a patient screens positive for MST, they shall be asked if they would like a referral for MST-related treatment. If they respond yes, they shall be connected with mental health and/or medical services, as appropriate to meet their MST-related health concerns. If the patient declines such services, this shall be documented as well. More information can be obtained by calling the [REDACTED] and asking for the Military Sexual Trauma Coordinator. A list of MST Coordinators is also available at: <https://www.mentalhealth.va.gov/msthome/vha-mst-coordinators.asp> **NOTE:** Contractor sites with 5,000 or more patients shall provide care for MST-related health conditions on-site. Contractor shall ensure that there are a sufficient number of clinicians able to provide specialized mental health care for conditions related to MST to adequately meet the demand for care. Types of services and the manner in which services shall be delivered is described in VHA Directive 1115(1). Contractor's providers shall meet education requirements as specified in VHA Directive 1115.01(1), MST Mandatory Training and Reporting Requirements for VHA Mental Health and Primary Care Providers, including completion of mandatory training on MST by mental health and primary care providers.

Disaster Preparedness: All contractor sites shall have a designated Mental Health Disaster POC, who can serve as a member of the VA parent facility's Disaster Response Team. Training for the mental health disaster POC needs to be coordinated with training for other disaster response clinicians and emergency management teams at the VA parent facility and VISN levels.

Mental Health No-Show and Cancellation Procedures: Contractor shall follow guidance in the Appointment Management Business Rules SOP listed in Appendix B of VHA Directive 1230 "Outpatient Scheduling Management" for cancellations/no-show process business rules. Contractor shall have a local standard operating procedure for mental health no-shows which includes a mechanism for supervisors to audit compliance by performing chart reviews.

If contact with the Veteran is unsuccessful, contacting local law enforcement for assistance is recommended when risk for harm is deemed to be imminent. Consideration for contacting local law enforcement should be based upon the documented clinical determination of imminent risk, which applies to all Veterans regardless of High-Risk status.

Extended MH Hours: The contractor shall meet the following requirements for extended hour clinics if indicated by the VA.

Mental Health Support Services: The contractor shall provide the following clerical support services for mental health providers (including VA providers if VA provides MH services onsite) at the CBOC:

- Schedule and reschedule patients for mental health providers assigned to the clinic and the Quit Smart program.
- Check in patients for the above clinics.
- Check out patients/encounters for the above clinics.
- Utilize the Insurance Capture Buffer to record Veteran insurance information.
- Place reminder calls to patients no later than one day prior to their appointment.
- Call patients to cancel and reschedule appointments when the provider is unable to see the patient.
- Receive and screen phone calls for the above clinics and providers.
- Process all consults for the above clinics.
- Provide support for the telemental health appointments including scheduling, set-up.
- Participate in triaging secure messages.

Mental Health Services Provided by MH Staffing: Mental health and substance use disorder treatment services are described in VHA Directive 1160.01 with additional references to additional directives, memos and guidance indicated within that directive. There are also unique requirements related to clinical management and administration, including but not limited to scheduling, cancelling appointments, documentation, and follow up that are outlined in VHA directives published by the Office of Integrated Veteran Care to include VHA Directive

1230, Outpatient Scheduling Management, VHA Directive 1231(4), Outpatient Clinic Practice Management, VHA Directive 1232(5), Consult Processes and Procedures.

Primary Care-Mental Health Integration (PCMHI): Contractor shall provide PCMHI services in accordance with VHA Directive 1160.01. Per VHA Directive 1160.01, PCMHI is a required component of the PACT. Where primary care services are offered, PCMHI services shall include co-located collaborative care (CCC) providers who attend PACT huddles and provide consultative advice, brief assessment and problem-focused treatment, and mental health collaborative care managers (CoCM) who use the evidence-based mental health collaborative care management model to provide longitudinal follow-up and decision support for specific mental health conditions. VA Mental Health and Social Work Services will have 1 day a month in clinic where VA patients can be seen in person.

General and Specialty Mental Health Services: General and Specialty MH services will be provided by Telehealth referral to VAPORHCS Mental Health Division. The Contractor will facilitate communication and warm handoff when appropriate. The VA staff provider will be the referring provider.

Evidence-based Psychotherapy for PTSD: All Veterans with PTSD shall have access to Cognitive Processing Therapy (CPT) or Prolonged Exposure Therapy as designed and shown to be effective. Contractor shall provide evidence-based psychotherapy when it is clinically indicated for patients.

Evidence-based Psychotherapy for Depression and Anxiety Disorders: All Veterans with depression or anxiety disorders shall have access to cognitive behavioral therapy (CBT), acceptance and commitment therapy (ACT), or interpersonal therapy. Contractor shall provide evidence-based psychotherapy when it is clinically indicated for patients.

Evidence-based Somatic Therapies: (1) Evidence-based somatic treatments and pharmacotherapy shall be provided when indicated for mood disorders, anxiety disorders, PTSD, psychotic disorders, SUD, dementia, and other cognitive disorders. Such care shall be consistent with current VA clinical practice guidelines and informed by current scientific literature. **NOTE:** Current VA clinical practice guidelines can be found at: <http://www.healthquality.va.gov>. (2) Care can be provided by a physician or appropriately credentialed and supervised advanced practice nurse or physician assistant and may be provided using telemental health when appropriate. (3) Because in many cases combinations of psychosocial, psychopharmacological, and other somatic treatment has been shown to be more effective than a single intervention alone, Veterans shall have access to combined treatment when indicated. Pharmacotherapy and somatic treatments need to be coordinated with other psychosocial or psychological interventions patients may be receiving, as well as primary and other specialty medical care.

Electroconvulsive Therapy (ECT): Veterans shall have access to ECT and other somatic treatments in the VISN in which they receive care. ECT shall be provided when it is clinically indicated consistent with VA clinical practice guidelines found at: <https://www.healthquality.va.gov/guidelines/MH/mdd/> as well as those of the American Psychiatric Association. Staff needs to be knowledgeable about the current scientific literature and ECT needs to be coordinated with other psychosocial, psychological, psychopharmacological, and medical care that patients may be receiving.

Psychotherapy Groups: The mental health staff shall identify situations where group therapy may be beneficial to Veterans and their families. Groups should be time limited (10-12 sessions) and goal directed. Psychotherapy groups can be closed or cohort-based, or they can continually be open to new members. There are several arguments in favor of closed groups. However, waiting for the formation of a new group can lead to delays in the initiation of treatment. Accordingly, closed or cohort-based groups are allowable in VHA facilities only when the facility's care system ensures that they do not lead to the denial of care for any Veteran, and that waiting for the start of a new psychotherapy group does not lead to delays in the implementation of care. Patients awaiting the start of a therapy group shall be monitored on an ongoing basis. Their care needs shall be evaluated, and alternative treatments shall be implemented when needed, for example: When patients are a danger to themselves or others, when they are experiencing increasing degrees of impairment, or when they are suffering from severe symptoms. Waiting periods need to be utilized to provide pre-group preparation to enhance the experience and benefits of group treatment. Whenever patients need to wait for the start of a group, they shall be offered an appropriate form of interim treatment.

Family Education and Involvement: Contractor shall provide family education, family consultation, family psychoeducation, and marriage and family counseling for Veterans who need these services as part of their overall mental health treatment.

Providers need to discuss family involvement in care with all patients with an ICD 10 mental health diagnosis or as clinically indicated, at the time of mental health treatment initiation or inpatient or residential treatment discharge, at the time of a significant change in the treatment plan, and at a minimum, annually. The treatment plan needs to identify at least one family contact, or the reason for the lack of a contact (e.g., absence of a family, Veteran preference, lack of consent).

Family consultation, family education, family psychoeducation, or marriage and family counseling within existing statutory and regulatory counseling authority for Veterans with an ICD 10 mental health diagnosis or as clinically or as clinically indicated shall be provided for those who need them. This can be provided in the clinic, by telehealth, or by a community provider. Trained staff shall be available to provide these services.

Opportunities for family consultation, family education, family psychoeducation or marriage and family counseling within existing statutory and regulatory counseling authority shall be available to all Veterans with an ICD 10 mental health diagnosis or as clinically indicated.

Social Skills Training: Social skills training is an evidence-based psychosocial intervention that shall be provided when clinically indicated and shall be available to all Veterans with serious mental illness (SMI) who would benefit from it.

Peer Support Services: Contractor sites treating 10,000 patients or more shall provide individual or group peer support services for Veterans when this service is clinically indicated and included in the Veteran's treatment plan. Other contractor sites shall make peer support services available for Veterans when it is clinically indicated and included in the Veteran's treatment plan. In these cases, peer support services may be made available by telemental health, referral to VA facilities that are geographically accessible, or by referral to community-based providers.

Compensated Work Therapy (CWT), Transitional Work, and Supported Employment: Contractor shall provide information about the CWT program and criteria for participation shall be made available to Veterans. Whether a particular patient's participation in the CWT program would be appropriate is a clinical determination to be made by the responsible clinician, consistent with CWT program criteria. Offer CWT with both transitional work and supported employment services for Veterans with occupational dysfunctions resulting from their mental health or other complex health conditions, or who are unsuccessful at obtaining or maintaining stable employment patterns due to mental illnesses or physical impairments. Participation in the CWT program shall be available to any Veteran receiving care through VA whom VA finds would benefit therapeutically from participation.

Substance Use Disorders (SUD): Contractor shall provide appropriate services addressing the broad spectrum of substance use conditions including but not limited to tobacco use disorders. Contractor shall provide behavioral counseling for smoking and tobacco cessation. In addition to offering brief and intensive behavioral counseling for tobacco use treatment, all FDA-approved tobacco cessation medications shall be available for all adult patients using tobacco products. Contractor shall provide services for tobacco-related disorders to those who need them in a manner that is consistent with the 2008 U.S. Public Health Service Clinical Practice Guidelines which can be found at: <https://www.ahrq.gov/prevention/guidelines/tobacco/index.html>

During new patient encounters and at least annually, patients in primary care, appropriate medical specialty care settings, and mental health care services need to be screened for tobacco use.

All patients seeking assistance with quitting tobacco use shall be offered a combination of pharmacotherapy and behavioral counseling and provided with information about the VA proactive quit line, Quit VET.

To the greatest extent practicable and consistent with clinical standards, interventions for substance use conditions shall be provided when needed in a fashion that is sensitive to the needs of Veterans and of specific populations including, but not limited to: the homeless; ethnic minorities; women; geriatric patients; and patients with PTSD, other mental health conditions, and patients with infectious diseases (human immunodeficiency virus (HIV), acquired immune deficiency syndrome (AIDS), and hepatitis C); TBI; and SCI.

Services addressing substance use conditions can be provided in VA facilities in SUD specialty care, in primary care and other medical care settings (especially in programs that integrate mental health and primary care), through programs integrating treatment for co-occurring mental health disorders and SUD (dual diagnoses) in mental health settings, or in community settings through sharing agreements, contracts, or VA Community Care to the extent that the Veteran is eligible. Regardless of the setting, the process of care shall recognize the principle that SUDs are, in most cases, chronic or episodic and recurrent conditions that require ongoing care.

Consistent with the National Voluntary Consensus Standards for Treatment of Substance Use Conditions endorsed by the National Quality Forum (2007) and the 2021 VA-DOD Clinical Practice Guidelines for Management of Patients with SUD, the following services shall be readily accessible to all Veterans when clinically indicated:

During new patient encounters and at least annually, patients in primary care, appropriate medical specialty care settings, and mental health care services need to be screened with the AUDIT-C.

Because population screening is not evidence-based for substance use conditions other than alcohol misuse and tobacco use; primary care, medical specialty, and mental health services need to use targeted case-finding methods to identify patients who use illicit drugs or misuse prescription or over-the-counter agents. These methods need to include evaluation of signs and symptoms of substance use in patients with other relevant conditions (e.g., other mental health disorders, hepatitis C, or HIV disease).

Patients who have a positive screen for, or an indication of, a substance use problem shall receive further assessments to determine the level of misuse and to establish a diagnosis. Diagnostic assessment can be conducted by primary care or other medical providers, mental health providers, or specialists in substance use disorders. Patients diagnosed with a substance use illness shall receive a multidimensional, bio-psychosocial assessment to guide patient centered treatment planning for substance use illness and any coexisting mental health or general medical conditions.

All patients who screen positive on the AUDIT-C shall receive brief intervention (brief motivational counseling by a health care worker with appropriate training in this area) and referral to treatment when indicated (depending upon the severity of the condition and the patient's preferences). For patients who are identified with alcohol use disorder, further treatment shall be offered, with documentation of the offer and the care provided.

All health care providers caring for an individual Veteran shall systematically promote the initiation of treatment and the ongoing engagement in care for patients with SUD. For patients with SUD who require SUD specialty care but decline referral to specialty SUD treatment, providers in primary care, mental health, or other settings need to continue to monitor patients and their substance use conditions. They are to utilize their interactions with the patient to address the substance use problems and to work with them to accept referrals. **NOTE:** Strategies that may enhance motivation to seek SUD specialty care include providing the patient easy-to-read information on the adverse consequences of drinking; having the patient identify problems that alcohol has caused; urging the patient to maintain a contemporaneous diary of alcohol use and the circumstances and consequences associated with it; and frequent appointments with the patient. Interventions with SUD treatment-reluctant patients are always to be characterized by a high-degree of provider empathy. Motivational counseling needs to be available to patients in all settings who need it to support the initiation of treatment.

When patients are evaluated as appropriate and are willing to be admitted to inpatient or residential treatment settings for substance use conditions, but admission to those settings is not immediately available, interim services shall be provided as needed to ensure patient safety and promote treatment engagement.

All contractor sites shall make medically supervised withdrawal management available by referral as needed, based on a systematic assessment of the symptoms and risks of serious adverse consequences related to the withdrawal process from alcohol, sedatives or hypnotics, or opioids. Although withdrawal management can often be accomplished on an ambulatory basis, contractor sites shall make inpatient withdrawal management available by referral for those who require it. Withdrawal management alone does not constitute treatment for dependence and shall be linked with further treatment for SUD. Appointments for follow-up treatment shall be provided within 1 week of completion of medically supervised withdrawal management. Coordinated and intensive substance use treatment programs shall be available for all Veterans who require them to establish early remission from the SUD. These coordinated services can be provided through either or both following:

- Intensive outpatient services at least 3 hours per day at least 3 days per week in a designated program delivered by staff with documented training and competencies addressing SUD.
- An MH RRTP, either in a facility that specializes in SUD services or a SUD track in another MH RRTP that provides a 24/7 structured and supportive residential environment as a part of the SUD rehabilitative treatment regimen.

Multiple (at least two) empirically validated psychosocial interventions shall be available for all patients with substance use disorders who need them, whether psychosocial intervention is the primary treatment or as an adjunctive component of a coordinated program that includes pharmacotherapy. At least one of the empirically validated psychosocial interventions shall be either individual or group cognitive behavioral interventions for substance use disorders.

Empirically validated interventions include motivational enhancement therapy, cognitive behavioral therapy for substance use disorders, 12-step facilitation counseling, contingency management, and SUD-focused behavioral couples counseling or family therapy.

Pharmacotherapy with approved, appropriately- regulated medications for opioid use disorder (e.g., Buprenorphine, methadone, or injectable extended-release naltrexone) shall be available to all patients diagnosed with opioid dependence for whom it is indicated and for whom there are no medical contraindications. It needs to be considered in developing treatment plans for all such patients. Psychosocial treatment and support shall be offered along with pharmacotherapy when indicated. Medications for opioid use disorder can be delivered in either or both following settings:

- Opioid Treatment Program (OTP). This setting of care involves a formally approved and regulated opioid treatment clinic within which patients receive maintenance treatment using methadone or buprenorphine.
- Office-based Buprenorphine Treatment and injectable extended-release naltrexone can be available in non-specialty settings (e.g., primary care).
- The VA would like to see Veteran's via Telehealth or Vtel for the Opioid Treatment Program (OTP).

Pharmacotherapy with an evidence-based treatment for alcohol use disorder shall be offered and available to all adult patients diagnosed with alcohol use disorder and without medical contraindications.

Psychosocial treatment and support shall be offered along with pharmacotherapy when indicated. Patients with substance use disorder shall be offered long-term management for substance use disorder and any other coexisting mental health and general medical conditions. The patient's condition needs to be monitored in an ongoing manner, and care needs to be modified, as appropriate, in response to changes in their clinical status.

When PTSD or other mental health conditions co-occur with substance use disorders, evidence-based pharmacotherapy and psychosocial interventions for the other conditions need to be made available where there are no medical contraindications, with appropriate coordination of care. Substance use disorder shall never be a barrier for treatment of patients with other mental health conditions. Conversely, other mental disorders shall never be a barrier to treating patients with substance use disorders. When it is appropriate to delay any specific treatment, other care shall be provided to address the clinical needs of the Veteran. Consultations from specialists in substance use disorders or dual diagnosis shall be available when needed to establish diagnoses and plan treatment.

Suicide Prevention: The contractor shall complete the following suicide prevention programming requirements described in this section to include requirements for suicide risk screening and evaluation, lethal means safety education and counseling training for relevant staff, reporting of suicidal behaviors, safety planning with patients at risk of suicide, management of patients identified through the REACH VET program and coordinating with the parent facility suicide prevention coordinator to manage patients with a high risk of suicide patient record flag.

1. Suicide Risk Screening and Evaluation: In addition to universal screening for suicide (see Mental Health and Substance Use Screening and Care section of this document), which is required annually, the contractor shall complete screening for suicide in the following setting specific locations:

- Outpatient Mental Health: C-SSRS screener during intake evaluation as clinically indicated thereafter. If the C-SSRS screen is positive, the CSRE shall be completed.

- Sleep Clinic: C-SSRS screener at referral or intake; C-SSRS screener shall be completed during intake evaluation if > 30 calendar days from referral as clinically indicated thereafter. If the C-SSRS screen is positive, the CSRE shall be completed.
- Pain Clinic: C-SSRS screener at referral or intake; C-SSRS screener shall be completed during intake evaluation if > 30 calendar days from referral as clinically indicated thereafter. If the C-SSRS screen is positive, the CSRE shall be completed.

The contractor shall also complete screening for suicide when clinically indicated. The universal and setting-specific requirements provide a general approach for completion of the suicide risk screening and evaluation protocols. There may be additional clinical situations, such as when a patient presents with a new behavioral health concern, when use of the C-SSRS and/or the CSRE is indicated. Contractor shall complete 100% of required universal and setting-specific screenings and CSREs and use clinical judgment for screening when clinically indicated. Contractor shall have processes in place to facilitate timely completion of the CSRE. The CSRE should be completed and documented on the same day as a positive C-SSRS screen in all ambulatory care settings.

2. Lethal Means Safety (LMS) Education and Counseling Training: In addition to completion of all mandatory training as per section Training (VA Mandatory/ACLS/BLS) of this document, the contractor, to include all clinical staff, shall complete Talent Management System (TMS) course VA34560 VA TMS course VA34560 - Lethal Means Safety Education and Counseling as a one-time training. All clinical staff shall complete the training within 90 calendar days of entering their position. This LMS training course includes directions for clinical staff on how to work with Veterans and their friends and family to facilitate LMS during high-risk periods.

3. Suicide Behavior and Overdose Reporting: The contractor shall:

- Report all self-directed violence (SDV) behaviors of suicidal and undetermined intent, and all overdose events, regardless of suicidal intent, that have occurred within the past 12 months from the date staff have been notified, by completing the national standardized note templates (i.e., Suicide Behavior and Overdose Report (SBOR) or Comprehensive Suicide Risk Evaluation (CSRE)).
- Notify the parent facility Suicide Prevention Team of suicidal and undetermined SDV behaviors upon completion of the SBOR or CSRE.
- Perform in coordination with the parent facility a data-based risk review by an interdisciplinary team in place to ensure that accidental and undetermined overdose events are reviewed (e.g., overdose review team) with a focus on engaging patients in treatment, including increased frequency of mental health or substance use disorder visits post overdose, as per requirements noted in Appendix F, VHA Directive 1160.04, VHA Programs for Veterans with Substance Use Disorders.

Note: See Department of Veterans Affairs (VA) Suicide Prevention Safety Plan and Suicide Behavior and Overdose Report (SBOR) Templates Staff Specific Guidance attachment for which staff may complete the SBOR.

4. Suicide Prevention Safety Planning: The contractor shall complete and document safety plans with patients, when clinically appropriate or pursuant to requirements identified elsewhere within this document and provide a copy of the safety plan to those patients. (**NOTE:** See Department of Veterans Affairs (VA) Suicide Prevention Safety Plan and Suicide Behavior and Overdose Report (SBOR) Templates Staff Specific Guidance attachment for which staff may complete the Suicide Prevention Safety Plan.) Safety plans shall be completed utilizing the national suicide prevention safety plan note titles and associated templates in the electronic health record as follows:

- For patients who have a new or updated safety plan, the Suicide Prevention Safety Plan note template should be completed.
- For patients who have a current safety plan that has been reviewed and has not been modified, the Suicide Prevention Safety Plan Review/Decline note template should be completed.
- For patients who decline completion of a safety plan, the Suicide Prevention Safety Plan Review/Decline note template should be completed, including documented efforts made by staff to develop or update the safety plan and the patient's reason for declining, in the "Reason" section of the note.
- For all sites using the Oracle Cerner EHR platform, the VA Safety Plan Power Form shall be used.

5. Recovery Engagement and Coordination for Health Veterans Enhanced Treatment (REACH VET):

The contractor shall complete the following for REACH VET identified patients within 14 calendar days of identification of patients by the REACH VET model:

- Work with the VA parent facility REACH VET coordinator to identify an appropriate REACH VET provider for each patient identified by the REACH VET model.
- If a contractor's provider is identified as the REACH VET provider, the contractor's provider shall review care provided for patients identified by the model, consider opportunities to enhance care, outreach to the patient to provide opportunities to modify the care plan, and use the national, standardized REACH VET PROVIDER progress note to document completed steps in the EHR.

(NOTE: See REACH VET documents in Section D (attachments) for additional information.

6. High Risk for Suicide Patient Record Flags (HRS-PRF): The contractor shall comply with all requirements of identification, notification and management of patients with an HRS-PRF, to include collaboration with the parent facility Suicide Prevention Coordinator(s) (SPC) pursuant to VHA Directive 1166, Patient Record Flags to include:

- Identification of patients who may be eligible for HRS-PRF placement.
- Referral of patients to the parent facility SPC for consideration of HRS-PRFs.
- Clinical management of patients with HRS-PRF to include safety planning, increased monitoring and frequency of mental health visits, and follow up for missed appointments.
- Clinical review of patients' suicide risk in conjunction with parent facility SPC(s) for consideration of HRS-PRF inactivation.

Homeless Programs: To ensure the availability of outreach and referral services to homeless Veterans, all contractor sites shall designate at least one outreach specialist, usually a clinical social worker, to provide services to homeless Veterans. Contractor sites with 10,000 or more patients shall have a dedicated specialist. In smaller sites serving less than 10,000, this may be a collateral assignment. All Veterans who are homeless, or at risk for homelessness, shall be offered shelter through collaborative relationships with providers in the community. Contractor staff shall ensure that homeless Veterans have a referral for emergency services and shelter or temporary housing. To the extent that it is possible under existing legal authority, facilities shall facilitate the Veteran's transportation to the shelter or temporary housing.

VA will provide information to contractor about collaborative formal, or informal, agreements with community providers for shelter, temporary housing, or basic emergency services and support them in working together to allow appropriate placement for Veterans together with their families when they are homeless or at risk of homelessness. VA will provide information to contractor about placement opportunities in grant and per diem program, a VA domiciliary, another VA MH RRTP, or other care settings that provide needed services. **NOTE:** Eligibility criteria may differ between several types of programs.

Use of emergency shelter services should generally not exceed 3 days and is only to be used as a last resort. Within that period, homeless outreach staff or other qualified clinical staff shall evaluate the Veteran's clinical needs and refer or place the Veteran for treatment and rehabilitation in therapeutic transitional housing, a MH RRTP, or another appropriate care setting. When longer stays in emergency shelters are unavoidable, this shall be documented in the medical record; in these cases, ongoing case management, assessment and evaluation, and referral services shall continue until more stable arrangements for transitional housing providing treatment or rehabilitation have been made.

Each VA medical center that has a designated grant and per diem-funded program in its area is responsible for designating a grant and per diem liaison. Each liaison is to provide case management services, as necessary, for grant and per diem patients, facilitate required quarterly reviews and annual inspections of grant and per diem facilities, and provide oversight and monitoring of the grant and per diem funded program as outlined in VHA Directive 1162.01. The Grant and Per Diem Liaison can be contacted [REDACTED]

Each VA Medical Center that has a designated Health Care for Homeless Veterans (HCHV) contracted residential services (CRS) program is responsible for designating a HCHV CRS liaison. Each liaison is responsible for

facilitating admissions, monitoring Veteran progress, providing contractor oversight and inspections. Please reference VHA Directive 1162.04 or contact your local VA parent facility for more information about the HCHV CRS Program.

Department of Housing and Urban Development (HUD)-VA Supported Housing (VASH) programs have been established in areas that have a high concentration of homeless Veterans. Through local public housing authorities, HUD provides rental assistance vouchers to homeless Veterans referred by VA case management staff for permanent housing. VA provides case management and other clinical services to Veterans in this program. When appropriate and available, the housing vouchers can be provided to Veterans together with their families. The dedicated outreach specialist at the contract site is required to reach out to the homeless services leadership at the VA parent facility for more information about the HUD-VASH Program, resources in their catchment, and how to connect Veterans with housing insecurity or homelessness with these resources.

Integrating Mental Health Services in the Care of Older Veterans: Older Veterans experience a wide range of behavioral and mental health concerns, including continuity/recurrence of earlier life mental disorders or onset of new challenges in late life. Guiding principles for providing mental health services for older Veterans include engagement of family or other caregivers as appropriate; communication and coordination with health care providers; assessment and optimization of Veteran functioning and participation in decision-making; and, when Veterans lack capacity for making certain decisions, engagement of surrogate decision-makers in care planning.

Contractor shall have the capacity to provide evaluation of medical decision making and independent living capacities. The contractor is required to reach out to the Geriatric and Extended Care service leadership at the parent facility for more information about the VA programs and services to meet the mental health needs of their local and regional older adult populations (e.g., neuropsychological evaluation, geriatric mental health clinics, family caregiver support for older Veterans with dementia or mental illness), make referrals as needed, and to inform the local medical center about unmet mental health needs of older Veterans.

Specialized PTSD Services: Veterans with PTSD can be treated in specialized PTSD services, general mental health services, or primary care. All contractor sites (i.e., CBOC) shall:

- Have the capacity to provide diagnostic evaluations and treatment planning for PTSD through full- or part-time staffing or by telemental health with parent VA medical centers.
- Contractor sites seeing more than 1,500 unique Veterans each year shall provide mental health treatment services, including evidence-based psychotherapies for PTSD for those who need them per VHA Directive 1160.3.
- When contractors who see less than 1,500 unique Veterans are within 1 hour of other VA facilities, they may make services for PTSD available to those who need them by referral to these other facilities.
- When there are no nearby facilities, smaller contractor sites shall provide needed services by telemental health, or by referral to the VA parent facility to the extent that the Veteran is eligible.
- Make PTSD clinical team (PCT) or PTSD specialist available for consultation or care for Veterans who may have PTSD, either on site, by referral to nearby VA medical centers, or by telemental health.
- All PCT or PTSD specialist programs shall be able to address the care needs of Veterans with both PTSD and SUD. These needs can be addressed in two ways with:
 - Distinct PTSD dual diagnosis programs or
 - Tracks that include providers with specific expertise in both PTSD and SUD.
- Structures, processes, and formal mechanisms to support the coordination of care for PTSD with that provided in SUD programs. These may include specialized programs of care management for these patients.
- Care of the intensity available in a PTSD RRTP or MH RRTP needs to be available to all Veterans receiving care from VHA to the extent that it is clinically indicated.

5.6 Specialty Care Services: The contractor shall place a consult to refer Veterans to the VA for specialty care services which includes but not limited to Podiatry Services, Physical Therapy Services and Physical Therapy Support Services.

5.7 Telehealth Services Requirements: This site has a requirement to support and provide telehealth services to VA patients. The contractor shall comply with all requirements outlined in Telehealth Conditions of Participation, Conditions of Participation Standards, Criteria and Evidence (Narrative). The contractor shall implement telehealth services using guidance provided within VHA's Office of Connected Care Telehealth

Manual and associated Asynchronous Store and Forward Telehealth Manual, Synchronous Telehealth Manual, Remote Patient Monitoring - Home Telehealth Manual and Telehealth Business Standard Operating Procedures, provided in the attachments section.

Required Telehealth Agreements: The contractor shall support and provide telehealth services and ensure the completion of a telehealth services agreement (TSA) for telehealth conducted between facilities. Telehealth space requirements are outlined in Space and Equipment Requirements for Telehealth” section of this PWS.

Telehealth Training for Staff: The contractor shall ensure that clinic staff complete the relevant VA telehealth training(s) including telepresenter training for staff assuming this role. In addition, to ensure that clinic staff are aware of the processes and actions necessary in the event of a patient emergency during video-to-clinic visits (e.g., acute medical event, violence, or threat of self-harm), VA’s telehealth-based emergency handoff procedures training is a requirement for staff participating in these types of telehealth visits. Regular knowledge checks of these processes with clinic staff are required to confirm that the specified requirements to manage clinic-based emergencies for telehealth are being met.

TMS links: [Emergency Training](#) and [Telehealth Training Plans by Role](#)

Telehealth Authorities of Contracted Staff: It is the responsibility of the contractor to ensure that telehealth services provided by contractor’s staff meet all clinical, technology, business, regulatory and legal requirements. Not all clinical, technology, business, regulatory and legal aspects of telehealth that apply to VA and VA practitioners will apply to contractor’s staff.

Space and Equipment Requirements for Telehealth: The contractor is required to follow VA requirements and processes for space and equipment when delivering telehealth services and shall be prepared to offer synchronous and asynchronous services by telehealth. Designated space for asynchronous telehealth services is required; for synchronous services, space may be interchanged with office space for in-person care, if necessary, equipment and connectivity requirements are met. The VA will provide all video conferencing equipment, peripherals, and asynchronous telehealth equipment (e.g., teledermatology camera, eye imaging equipment) and all associated information technology equipment and access to the VA confidential data network. The VA will maintain the VA-provided telehealth equipment. The following rooms have been designated for telehealth services at this site.

Networking Capability and Bandwidth for Telehealth: VA will also provide the networking capability to support the telehealth equipment. Sufficient bandwidth is required for satisfactory communication. Telehealth services will be conducted over a proprietary VA telehealth network that initially includes Tele-Video Conference Equipment, all associated Information Technology Equipment and T-1 access to the VA confidential data network.

Telehealth Services to be Offered at this Site: It is anticipated the VA will utilize telehealth services at this site for up to 10 hours per week during Monday through Friday from 8 a.m. to 5p.m. Telehealth staff will be available Monday through Friday from 8 am to 5 pm as needed.

The contractor shall provide the following telehealth services, which may be added or removed as needed throughout the duration of the contract.

Teleeye Care Services: The contractor shall provide teleeye imaging services for a target population of patients, to include those at risk of diabetic retinopathy, glaucoma, and macular degeneration. The contractor’s primary care providers (PCPs) will determine, based on the VA EHR eye clinic records or patient eye history documented in the VA EHR, which patients need to be imaged. The contractor will provide support for teleeye care services, providing staff to arrange appropriate scheduling, time and space for the Veteran, and technical support as needed including following national training, processes and workflows to acquire retinal images, intraocular pressure and other data from patients with VA provided equipment and training. The referring provider will be responsible for reporting results to the patient and following any recommended care plans or needed orders or consults.

Teledermatology Services: The contractor shall provide teledermatology services. Contractor’s PCPs and imagers (whether TCTs, nurses, mid-level provider or MDs) will complete all training required as specified by VA for asynchronous telehealth. The TCT will clean/maintain equipment and request maintenance/repair, beyond user-level, from the Office of Connected Care Help Desk (OCCHD) and if directed, VA Healthcare

Technology Management (HTM). The referring provider will be responsible for reporting results to the patient and following any recommended care plans or needed orders or consults.

Telespirometry Services: The contractor shall provide telespirometry services. Contractor's PCPs and technicians (whether TCTs, other technicians, nurses, mid-level provider or MDs) will complete all training required as specified by VA for asynchronous telehealth. The contractor will use compatible equipment with the VA parent facility PFT lab. The technician will clean/maintain equipment and request maintenance/repair, beyond user-level, from the Office of Connected Care Help Desk (OCCHD) and if directed, VA Healthcare Technology Management (HTM). The referring provider will be responsible for reporting results to the patient and following any recommended care plans or needed orders or consults.

Telesleep Services: The contractor shall provide telesleep services through synchronous and asynchronous telehealth, including distribution and data download of home sleep apnea testing devices. Contractor's PCPs and technicians (whether TCTs, other technicians, nurses, mid-level provider or MDs) will complete all training required as specified by VA for asynchronous telehealth. The technician will clean/maintain equipment and request maintenance/repair, beyond user-level, from the Office of Connected Care Help Desk (OCCHD) and if directed, VA Healthcare Technology Management (HTM).

Telemental Health Services: The contractor shall provide telemental health services. The contractor will provide support for VA provided telemental health services, providing staff to arrange appropriate time and space for the veteran, and technical support, including test calls for video-to-home visits, as needed. Use of telemental health to support the delivery of services is encouraged as a mechanism for meeting requirements throughout this document. Nevertheless, it is important to recognize that there may be limits to the services that can be provided using this technology. These may include certain highly interactive and "high-touch" evaluations or interventions.

Telepharmacy Services: The contractor shall provide clinical pharmacy telehealth services. The contractor shall provide clerical support, including scheduling, and ancillary support for VA telepharmacy services. The support services should be consistent for each scheduled clinic to include but not limited to intake vitals by LVN/LPN, unlicensed assistive personnel (health tech or nursing assistant), or similar, downloading of blood sugar from meters, POC INR testing and downloading, teaching patients how to use BP monitors at home, calling patients for lab reminders, scheduling patient visits and contacting patients who no-show for rescheduling.

Telephysical Therapy Services: The contractor shall provide clinical physical therapy telehealth services. The contractor shall provide clerical support, including scheduling, and ancillary support for VA telephysical therapy services. Nevertheless, it is important to recognize that there may be limits to the services that can be provided using this technology. These may include certain complex evaluations or interventions.

Telephone Access to Clinical Care: The contractor shall make provisions for toll free telephone care, twenty-four (24) hours a day, seven (7) days a week, including evenings, weekends, and holidays, for all Veterans, in accordance with VHA Directive 1090, "Telephone Access for Clinical Care," (or subsequent revisions thereto). The contractor shall ensure qualified, trained clinical and administrative staff members who have direct access to patient records are available to receive and answer incoming clinical care concerns to the clinic. This directive establishes telephone access performance metrics for telephone service, which can be used by VA to monitor contractor performance (e.g., call volume, abandonment rate, average speed to answer, and service line).

5.9 Communicating Test Results to Providers and Patients: Contractor shall communicate test results in accordance with VHA Directive 1088, Communicating Test Results to Providers and Patients. All test results requiring action shall be communicated by the ordering provider, or designee, to patients within 7 calendar days from the date on which the results are available. For test results that require no action, results shall be communicated by the ordering provider, or designee, to patients within 14 calendar days from the date on which the results are available. Depending on the clinical context, certain test results (such as emergent and imminently life-threatening and urgent not imminently life-threatening test results) may need review and communication to patients in shorter timeframes. See VHA Directive 1088, Communicating Test Results to Providers and Patients for content and method of communication, documentation, service-level workflows, and other test results communication requirements.

The contractor shall provide the VA parent facility with current contact numbers (e.g., cell phone/pager) for all staff (e.g., physicians, nurse practitioners, physician assistants, etc.) ordering tests (e.g., imaging exams and

laboratory tests) in order to receive emergent and imminently life-threatening and urgent not imminently life-threatening test results from VA diagnostic providers. For emergent and imminently life-threatening test results, the ordering provider shall respond back to the VA diagnostic provider within 30 minutes of the initial page or telephone call. The receiving ordering provider or designee shall document the test results in the record and conduct a "read back" procedure to ensure accuracy of transmission and translation of all verbal test results. The contractor shall also follow the VA parent facility test results communication policy. See policy Memorandum 11-44, Communicating Critical and Non-Critical Test Results to Providers and Patients. Section D (attachments) for additional requirements regarding communication of test results.

5.10 Referral for Non-Emergent VA Inpatient Services: The contractor shall be responsible to contact [REDACTED] to schedule admission should non-emergent inpatient care be deemed necessary by the contractor. **NOTE:** all inpatient care is outside the resultant contract-no costs should be charged to the resultant contract.

5.11 Emergency Response Requirements: The contractor shall have a local policy or standard operating procedure defining how emergencies are handled, including mental health emergencies. This policy or SOP should include any state specific procedures such as special forms to complete for involuntary evaluations. Should emergency care be deemed necessary by the contractor upon evaluation of the patient at the site, the contractor shall first call 911 to arrange for emergency transportation to the closest facility that provides emergency care. After the emergency is resolved, the contractor shall notify [REDACTED] during normal working hours and [REDACTED] after normal working hours. Mental health emergencies shall be considered with the same degree of urgency as other emergencies. Under no circumstances should emergent intervention be delayed pending administrative guidance from the VA. A VA Community Care consult is required to be entered by the provider after the patient has been transported to the emergency facility. After notification, the VA will make a determination of eligibility for payment purposes. For emergencies involving video-to-clinic telehealth, the contractor shall ensure compliance with Standard Telehealth Emergency Handoff Procedures for Patient Emergencies During Synchronous Video Telehealth Visits into a VA Clinic and Standard Telehealth Emergency Handoff Procedures for Patient Emergencies During Synchronous Video Telehealth Visits into the Home, an ATLAS Site, or Other Non-VA Location (see "Telehealth Training for Staff" under Telehealth Services Requirements of this PWS).

Persons not verified eligible for VA care who present to the contractor in need of urgent or emergent care shall be treated on a humanitarian basis until stable and discharged from the contractor's outpatient site of care or referred to the proper level of care in the community. If the patient is determined to have no authorization for services, and has received care by the contractor, the patient will be billed directly by the VA and will be informed by the contractor that they are not eligible to continue receiving services at this site.

Patients who self-refer to local emergency facilities and their associated charges for care are not the responsibility of the contractor; and shall not be provided service under this contract, even if the designated primary care provider under this contract is performing "on call" duties at the local facility. Patients who self-refer to emergency facilities should be referred to [REDACTED] as soon as possible to determine if emergency care will be paid for by VA.

If a patient who is not actually receiving care in contractor's facility contacts the contractor, and the contractor believes that the Veteran needs emergency care that the contractor cannot provide, the contractor shall advise the patient to go to the nearest emergency care facility or call 911. The contractor shall also advise the patient to contact [REDACTED] as soon as possible to determine if emergency care will be paid for by VA.

If any patient presents in need of urgent or emergent care contractor shall provide practicable monitoring and intervention until patient is stabilized or transported to a higher level of care. Immediate notification of [REDACTED] is mandatory. It is important to note that the VA can also provide emergency treatment for eligible Veterans based on 38 CFR 17.4020(c) and 38 CFR 17.4000-17.4040 via community care hospitals. These services can be authorized under 38 CFR 17.4020(c) when: a Veteran is referred from the VA facility to an in-network community facility or when a Veteran self-presents to an in-network community facility; proper notification is received timely; and the Veteran meets administrative and clinical eligibility criteria. The Veteran shall be followed with continued stay reviews until transferred to VA once stabilized, discharged, or the Veteran refuses information sharing with the VA. In instances where VA cannot provide the appropriate level of care or the referring VA facility does not have sufficient inpatient capacity or capability for subsequent care, and federal facilities with which VA has agreements with were unavailable, the Veteran can be referred to an in-network ED or inpatient services for further evaluation and

treatment when clinically indicated for transfer. Referrals originating from a VA Facility to a Community Care in-network ED, will be considered authorized and shall be accompanied by a VA referral. Under no circumstances shall emergency care be delayed pending administrative guidance from the VA.

The contractor shall maintain appropriate emergency response capability. Outpatient sites of care are required to have Automatic External Defibrillators (AEDs). The contractor is responsible for performing the device checks and supplying monthly reports to the COR verifying that the checks are being performed in accordance with the contract requirements. Smaller sites that do not have the appropriate staff mix to manage a code need to dial 911 in addition to retrieving and using the AED. At these facilities, the physician director, in consultation with the code team at the VA, shall determine the best location for AEDs throughout the facility. The VA will provide the contractor with an AED and train contractor's staff in its use and checks of the device.

- 5.12 Non-Emergency Transport Requests:** During business hours, the contractor shall contact [REDACTED]. The Patient Transportation Office will review the request and if patient is eligible will make arrangements for either in-house or contract transportation. Request for non-emergent transportation occurring after normal business hours should be made to [REDACTED].

- 5.13 Non-emergent Specialty Consultations, Care, and Diagnostic Tests Performed at VA:** Non-emergent specialty consultations and diagnostic tests not performed at the contractor's site will be performed at the VA. Contractor shall request specialty consultations electronically through the VA EHR and include consult service requested, urgency, diagnosis (when required), and reason for request. All additional information required by some specialty sections shall be entered by the referring contractor's primary care provider via the consult template.

The contractor is responsible for the coordination of the patient's primary care including referral to specialties as indicated. The VA serves as the referral center for any care or service outside the scope of this contract unless pre-authorized by the VA. The VA is responsible for communicating with the contractor results of any treatment provided by the VA for the patient. The primary communication link will be the VA EHR. Consult services available at VA via electronic request include the following:

Medicine, Surgery, Allergy, Anesthesia, Anticoagulation, Autopsy Request, Bariatric Surgery, Audiology, Speech, Cardiology, Cardiac Surgery, Mental Health, Dermatology, Colorectal Cancer, Clinical Pharmacy, Emergency Department, Referral Care, Community Based Care, Endocrine/Diabetes, ENT, Communication, General Medicine, General Surgery, Dental, Gastro Intestinal (GI), Gynecology, Laboratory, Hematology/Oncology, Neurosurgery, Geriatric, Hospice (Palliative Care), Ophthalmology/Optometry, , Orthopedic, Nutrition & Weight, Infectious Disease, Plastic, Pain Management, Neurology, Podiatry, Pastoral Care, Pulmonary, Pressure, Primary Care, Renal, Ulcer/Wounds, Prosthetics, Rheumatology, Thoracic Surgery, Radiation Therapy, Therapeutic Phlebotomy, Transplant, Recreation, Vascular, (Liver/Renal), Rehab Medicine, Physical Therapy, Urology, Social Work, Urogynecology, Speech Pathology

- 5.14 Non-Emergent Specialty Consultations, Care, and Diagnostic Tests Not Provided at VA or Contractor's Site:** The charges incurred from non-emergent specialty evaluations, diagnostic testing, and care provided at sites other than the VA will be the responsibility of the contractor, unless prior authorization is obtained from [REDACTED]. A request for Authorization for Community Care Services is requested by the ordering Provider by completing the VA EHR Community Care consult. Subsequent approval may be granted upon review by the Community Care approving physician or nurse.

6. ADMINISTRATIVE:

Meetings: Contractor's Personnel shall attend service staff meetings as required by the VA COS or designee. Contractor to communicate with COR on this requirement and report any conflicts that may interfere with compliance with this requirement.

Scheduling of Services and Cancellations: It is VHA policy that outpatient health care appointment requests, regardless of modality, are managed safely, timely and accurately, and are scheduled based on clinical need and Veterans' preference using available technologies. Veterans are to be provided an opportunity to schedule an appointment in either a VA medical facility or the community, when eligible. Staff responsible for scheduling shall ensure to offer VA Community Care as an option when appropriate, based on MISSION Act VA Community Care eligibility standards. The scheduling of all appointment requests originating from fully

processed VA Form 10-10EZs shall be initiated within 7 calendar days. The contractor shall meet the Veterans Health Administration's (VHA's) scheduling standards as outlined in VHA Directive 1230 "Outpatient Scheduling Management" and VHA Directive 1232(5) "Consult Processes and Procedures."

Staff responsible for scheduling shall follow the guidance in VHA Directive 1230 and the accompanying Appointment Management Business Rules SOP to offer VA Community Care appropriately and document the outcome of the discussion with the Veteran. As outlined within this directive and supporting SOPs, if the Veteran is eligible for VA Community Care, this information shall be communicated to the Veteran for consideration. If Veteran is not eligible for VA Community Care, the staff responsible for scheduling shall review to determine if the wait time eligibility criteria is a factor for the care. The Veterans selection for VA Community Care shall be documented as part of the referral or the appointment, once scheduled.

All staff scheduling VA appointments shall follow the guidance within VHA Directive 1230 directive as it pertains to:

- Minimum scheduling efforts, when attempting to contact patients to schedule appointments and shall ensure to document appropriately within the medical record as outlined in the supporting SOPs.
- Unable to schedule process for Veterans.
- Scheduling for same day services.
- Use of the LEAF process for intrafacility transfer requests.
- Scheduling training requirements.
- Patient self-referral process for direct scheduling services.
- Guidance for scheduling service members transitioning from active duty.

Additionally, all staff managing consults shall follow the guidance from VHA Directive 1232(5) and the supporting SOP as it relates to processes for:

- Timeline processing and scheduling of consults.
- Appropriate consult set up requirements.
- Roles and responsibilities in the consult management process.
- Consult management business rules.

NOTE: Patients referred for care outside the contractor's clinic (e.g., VA Community Care, VA facility, or similar) for care that the contractor is required to provide under the terms of this contract will be removed by the VA parent facility from the applicable billable roster the month after the patient is referred for care.

The VA Community Care (CC) Eligibility/Decision Support Tool (DST) tab in the Consult Toolbox in the VA EHR automates and streamlines VA Community Care eligibility determinations. The contractor shall offer Veterans the option of VA Community Care when appropriate, based on the six VA Community Care eligibility standards below:

- **Best Medical Interest (BMI) of the Veteran**
BMI decision shall be made and captured by a physician or non-physician provider (i.e., PA or NP), and not administrative staff. An RN can capture BMI only if a physician or non-physician provider has documented that BMI is appropriate. This can only be captured by an RN during the Consult Tool Box (CTB) forward workflow. BMI per episode of care allows the patient to obtain their care in the community for a specific episode of care. BMI hardship allows a patient to obtain some or all of their care in the community.
- **Quality Standards (i.e., 1703e Eligibility)**
VA conducts ongoing assessments to determine when a health care service is non-compliant with VA's standards for quality. When this occurs, the VA Secretary may decide Veterans are eligible to receive care from a community provider for those specific services while we work to remediate and improve them. NOTE: Currently, the VA Secretary has not made any designation and zero (0) Veterans are eligible for Community Care under Quality criterion, therefore no one should select "1703e" as an option for eligibility at this time in the Consult Toolbox or as a consult justification reason.

- **Access Standards**
This applies when care or services are not provided within designated access standards based on drive time or wait time:
Drive Time: Drive time is calculated using the VA Community Care (CC) Eligibility/Decision Support Tool (DST) tab in the Consult Toolbox and is calculated from Veteran's residence (physical address) to nearest VA medical facility that can provide the services. Actual drive time may differ depending on the route used, but we use the average drive time standards to determine eligibility. The standard for Primary Care, Mental Health and Non-Institutional Extended Care Services is 30 minutes average drive time. The standard for Specialty Care is 60 minutes average drive time. If the CTB/DST is not working, the drive time shall be calculated using Bing Maps. If using Bing Maps the scheduler shall add the appointment comment: #COO# DT=minutes if patient has other eligibilities, but opts out of community care, or DT=minutes if patient does not have other eligibilities. When the specific clinical service is not available at the VA parent facility, the CTB eligibility selection shall be DT=specific service not locally available.
Wait Time: Wait time is established by first determining if the appointment request is within the community care wait time standards (WTS). If the request is within the WTS, the scheduling staff shall then determine if the appointment can be scheduled within 20 calendar days for Primary Care, Mental Health and non-Institutional Care or within 28 calendar days for Specialty Care. At this time the contractor shall also review the average community care wait time for the type of care being requested. This can be reviewed using the CTB CC Eligibility/DST tab. This will assist with guiding the wait time conversation with the Veteran.
- **Grandfathered eligibility from Veterans Choice Program**
This applies to Veterans who live in North Dakota, South Dakota, Montana, Alaska & Wyoming and previously qualified under the Veteran's Choice Program's 40-mile distance requirement on June 6, 2018.
- **Veteran lives in a location with no Full-Service Medical Facility**
This applies to Veterans residing in Alaska, Hawaii, New Hampshire, and the U.S. territories of Guam, American Samoa, Northern Mariana Islands, and the U.S. Virgin Islands.
- **Services not available at VA**
Service not available means the service is not available at the VA as a whole. Veterans are eligible for community care if they need a specific type of care or service that VA does not provide directly to Veterans in-house at any VA medical facility. Examples include maternity care, in vitro fertilization/assisted reproductive technology (IVF/ART) treatment, homemaker/home health services, and hospice care. When a provider or service is not available at a specific VAMC the contractor shall review for potential other eligibility such as access standards (drive time or wait time) or best medical interest.

The contractor shall be responsible for scheduling face-to-face and virtual visits with other health care providers including nurses, mid-level providers, CPPs, or dietitians for the purposes of monitoring or preventing disease and providing patients with information and/or skills so they can participate in decision-making and self-care. All staff responsible for scheduling are required to attend the mandatory onboarding scheduling training, as outlined in VHA Directive 1230.

The contractor shall maintain a triage system for walk-in patients. Walk-in patients are to be triaged by a qualified clinician and care provided or patient will be transferred to a higher level of care if the care required exceeds that of the level of care provided at the contractor's site of care. The contractor shall provide same day appointments as needed and in accordance with VHA Directive 1230 and associated SOP's (Refer to Same Day Service (SDS) requirements). Traveling Veterans shall be cared for in accordance with VHA Handbook 1101.11(4), "Coordinated Care for Traveling Veterans

Cancellations: Contractor shall not unnecessarily cancel patient appointments and will reschedule cancelled appointments in a timely manner. Cancelled appointments will be rescheduled with patient input and use of the appropriate patient indicated date (PID), based on the type of cancellation.

No Shows: Contractor shall reschedule appointment as close to the original PID as possible. For further guidance see Appointment Management Business Rules SOP listed in Appendix B of VHA Directive 1230 "Outpatient Scheduling Management" for cancellations/no-show process business rules. For MH services no shows, see MH services section of this PWS.

7. **EDUCATION AND SUPERVISION OF HEALTH PROFESSIONS TRAINEES (HPTS):** Education and Supervision of HPTs: Per the guidelines dictated by the VA Office of Academic Affiliations (OAA) directives and health professions education (HPE) accrediting agencies, the contractor performing the services of the contract will be responsible for the education and supervision of health professions trainees. Contract provider shall be responsible for:

Education of HPTs: Contractor shall provide an academic environment conducive to the training and professional development of HPTs rotating through the Lincoln County clinic in accordance with current VA guidelines. Contractor shall meet the educational objectives of the training program as outlined in the program letter of agreement. Contractor may provide practice-based learning opportunities and/or didactic teaching to HPTs.

Supervision of HPTs: Contractor shall provide supervision of HPTs in accordance with current VA, guidelines. Contractor shall ensure HPT supervision in accordance with VHA Directive 1400.01, Supervision of Physician, Dental, Optometry, Chiropractic, and Podiatry Residents and VHA Handbook 1400.04, Supervision of Associated Health Trainees. Contractor shall be responsible for overseeing HPTs' acquisition and demonstration of knowledge, skills, attitudes, and professionalism while the HPT participates in patient care. Contractor shall be responsible for periodic evaluation of HPTs and the training program as required by accrediting agencies and program letters of agreement. Contractor shall be responsible for ensuring that all notes and encounters are completed, clearly demonstrate the involvement of the supervising practitioner, and comply with VA standards and standard commercial practice and guidelines established by VHA Directive 1400.01 and VHA Handbook 1400.04. The contractor shall also perform any administrative duties related to documentation of HPT training, as required, and directed by the VA COS or designated representative.

8. **MY HEALTHEVET PROMOTION:** Veterans interested in the My HealtheVet initiative will be directed to the web site <http://www.myhealth.va.gov> where they can register as a Veteran seen at the VA Health Care System. Once registered, the Veteran can present to the contractor's outpatient site of care to be authenticated.

9. **MEDICAL RECORDS/COMPUTERIZED RECORD SYSTEMS/DISCLOSURE/RECORD RETENTION:**

MEDICAL RECORDS REQUIREMENTS: Authorities: Contractor providing treatment and healthcare services to VHA patients shall comply with the 5 U.S.C. 552a (Privacy Act), 38 U.S.C. 5701 (Confidentiality nature of claims), 5 U.S.C. 552 (FOIA), 38 U.S.C. 5705 (Confidentiality of medical quality assurance records) 38 U.S.C. 7332 (Confidentiality of certain medical records) and 45 C.F.R. Parts 160, 162, and 164 (Health Insurance Portability and Accountability Act's Privacy Rule).

The resultant contract and its requirements meet the exception in 45 CFR 164.502(e) for a covered entity such as VHA to disclose protected health information to another health care provider for treatment. Based on this exception, a Business Associate Agreement (BAA) is not required for this contract. Treatment and administrative patient records generated by this contract or provided to the contractor by the VA are covered by the VHA system of records titled 'Patient Medical Records-VA' 24VA10P2. Contractor generated VHA patient records are the property of VHA and shall not be accessed, released, transferred, or destroyed except in accordance with applicable laws and regulations. Contractor shall ensure that all records pertaining to medical care and services are available for immediate transmission when requested by VHA. Records identified for review, audit, or evaluation by VHA representatives and authorized federal and state officials, shall be accessed on-site during normal business hours, or mailed by the contractor's provider at his expense. Contractor shall deliver all final patient records, correspondence, and notes to VHA within twenty-one (21) calendar days after the contract expiration date.

Neither the VA nor the contractor has the legal authority to require that a Veteran provide their social security number to the VHA as a condition for receiving medical care under Title 38, United States Code. If the patient does not provide a social security number, the VA will assign a unique identification number to the patient.

VA utilizes both a scanned and electronic health record (EHR). The scanned component of the medical record will consist only of those items not already in the VA EHR (such as health records from a non-VA provider). The VA requires that all medical entries be entered into the VA EHR, including, but not limited to, prescriptions, labs, radiology requests, progress notes, vital signs, problem lists, and consults. VHA will provide the necessary training to contractor personnel on the proper use and operation of the VA EHR.

Clinical Reminders: Proper documentation and completion of all clinical reminders is required as they appear during a patient's visit. Standard is 90% completion of all clinical reminders monthly. The VA EHR will automatically remind providers to complete clinical reminders during patients visits.

Professional standards for documenting care: Medical record entries shall be maintained in detail consistent with medical and professional practices to facilitate internal and external peer reviews, medical audits, and follow-up care. The quality of medical practice shall meet or exceed reasonable standards of professional practice for the required services in health care as determined by the same authority that governs VA parent facility medical professionals and will be audited by the VA parent facility or in accordance with other processes established for that purpose.

The contractor shall maintain up-to-date electronic medical records at the site where medical services are provided for each patient under this contract. Records accessible by the contractor during the performance of this contract are the property of the VA and shall not be accessed, released, transferred, or destroyed except in accordance with applicable federal law and regulations. The treatment and administrative patient records created by, or disclosed to the contractor under this agreement are maintained in VA's Privacy Act system of records entitled "Patient Medical Records-VA" 24VA10P2. VA shall have unrestricted access to patient medical records received or created by the contractor.

The contractor shall maintain electronic medical records using the VA EHR making sure they are up-to-date and shall include the enrolled patients' medical records to all subcontractor providers. The electronic record shall include, at a minimum, medical information, prescription orders, diagnoses for which medications were administered or prescribed, documentation of orders for laboratory, radiological, EKG, hearing, vision, and other tests and the results of such tests and other documentation sufficient to disclose the quality, quantity, appropriateness, and timeliness of services performed or ordered under this contract. Each member's record shall be electronic, which includes scanned images, and will be maintained in detail consistent with medical and professional practice and VA policy. No documents from the electronic medical record will print and no shadow or duplicate records are authorized. Effective internal and external peer review and/or medical audits facilitate an adequate system of follow-up treatment. Hard copies of external source documents may be scanned into the electronic medical record by the contractor, or a summary progress note written by an appropriate clinician after a review of the external source documents may be used in lieu of scanning any external source documents. After these documents, have been scanned, an audit of the scanned records shall be conducted by the contractor to assure they are scanned properly, and then the original documents are to be sent via a secure delivery service that tracks mail from pick-up to delivery at the contractor's expense to [REDACTED] to be stored for 9 months and then destroyed. Scanning and audit reports will be sent via PKI encrypted e-mail to the VA file room/scanning supervisor and file room/scanning lead by the end of the first week of every month. No paper record shall be maintained. If there are no errors found, the contractor shall report via email that there were no errors to be reported for the previous month.

Documentation and Clinical Records: Documentation and clinical records shall be complete, timely, and compliant with VA policies. The contractor shall not allow its inability to access the VA EHR to prevent any patient from being seen by a provider. In the event, and for any reason, that the contractor is not able to access the VA EHR system, the contractor shall record all data manually including the completion of the encounter form. Upon recovery of the contractor's ability to access the VA EHR, the contractor shall input all data recorded manually into the VA EHR within forty-eight (48) hours of the system becoming operational. See local documentation and clinical records policy in Section D (attachments).

The contractor shall report workload (check-in, check-out) within two (2) business days. The contractor shall provide patient encounters (visits) workload in accordance with established VA reporting procedures. All progress notes, medication orders, and test results, applicable to services which the contractor is responsible to provide and perform at its site or subcontractor's site, shall be entered into the VA EHR by the contractor. The contractor shall enter the return to clinic (RTC) order to allow follow-up appointments to be made during check-out.

VA radiologist's professional interpretation of diagnostic radiology and diagnostic imaging performed by the contractor shall be entered into the VA EHR by VA. contractor shall be responsible for entering all information and requests for laboratory and radiology test requests into the VA EHR. Progress Notes shall be entered into the VA EHR the same day as the visit/encounter and shall meet CMS guidelines for documentation which include the 3 key components to determine the level of evaluation and management (E/M). These key components include: (1) History; (2) Exam; and (3) Medical decision making. Progress notes associated with

each visit/encounter will include pertinent medical treatment, test results, a treatment plan, teaching that was provided to the patient and/or the patient's family, the date of appointment, and the electronic signature of the treating clinician.

All progress notes shall be linked to the correct visit/encounter and location. A patient problem list shall be present on the patient's record by the third clinic visit and will be entered via the VA EHR on the Problem List tab. This list will include all diagnoses, medications and procedures and will be updated as the patient's condition changes. Laboratory reports and results will be entered into the Laboratory Package. The process for entry of data may include manual entry or an automated procedure; however, it shall adhere to applicable VA Automated Information Security (AIS) system regulations. Questions may be directed to [REDACTED]

Patient Care Encounter (PCE) module: The contractor shall adhere to the most current American Medical Association Current Procedural Terminology (CPT) and the Centers for Medicare and Medicaid Services' International Classification of Diseases (ICD) coding standards for all services provided. The contractor shall use accurate and complete CPT and ICD codes for each patient care encounter. The contractor shall electronically complete encounter form data within two (2) business days of visit. Completed encounter forms will include, but are not limited to, the problem list, appropriate CPT codes, appropriate ICD codes, designation of a primary provider, and whether the treatment or care rendered was for a service-connected condition or as a result of exposure to agent orange, environmental contaminants, or ionizing radiation. The contractor is responsible for resolving all action required and encounter errors daily.

Event Capture System: Contractor shall ensure that dietitians electronically complete an Event Capture System form for all direct patient care, whether individual visits, group visits or team meetings that discuss individual patients.

Forms: The contractor shall utilize the standardized, VHA templates, available in the VA EHR. Requests for revisions to the standardized templates or new templates are to be submitted to [REDACTED] for approval.

Access to VA Records: Subject to applicable federal confidentiality laws, the contractor or its designated representatives may have access to VHA records at VHA's place of business on request during normal business hours where necessary to perform the duties under this resultant contract.

Reports: The contractor is responsible for complying with all related VA reporting requirements requested by the VHA.

Availability of Records: The contractor shall make all records available at the contractor's expense for review, audit, or evaluation by authorized federal, state, and comptroller or VHA personnel. Access will be during normal business hours and will be either through on-site review of records or through the mail. All records to be sent by mail will be sent via a secure delivery service that tracks mail from pick-up to delivery at contractor's expense to the VA within one (1) business day of request.

External Peer Review Program: The contractor shall document in the medical record preventive health case management measures and the chronic disease indicators of the enrolled patient. The medical treatment records generated by the contractor during performing services under this contract shall be made available for audit by the VHA's External Peer Review Program (EPRP). Medical record data shall be available in the VA EHR and any additional records required for EPRP audit will be promptly forwarded to the VA upon request. This data will be sent via a secure delivery service that tracks mail from pick-up to delivery at contractor's expense if necessary to meet the due date requested by the VHA. EPRP is provided to the VHA by other contractors. Contract providers who are seeing VA patients are the VHA providers and as such are provided access to confidential patient information as contained in the medical record.

Release of Information: The VHA's Release of Information Section shall provide the contractor with assistance in completing forms. Additionally, the contractor shall use VA Form 10-5345, Request for and Authorization to Release Medical Records or Health Information, when releasing protected health information or any records protected by 38 U.S.C. § 7332. Treatment and release records shall include the patient's consent form. Completed Release of Information requests will be forwarded to [REDACTED]

Disclosure: Contractor may have access to patient medical records; however, contractor shall obtain permission from VHA before disclosing any patient information. Subject to applicable federal confidentiality or privacy laws, the contractor, or their designated representatives, and designated representatives of federal regulatory agencies having jurisdiction over contractor, may have access to VHA's records, at VHA's place of business on request during normal business hours, to inspect and review and make copies of such records. VHA will provide the contractor with a copy of VHA Directive 1907.01, VHA Health Information Management and Health Records and VHA Directive 1605.01, Privacy and Release of Information. The penalties and liabilities for the unauthorized disclosure of VHA patient information mandated by the statutes and regulations mentioned above, apply to the contractor and/or subcontractors.

The contractor shall provide copies of medical records, at no charge, when requested by the VHA to support billing and/or VA mandated programs if these records are not available in the VA EHR. The contractor shall use VA Form 10-5345a, Individuals' Request for a Copy of Their Own Health Information when releasing a Veteran's health information to the Veteran. The contractor shall release information in accordance with the Privacy Act of 1974, and the Health Insurance Portability and Accountability Act's Privacy Rule, 38 U.S.C. §§ 7332, 5701 and 5705. Release of Information software will be used to print and release record information thus accounting for all disclosures of record information. The contractor shall use the provided software package DSS ROI Manager to record and account for all release of information request processed by the contractor. When releasing medical records to the Veteran themselves, the 10-5345a form will clearly indicate: The Veteran full name and full SSN; The information that was released as authorized by the Veteran; The date the information was released (inferred that date signed is date released); Block will be checked that the information was released in person to the Veteran. When releasing the information to an outside third party, the 10-5345 form will clearly indicate: Full name of Veteran and full SSN; Complete address of third party to who the records were released to; The exact information that was released as authorized by the Veteran; The purpose for third party receiving the records; The expiration date for authorization; Whether any or all §7332 – protected information may be disclosed.

Records Retention: The contractor shall retain records generated during services provided under this contract for the time periods required by VHA Record Control Schedule 10-1, VA regulations (24VA10A7, Patient Medical Records - VA, par. Retention and Disposal), VHA Directive 6300(1) and VHA Directive 1605.01. No hard copies of medical records or logbooks of any type may be maintained. If this agreement is terminated for any reason, the contractor shall promptly provide the VA with any individually identified VA patient treatment records or information in its possession, as well as the database created pursuant to this agreement, within two (2) weeks of termination date.

Work Related Incident Treatment: When treating the Veteran for injuries sustained because of a work-related incident or an accident, the contractor shall complete the appropriate forms to allow the VA to assert a Federal Medical Care Recovery Act (FMCRA) or a Workers Compensation Claim.

10. PATIENT RIGHTS, SAFETY, COMPLAINTS, GRIEVANCE SYSTEM PROCESSES:

Patient Rights and Responsibilities: Contractor shall conform to all patients' rights issues addressed in VA Medical Center Memorandum 11-06, which is provided in the attachments section.

Safety: Adverse events at the contractor's site shall be reported by any staff member with a Personal Identify Verification (PIV) card using the Patient Safety Reporting System, as outlined in VHA Directive 1050.01, VHA Quality and Patient Safety Program. Adverse events will be scored utilizing the Safety Assessment Code (SAC) for determination of the need for conducting a root cause analysis (RCA). **NOTE:** Reporting adverse events in the patient safety reporting system does not eliminate the need for the provider to document or report events related to a patient in the VA EHR, or to disclose an adverse event to a patient, as defined by other requirements.

Patient Complaints: The VA Patient Advocacy Program was established to ensure that all Veterans and their families, who are served in VHA facilities and clinics, have their complaints addressed in a convenient and timely manner in accordance with VHA Directive 1003.4, "VHA Patient Advocacy".

All patient complaints are to be reported immediately (within 24 hours) to the CO and COR. The CO shall resolve complaints received from the COR concerning contractor relations with the Government employees or patients. Providers and staff are familiarized with the process outlined in contractor's grievance procedures as well as patient rights. The CO is final authority on validating complaints. If the contractor is involved and named in a validated patient complaint, the government reserves the right to refuse acceptance of the services of

such personnel. This does not preclude refusal in the event of incidents involving physical or verbal abuse. Response to complaints will occur as soon as possible, but no longer than seven (7) business days after the complaint is made. All patient complaints will be entered in the National Patient Complaint database. Information concerning the Patient Advocacy Program shall be prominent and available to patients seen at the outpatient site of care. The VA will provide the contractor with informational handouts describing the program and how to contact the VA Patient Advocate.

Grievance System Requirements: Patients have the right to grieve actions taken by the contractor directly to the contractor. The contractor shall provide readable materials reviewed and approved by VA, informing enrolled patients of their grievance rights. The contractor shall develop internal grievance procedures and obtain VA approval of the procedures prior to implementation. The grievance procedures shall be governed by the guidelines in VHA Directive 1003.4, "VHA Patient Advocacy".

- 11. CONTRACTOR BREACHES OF CONDUCT:** The Government reserves the right to refuse acceptance of Contractor, if personal or professional conduct jeopardizes patient care or interferes with the regular and ordinary operation of the facility. Breaches of conduct include intoxication or debilitation resulting from drug use, theft, patient abuse, dereliction, or negligence in performing directed tasks, or other conduct resulting in formal complaints by patient or other staff members to designated Government representatives. Standards for conduct shall mirror those prescribed by current federal personnel regulations. The CO and COR shall deal with issues raised concerning contract personnel's conduct. The final arbiter on questions of acceptability is the CO.
- 12. SPACE REQUIREMENTS:** PACT space standards are found in the PACT Space Module Design Guide at <http://www.cfm.va.gov/til/dGuide/dgPACT.pdf>. Accessibility design standards are defined in the VA Barrier Free Design Standard at: <http://www.cfm.va.gov/til/etc/dsBarrFree.pdf>. These documents are to be used by VA as a reference in defining space requirements for this contract. Contractor's design of space will be evaluated on how it best supports accessibility and the PACT model of care. For any buildout/construction that might be required to prepare space for performance of this contract, Davis-Bacon Wages (<https://www.dol.gov/agencies/whd/government-contracts/construction>) shall be paid to any personnel working on the buildout/construction of the space and certified payrolls for all buildout/construction workers are due to the COR and CO , no later than 10 business days prior to the clinic opening.

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Contractors shall provide space that meets the standards outlined in the PACT Space Module Design Guide (see link above) and for the purposes of this contract the following minimum space requirements are:

Table 1-Space Requirements			
Function	ABOA Square Feet	Number of Rooms	Total ABOA Square Feet
Janitorial	80	1	80
IT closet	170	1	170
Med room	125	1	125
Clean utility	150	1	150
Soiled utility	80	1	80
Exam room	125	4	500
Telehealth	125	2	250
Counseling room	125	1	125
Waiting	225	1	225
Group room	240	1	240
Private office	100	2	200
Reception	385	1	385
Storage	100	1	100
Lab	150	1	150
Staff toilet	60	2	120
Waiting toilet	60	1	60
Patient toilet	60	2	120
Lab toilet	60	1	60
Entry	200	1	200
Break room	220	1	220
Circulation Space	TBD	TBD	TBD
Total ABOA Square Feet:	3,560		

VA Portland Health Care System (VAPORHCS) requires clinic space, equipment, supplies and staff as detailed in this solicitation Primary Care services for to up to 1600 patients in the Lincoln County area. Space will be shared by VA Mental Health providers. The requirement shall be met in one physical location within Lincoln County, Oregon.

The codes, standards, and references listed below indicate minimum design and building performance requirements. Compliance is required with applicable codes and standards throughout the process of design, construction, acceptance, and on-going maintenance of the outpatient site of care facility. Design and documentation of the outpatient site of care shall be in compliance with the requirements of the codes, standards, and references listed below. Facility shall conform to the most recent applicable codes, which includes the state and local codes as required by the local Authority Having Jurisdiction (AHJ). This also covers construction materials and standards not fully addressed by the codes, standards, and references below.

Codes: All contracted outpatient sites of care are to conform to all applicable codes, which includes but are not limited to following:

- National Fire Protection Association (NFPA) 101 Life Safety Code, NFPA 99
- Health Care Facilities Code and all standard references therein, current edition.
- Occupancy classification to be Chapter 38 New Business Occupancy.
- NFPA 75 Standard for the Fire Protection of Information Technology, current edition, applicable where the outpatient site of care contains a Data Center.
- Architectural Barriers Act Accessibility Standards (ABAAS), current edition.
- International Building Codes (IBC): New buildings shall comply with IBC 2015 or later edition (within 2 years of release).
- International Energy Code
- International Mechanical Code
- International Plumbing Code
- State and local International Building Codes (IBC) amendments as required by the local Authority Having Jurisdiction (AHJ).
- United States Pharmacopeia Convention (USP) Chapters <795>, <797>, and <800>

Design Standards: All contracted outpatient sites of care are to conform to the following standards:

- Facility Guidelines Institute (FGI) Guidelines for the Design and Construction of Outpatient Facilities, current edition (referred to as FGI Guidelines herein).
- Accreditation standards for outpatient clinics shall reflect a business occupancy. Contractor shall provide and submit all documentation as required for the accreditation requirements.
- GSA Facility Security Level (FSL) shall be Facility Security Level II.
- Sustainability standards shall be met by the contractor as outlined by the VA.
- American Society of Heating, Refrigerating and Air-Conditioning Engineers (ASHRAE) Standard 170, Ventilation of Health Care Facilities, current edition (referred to as ASHRAE 170 herein)
- ASHRAE Standard 188, Legionellosis: Risk Management for Building Water Systems, current edition.
- ASHRAE Standard 62.1, Ventilation for Acceptable Indoor Air Quality, current edition (referred to as ASHRAE 62.1 herein)
- Sheet Metal and Air Conditioning Contractors National Association (SMACNA) Standard, HVAC Duct Construction Standards – Metal and Flexible.
- USP General Chapter 797 Pharmaceutical Compounding – Sterile Preparations
- USP General Chapter 800 Hazardous Drugs – Handling in Healthcare Settings
- Association for the Advancement of Medical Instruments (AAMI) standards as applicable to SPS and RME storage.
- Illuminating Engineering Society North America (IESNA) Handbook – latest edition
- American National Standards Institute (ANSI) Z358.01 Emergency Eyewash and Shower Standard

References: Following are guides, manuals, and other references developed by VA applicable to contracted outpatient sites of care and can be located at: <https://www.cfm.va.gov/til/>

VA Signage Design Manual PG 18-12

- H 18-8 Seismic Design Handbook
- VA Physical Security and Resiliency Design Manual (VA PSRDM) and Appendix B of VA Handbook 0730
- VA Enterprise Facility IT Support Infrastructure Standard posted here: [Telecom Data Center and Infrastructure Engineering Design Standards Service Catalog - Office of Construction & Facilities Management \(va.gov\)](#).
 - Building Prototypes, Community Based Outpatient Clinic
 - PACT Space Module Design Guide
 - Space Planning Criteria PG 18-9
 - Room Templates, Leased Community Based Outpatient Clinics
 - Physical Therapy Space requirements: Physical Medicine and Rehabilitation Design Guide

The contractor's floor plan shall meet the design standards in the PACT Space Module Design Guide.

The contractor's facility shall be in compliance with National Fire Protection Association (NFPA) Life/Safety requirements and the Americans with Disabilities Act. VA shall inspect the contractor's facility before contract start date and retains a right of inspection throughout the period of performance during normal business hours of 8:00 AM – 4:30 PM, Monday through Friday. Contractor shall be in compliance with these requirements prior to clinic opening. A list of any deficiencies identified during an inspection will be provided to the contractor along with a required date for correction of the deficiencies. Any planned changes in the physical environment at the outpatient site of care shall be reviewed and approved by the VA to ensure that all life safety codes are met. Parking should be adequate to accommodate patients and shall include a minimum of two (2) handicapped parking spaces.

Automatic Fire Sprinkler System:

- The entire building shall be protected throughout by an automatic fire sprinkler system. Automatic fire sprinkler system(s) shall be installed in accordance with the requirements of NFPA 13, Standard for the Installation of Sprinkler Systems that was in effect on the actual date of installation.
- Automatic fire sprinkler system(s) shall be maintained in accordance with the requirements of NFPA 25, Standard for the Inspection, Testing, and Maintenance of Water-based Fire Protection Systems (current as of Contract Award Date).
- "Equivalent level of safety" means an alternative design or system (which may include automatic fire sprinkler systems), based upon fire protection engineering analysis, which achieves a level of safety equal to or greater than that provided by automatic fire sprinkler systems.
- VA requires a fire alarm system regardless of floor.

Fire Alarm System:

- A Building-wide fire alarm system shall be installed in the entire building.
- The fire alarm system shall be installed in accordance with the requirements of NFPA 72, National Fire Alarm and Signaling Code, that was in effect on the actual date of installation.
- The fire alarm system shall be maintained in accordance with the requirements of NFPA 72, National Fire Alarm and Signaling Code (current as of the Contract Award Date).
- The fire alarm system shall transmit all fire alarm signals to the local fire department via any of the following means: directly to the local fire department, to the (911) public communications center, to a central station, to a remote supervising station, or to a proprietary supervising station.
- If the building's fire alarm control unit is over 25 years old as of the date of award of this contract, contractor shall install a new fire alarm system in accordance with the requirements of NFPA 72, National Fire Alarm and Signaling Code (current as of the Contract Award Date), prior to government acceptance and occupancy of the space.

Access by the government: The government shall have the right to access any space within the building during construction or build out for the purposes of performing inspections or installing Government furnished equipment. The government shall coordinate the activity of government contractors with the contractor to minimize conflicts with and disruption to other contractors on site. Access shall not be unreasonably denied to authorized government officials including, but not limited to, government contractors, subcontractors, or consultants acting on behalf of the government on this project.

Maintenance and Testing of Systems: The contractor is responsible for the total maintenance and repair of the contracted premises. Such maintenance and repairs include the site and private access roads. All equipment and systems shall be maintained to provide reliable, energy efficient service without unusual interruption, disturbing noises, exposure to fire or safety hazards, uncomfortable drafts, excessive air velocities, or unusual emissions of dirt. The contractor's maintenance responsibility includes initial supply and replacement of all supplies, materials, and equipment necessary for such maintenance. Maintenance, testing, and inspection of appropriate equipment and systems shall be done in accordance with current applicable codes, and inspection certificates shall be displayed as appropriate. Copies of all records in this regard shall be forwarded to the government's designated representative administrative specialist, [REDACTED] Frequency of each deliverable shall be established with the government's designated representative. At minimum, systems and equipment for which inspections and reports are required include, but are not limited to, those systems as enumerated in NFPA 99, other applicable NFPA guidelines, and the following:

- Essential Electrical System
- Gas and Vacuum Systems
- Environmental Systems
- Fire Doors and Shutters
- Portable Fire Extinguishers
- Fire Suppression Systems
- Standpipe Systems
- Fire Detection and Alarm Systems

At the contractor's expense, the government reserves the right to require documentation of proper operations, inspection, testing, and maintenance of fire protection systems, such as, but not limited to, fire alarm, fire sprinkler, standpipes, fire pump, emergency lighting, illuminated exit signs, emergency generator, essential power system, prior to occupancy to ensure proper operation. VA has the right to request that these tests shall be witnessed by the government's designated representative.

Janitorial Services and Requirements: The contractor is responsible for maintaining the contracted premises in a condition that meets all housekeeping and sanitization requirements as required by the current healthcare standards (example: Joint Commission, Center for Disease Control and Prevention).

Operating and Maintenance (O&M) Plan: Contractor shall have an O&M plan which shall describe the organization and structure the workforce of both operations and maintenance personnel. The O&M plan shall outline procedures for dealing with scheduled and non-scheduled maintenance and repairs. The O&M plan shall indicate how the scheduled (routine) maintenance and repair of the building will be managed. For each activity in the O&M plan, the contractor shall identify the responsible entity and frequency of the maintenance. O&M shall be broken into the following sections:

- Exterior Maintenance
- Interior Maintenance
- Quality Control
- Non-Scheduled Maintenance
- Emergency Calls
 - Service calls will be classified as emergency calls when the condition constitutes an immediate danger to personnel, threatens to damage property, or has a direct impact on operations or security. Classification of calls as emergency shall be at the sole discretion of the government. During normal working hours, contractor's on-site superintendent shall respond immediately upon notification. Emergencies occurring at other than normal working hours shall be responded to (on scene) within 60 minutes of receipt of call. Contractor shall repair or neutralize the emergency condition before departing the site. If necessary, contractor shall acquire material by quickest available means and

include work outside normal hours. If further labor or materials are required after the emergency is neutralized, the contractor shall have the repairs completed within three (3) business days or within such other reasonable time as agreed to by the government. The contractor shall have available generally used spare parts and common building materials to support emergency requirements.

- Routine Calls
 - Routine calls will be classified as routine when the condition does not qualify as an emergency call. Eighty-five (85%) of all routine calls shall be completed within five (5) business days after the receipt of the call, with remainder being completed within ten (10) business days.

Acceptance of Space and Certificate of Occupancy: The contractor shall issue written notice to the government to schedule inspection of the space for acceptance. The government shall only accept the space if the construction or build-out is conforming to the requirements of the contract. The space shall be considered complete only if the space may be used for its intended purpose and completion of any remaining work will not interfere unreasonably with the intended use of the space as defined under the contract. Acceptance is final with the exception of items identified on a punch list generated as a result of the inspection; concealed conditions, latent defects or fraud shall not relieve the contractor of any other contract requirements.

VA retains a right of inspection of the contractor's facility throughout the period of performance of the contract during normal business hours of 8:00 AM – 4:30 PM, Monday through Friday. A list of any deficiencies identified during an inspection will be provided to the contractor along with a required date for correction of the deficiencies. Any planned changes in the physical environment at the Outpatient Site of Care shall be reviewed and approved by the VA to ensure that all life safety codes, applicable codes and healthcare standards are met.

The contractor shall provide a valid Certificate of Occupancy, issued by the local jurisdiction, for the intended use as outlined in the contract. If the local jurisdiction does not issue Certificate of Occupancies or if the Certificate of Occupancy is not available, the contractor may satisfy this condition by providing a report prepared by a licensed fire protection engineer that indicates the space and building are compliant with all applicable local codes and ordinances and all fire protection and life safety-related requirements of this contract.

Privacy Standards: Veterans shall be provided adequate visual and auditory privacy at check-in. Patient names are not posted or called out loudly in hallways or clinic areas. Veterans shall be provided adequate visual and auditory privacy in the interview area. Patient-identified information shall not be visible in the hall including charts where names are visible. Every effort should be made to restrict unnecessary access to hallways by patients and staff who do not work in that clinic area. Patient dignity and privacy shall be maintained at all times during the course of a physical examination. Examination rooms shall be located in a space where they do not open into a public waiting room or a public corridor. Appropriate locks which allow staff members to have emergency key or code access are required for all examination room doors. Locks shall be installed in all examination rooms in all clinics and outpatient testing or procedure areas, not only those clinical areas primarily serving women. All locks shall be designed to always allow a safe exit from locked rooms without a key or code. Privacy curtains/screens shall be present and functional in examination rooms. Curtains/screens are to ensure privacy from incidental door openings, or from view of others in the room that are not taking part in the examination. Curtains/screens shall fully shield the patient while dressing/undressing, during examination, and offer sufficient work space for the provider to perform the examination. Rooms where a patient would not be expected to disrobe within a private room are exempted from this requirement (for example rooms for dental, audiology, ophthalmology/optometry services). All examination tables shall be placed in such a way that the genital area is not visible from the doorway. Toilet rooms should be located in close proximity to the examination room. Patients who are undressed or wearing examination gowns shall have access without going through public hallways or waiting rooms. Personal hygiene products shall be available in public female, unisex, and family toilet rooms at no charge. Diaper changing tables shall be available in designated public male, female, unisex, and family toilet rooms. Diaper changing tables shall be placed at least one per floor in male, female, and unisex toilet rooms, and no more than 300 feet from areas accessible to a patient. Rooms with changing table shall be identified, and toilet rooms without changing tables should include signage directing users to the nearest changing table. Cameras (telehealth, computer, teaching) shall be shielded/ covered/in locked cabinet/room when not in use.

“Veterans Only” Clinic Requirements for Facilities: To meet VA's requirements for a “Veterans Only” clinic, site cannot be a co-located facility. Space and contracted staff are expected to serve the Veterans associated with this clinic.

Physical Security: The contract clinic site for the VA clinic shall comply with VA Physical Security requirements which may be found in the Physical Security and Resiliency Design Manual located at the following link: <https://www.cfm.va.gov/til/> and with Appendix B of VA Handbook 0730 which defines specifications for physical barrier security, lock set hardware, alarms, and storage containers for high value items and dangerous drugs. In order to comply with Executive Orders, federal laws, and VA policy, space contracted by VA shall meet certain physical security requirements. VA has adopted the protection of the Interagency Security Committee (ISC) Security Design Criteria for all facilities as the minimum requirement.

Panic Alarms: The contractor shall provide a panic alarm system per VA Handbook 0730/4, "Security and Law Enforcement". This system shall provide coverage of entire facility to protect staff in all rooms. This system shall be used to provide rapid notification to onsite staff at the outpatient site of care who will ascertain the need for notification of local law enforcement. The alarm may be activated by a covertly placed switch or button and enough switches/buttons shall be available to personnel receiving patients. The alarm annunciator will be monitored by local staff (and paid for if necessary) by the contractor. The exact location of panic/duress alarm switches shall be determined by a VA police physical security survey of the protected area. VA police will provide annual physical security surveys. Compliance with items marked as "Findings" shall be corrected at the contractor's expense. A written response for any "Findings" shall be sent to VA police within 30 calendar days of receiving the letter. All alarm switches or buttons will be tested once per month by the contract clinic company to ensure operational effectiveness with results provided monthly to the VA police located at the VA Portland Health Care System.

Intrusion Detection System (IDS): At a minimum, there shall be motion detection provided near all entry doors to the clinic from an outside area. Door switch type alarms can also be used in conjunction with the motion detection equipment. It is highly recommended that all IDS be monitored by an outside contracted agency to summon local law enforcement to the outpatient site of care.

Closed Circuit Television (CCTV): Shall be provided by the contractor to monitor building entrances, restricted areas, mission critical asset areas, and alarm conditions. CCTV system shall be used for surveillance and observations of defined exterior areas, such as site and roadway access points, parking lots, and building perimeter, and interior areas such as hallways, common areas and waiting areas. CCTV system will be viewed from a VA location determined by the VA Physical Security Officer. The design, installation, and use of CCTV cameras shall support the visual identification and surveillance of persons, vehicles, assets, incidents, and defined locations. The contractor shall contact [REDACTED] prior to installation of CCTV system to ensure proper placement..

13. **ENVIRONMENT OF CARE (EOC):** Contractor shall meet VHA standards regarding EOC and shall provide EOC management plans addressing safety, security, hazardous materials, hazardous waste, emergency preparedness, life safety, medical equipment, infection control, and utility systems with the submission of their proposal and within 15 calendar days after contract award. The VA parent facility safety officer shall approve the EOC documents prior to commencement of patient care activities at the clinic. The EOC management plans shall be updated annually, along with a summary of performance and opportunities for improvement. Environment of care (EOC) rounds will be conducted at least once per fiscal year (FY) in nonpatient care areas and twice per FY in patient care areas. These rounds will be conducted by VA in accordance with VHA Directive 1608 Comprehensive Environment of Care (CEOC) Program using the Comprehensive Environment of Care Assessment and Compliance Rounding Tool. Contractor shall address deficiencies identified during EOC rounds and ensure they are closed within 14-business days or have a documented plan for action (PFA) for deficiencies that take longer than 14 business days to correct.
14. **EQUIPMENT, OFFICE SUPPLIES AND TECHNICAL SUPPORT:** In accordance with VA and VHA directives, policies, and handbooks, all equipment attaching to a VA network will be owned by the VA and controlled by the VA. No other equipment will be connected to this network. The use of the equipment will be for the benefit of the Government in providing care to our Veterans. The equipment will only be used by those expressly authorized in support of the VA parent facility. All users shall comply with and adhere to VA directives and VA cyber security policies.

The contractor shall be responsible for:

- Providing a current computer aided design (CAD) (or other acceptable file type) drawing of the contract structure and providing updated drawings when changes are made to the space.
- Obtaining and providing upon request, Local Exchange Carrier Right of Entry documentation for installation of voice/data services
- The installation and maintenance of the network cabling infrastructure within the facility including all cabling located inside the walls and plenum space of the structure, cabling racks, network equipment racks, patch panels, wall ports, , and a secure communications closet space to house the networking equipment. Cabling infrastructure and closet design shall adhere to American National Standards Institute (ANSI)/Independent Technical Authority (ITA)-568, International Organization for Standardization (ISO)/International Electrotechnical Commission (IEC) 11801, and VA EHRM standards.
 - Space and pathway from Minimum Point of Entry (MPOE) to IT closet
 - 2 4" conduits from building MPOE to street/carrier entrance point to property
 - Contractor to install and maintain VA approved Uninterruptable Power Supply (UPS)
 - Grounding in the IT closet separate from the building ground IAW Infrastructure Standard for Telecommunications Spaces
 - VA's contracted Enterprise Infrastructure Solutions (EIS) carrier will perform inside wiring as part of the install of telecommunication services.
- VA OIT will provide the primary data and voice circuit.
- The maintenance and on-going technical support for all data and voice wiring within the walls and ceilings from the data closet to the endpoints of the network.
- The procurement, installation and maintenance of other peripheral office equipment and all related and ongoing supplies (paper, toner, ink cartridges) required to operate the equipment in support of the facility under the specifications of this contract. VA Handbook 6500 that requires the following statement on all fax cover sheets be included: "This fax is intended only for the use of the person or office to which it is addressed and may contain information that is privileged, confidential, or protected by law. All others are hereby notified that the receipt of this fax does not waive any applicable privilege or exemption for disclosure and that any dissemination, distribution, or copying of this communication is prohibited. If you have received this fax in error, please notify this office immediately at the telephone number listed above."
- All office supplies (pens, paper, pencils, folders, paper clips) and other supplies to facilitate operation of the clinic.
- All clinical supplies to accomplish all required work in this contract, other than those provided by the VA parent facility specifically mentioned in this contract.
- Having a contingency plan for computer downtime that defines the processes to ensure continuity of patient care and maintenance of the integrity of the patient's medical record during periods of loss of computer functions. The contingency plan shall be reviewed and approved by the CO prior to award of the contract. In addition, a contingency plan template that designates criticality of application/system, estimate of impact, locations of equipment, and contact persons will be provided to the contractor for completion after award of the contract. The contingency plan shall mirror VA OIT DevSecOP's COOP plan. Providing one small desktop color printer for printing patient education information.
- Ensuring hardware/software compatibility with VA approved list: a listing in Attachment D of the printers have passed compatibility testing with the VA EHR Encounter Form. The following scanners listed in Attachment D has passed compatibility testing with the VA EHR imaging system and any other model used will require approval and certification for the VA EHR imaging.

The contractor shall provide the following requirements for network/data rooms: No co-location with other tenants is permissible, Room size: min 10' x 10'. Adequate lighting to work. All equipment will be mounted or stored off the floor. Floor will be tile or painted concrete. Physical and environmental protection shall be in accordance with VA Handbook 6500 Risk Management Framework for VA Information Systems – Tier 3: VA Information Security Program, to include physical and environmental protection policy and procedures, physical access authorizations, physical access control, access control for transmission medium, access control for output devices, monitoring physical access, visitor control, visitor access records, power equipment and cabling, emergency shutoff, emergency power, emergency lighting, fire protection, temperature and humidity controls, water damage protection, delivery and removal, alternate worksite, location of information system components, information leakage, and asset monitoring and tracking.

Door:

- Only access will be through the main door.
- ¾ hour fire rating.
- Steel frame and door, minimum 36" width.
- Magnetic locking mechanism linked to an exterior proximity badge reader with keypad and access logging functionality.
- Hinges on the inside of the room, or if on the outside the pins will be spot welded for security.
- No signage for the room other than a room number if required.

Fire suppression:

- Contractor supplied 10 LB Dry Chemical ABC fire extinguisher mounted on the inside wall of the IT Room.
- If room does not have sprinklers, it shall have a gaseous clean extinguishing system.

Walls/ceiling:

- Provide a one-hour fire rating on all four sides, top, and bottom.
- Inside walls will be sheetrock, shall extend from floor to ceiling.
- Ceiling may be sheetrock, no drop ceilings.
- Painted flat white.
- All data communication lines will be terminated in this room on the back wall or VA approved IT rack.
- This wall will be ¾" plywood.
- Fire retardant rating or painted for fire resistance.

Air exchange:

- Sufficient air exchanges or another acceptable means to cool space.
- Independent air conditioner.
- Maintain the room at 70 degrees.

Power requirements:

- Four (4) L6-20 receptacles.
- One (1) L6-30 receptacle in the closet.
- Each receptacle should be on a separate circuit.
- Receptacles should be located in close proximity to the top or side of the rack, not to impede the walkway or cable management.

Racks:

- Two standard 19" wide data/relay racks; one to support network equipment, one to support PC/Servers, similar to the one displayed below.
- Height from 40-48 RU and a depth no less than 21".
- Secured and grounded to the floor.
- Placed such that there is at least 3 feet in the front of and behind the rack.
- Either aluminum or steel construction and the holes should be standard 10-32 tapped.
- Both racks will have 2 shelves on the bottom installed to hold IT computers.
- Infrastructure installer should install CAT6A patch panels and connectors in the rack.

Cable management:

- Cables should be installed to the patch panel at the hinged end so that the articulation of the panel doesn't stress the terminations.
- Vertical cable trough on both sides of rack.
- Horizontal cable troughs between patch panels.
- Cable ladder assembly above rack for cable into closet to patch panels.
- "B" connection terminated at each end (wall jack & communications closet, where applicable) IAWTIA/EIA standards. Contractor shall provide testing and acceptance results to the VA Project Manager/VA IT Area Manager.

The VA parent facility will be responsible for:

- Providing PC workstations, software, primary telecommunications lines and networking equipment required to access the VA EHR.
- Providing antivirus software for PC workstations and ensure that data definition files are current. In addition, the VA parent facility will ensure that all Microsoft critical updates and patches are current.
- Providing the primary data (including internet) and voice services..
- Providing the backup, contingency, COOP connectivity to the VA and will be established through a VA parent facility provided site-to-site VPN connection
- Providing advisory technical support to the contractor's technical support person for the initial site set-up relative to the VA EHR and VPN connectivity. The VA parent facility will provide on-going technical support for the VA EHR software and any other VA software applications. Technical support will be through an escalation process. The contractor's technical representative will submit a "Help Desk" request by calling [REDACTED]. Initial technical support will be provided by the VA via telephone, which will consist of a VA technical representative speaking to a contractor employed representative to identify the problem, trouble-shoot and attempt to resolve the problem with the contractor's end-user. If the problem cannot be resolved the VA will provide on-site support for VA owned equipment, VA EHR software and other VA software applications, if necessary, within two business days or less depending on the nature and severity of the problem.
- Providing network equipment such as WAN router, LAN switches, monitoring devices (such as Ciena and Uplogix boxes) will be procured, installed, and maintained by VA parent facility OIT.

15. PERFORMANCE STANDARDS, QUALITY ASSURANCE AND QUALITY IMPROVEMENT:

Services and documentation of care provided under the contract shall be subject to quality management and safety standards as established by VA, consistent with VA policies. The contractor shall develop and maintain quality improvement/quality assurance programs and provision of care equal to or exceeding VA Standards. The results of all quality improvement activities performed by the contractor involving VA parent facility patients will be shared with VA Quality Management Office.

Documentation by the contractor provided to the VA parent facility includes, but is not limited to the following:

Quality improvement plans: Staff meetings minutes (or summary minutes) where quality improvement has been discussed and which include practitioner-specific findings, conclusions, recommendations, and written plans for actions taken in response to such conclusion and recommendations, and evaluation of those actions taken.

Contractor shall meet or exceed VA policies and health care accreditation requirements in VHA Directive 1100.16. Documentation verifying accreditation requirements shall be provided by the contractor to the CO prior to award and annually during contract performance.

The contractor shall notify the VA parent facility Chief of Staff (COS), COR, and CO in writing whenever a malpractice claim involving a VA patient has been filed against the contractor. The contractor shall forward a copy of the malpractice claim to the VA parent facility COS within three (3) business days after receiving notification that a claim has been filed. The VA parent facility COS will notify the CO of any information received from the contractor which impact contractor personnel ability to perform under the contract.

The contractor shall also notify the VA parent facility COS, COR, and CO in writing when any provider furnishing services under this contract is reported to the National Practitioner Data Bank. This notification will include the name, title, and specialty of the provider. The VA parent facility COS will notify the CO of any information received from the contractor which impact contractor personnel ability to perform under the contract.

The contractor shall permit on-site visits by VA personnel and accompanied by VA personnel and/or other accrediting agencies to assess contracted services, e.g., adequacy, compliance with contract requirements, record-keeping, etc.

The contractor is responsible for the quality management plan for monthly clinical pertinence review of ambulatory care records. The results shall be forwarded to [REDACTED]

[REDACTED] If during VA business, a concern is identified, the issues shall be addressed by the contractor and a performance improvement plan initiated.

Recommendations and implementation of performance improvement activities will be the responsibility of the Medical Director of the clinic.

The VA is committed to providing high quality primary care. VHA's Office of Quality and Patient Safety provides tools and intelligence for gauging and improving care for Veterans. Several "process" and "outcome" measures are tracked using various methods and programs, to include but not limited to, the External Peer Review Program (EPRP), the Survey of Healthcare Experience of Patients (SHEP), the Senior Executive Services (SES) Performance Plan, and the Clinical Performance Measurement Program. These efforts serve to strengthen accountability, efficiency, transparency, and benchmarking, and to support continuous improvement in care. The contractor is responsible for achieving levels of performance on these measures that meet or exceed the performance of the VA parent facility. The contractor is required to utilize the VA EHR clinical reminder system as a means of both ensuring high performance on many of these measures and to facilitate monitoring of performance at the site. Levels of performance on these measures will be used as a factor in decisions about renewal of the contract.

The contractor shall document in writing on appropriate orientation programs for all staff involved in the delivery of patient care, e.g., infection control procedures, patient confidentiality, handling emergencies, patient safety, etc., and provide a copy to the VA parent facility COR. Contractor shall be required to furnish method/guidelines by which he/she intends to meet above requirement.

The contractor shall have a quality monitoring/performance improvement program. This program shall be available to VA parent facility staff and accrediting agencies. The VA parent facility will provide regular feedback on clinic performance measures, including but not limited to the following: licensure verification, workload, consults, drug and lab utilization, formulary compliance, prescription writing patterns, prevention and performance measures, patient satisfaction, and medical record completeness. The contractor shall conduct audits pertaining to access, quality improvement, documentation, safety, and performance measures. These reports shall be submitted to the COR monthly and sent via secured email using PKI or utilizing a secure delivery service that tracks mail from pick-up to delivery.

The contractor shall comply with all PBM formulary guidance regarding medication use, monitoring and safety.

The contractor shall collaborate with VA parent facility pharmacy when patients are identified that require intervention.

The contractor shall meet all Federal, State, and local fire and life safety codes.

The contractor shall be responsible for meeting national quality standards and shall comply with mandated policies established by VA.

The contractor shall participate with local facility quality improvement, radiation safety and MRI safety committees, as appropriate.

16. **PERFORMANCE STANDARDS AND SURVEILLANCE:** If any portion of the requirement is not met and/or the acceptable quality level in any of the elements in this section/subsection are not maintained, the contractor will be notified by the CO for response outlining the contractor's proposed remedy. The Government may seek an equitable price reduction or adequate consideration for acceptance of nonconforming services per FAR 52.212-4(a) **NOTE:** Any change to the level of performance (standard/AQL) after award of the contract shall be conducted via bi-lateral modification. Please consider this when setting AQLs.

Domain: Access

Measure: New PC Appointments Completed Within 20 Calendar Days Of Reference Date (RD) (New primary care appointments completed within 20 calendar days of RD)

Performance Requirement: Contractor shall provide completed appointments for new PC patients within 20 calendar days of the RD.

Standard: Contractor shall provide completed appointments for new PC patients within 20 calendar days of the RD no less than 53% of the time.

Acceptable Quality Level: Contractor shall provide completed appointments for new PC patients within 20 calendar days of the RD no less than 53% of the time.

Surveillance Frequency and Method: VA COR will monitor quarterly using VSSC (Veteran Support Service Center) data reporting. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Established PC Patient Appointments Completed Within 20 Calendar Days Of Patient Indicated Date (PID) (Established primary care appointments completed within 20 calendar days of PID)

Performance Requirement: Contractor shall provide completed appointments for established PC patients within 20 calendar days of the PID.

Standard: Contractor shall provide completed appointments for established PC patients within 20 calendar days of the PID no less than 93% of the time.

Acceptable Quality Level: Contractor shall provide completed appointments for established PC patients within 20 calendar days of the PID no less than 93% of the time.

Surveillance Frequency and Method: VA COR will monitor quarterly using Clinic Proactive Management Report (CPM). COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: PCMHI Penetration (PACT21) (The percent of assigned Primary Care patients seen in Primary Care- Mental Health Integration (PCMHI))

Performance Requirement: Contractor shall involve PCMHI staff in the care of patients with mental health needs.

Standard: Contractor's PCMHI penetration rate shall meet or exceed 9%.

Acceptable Quality Level: Contractor's PCMHI penetration rate meets or exceeds 9 %.

Surveillance Frequency and Method: VA COR will monitor progress quarterly using data from the PACT Compass. COR will send reports to the contractor and CO quarterly to notify them of current performance.

Measure: PCMH SHEP Access Composite (Composite % based on 3 questions from the SHEP/PCMH report: 1) Get an urgent care appointment as soon as needed, 2) Get a routine care appointment as soon as needed, and 3) Get same day answer to your medical question.)

Performance Requirement: Contractor shall provide: 1) urgent care appointments as soon as needed, 2) routine care appointments as soon as needed, and 3) same day answers to patients' medical questions.

Standard: Contractor's SHEP access composite score shall meet or exceed 49%.

Acceptable Quality Level: Contractor's SHEP access composite score shall meet or exceed 49%.

Surveillance Frequency and Method: VA will monitor progress: COR will monitor Quarterly by Primary Care Data Analytics team using SHEP report. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: SHEP Question 7 (In the last 6 months, how many days did you usually have to wait for an appointment when you needed care right away? (Less is better))

Performance Requirement: Contractor shall provide timely appointments when patients need care right away.

Standard: Contractor's SHEP question 7 score shall meet or exceed 33%.

Acceptable Quality Level: Contractor's SHEP question 7 score shall meet or exceed 33%.

Surveillance Frequency and Method: VA COR will monitor progress quarterly using data from SHEP/PCMH report. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Same-Day Appts w/ PCP (Measures the percent of requested same day appointments with PCP)
Performance Requirement: Contractor shall provide same day appointments with primary care provider.
Standard: Contractor's same day appointments with PCP rate shall meet or exceed 43%.
Acceptable Quality Level: Contractor's same day appointments with PCP rate shall meet or exceed 43%.
Surveillance Frequency and Method: VA will monitor progress: COR will monitor Quarterly by PACT Compass Dashboard. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Appointment Cancellations (MOP13) (Cancelled by clinic rate for all active clinic stop codes (mop13) (Lower is better))

Performance Requirement: Contractor shall not unnecessarily cancel patient appointments.
Standard: Contractor's rate of appointments cancelled by clinic shall not exceed 9%.
Acceptable Quality Level: Contractor's rate of appointments cancelled by clinic shall not exceed 9%.
Surveillance Frequency and Method: VA COR will monitor progress quarterly using data from the Performance Measure Report in VSSC. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Domain: Quality of Care

Measure: Composite BH90 - Behavioral Health Screening (Composite of mdd40 -- Vets screened annually for major depression dx.ptsd51 -- PTSD screening using the PC-PTSD at required times sa17 -- Vets screened for alcohol misuse w/ score GE 5 w/ timely brief counselling sa7 -- Vets screened annually for alcohol misuse CSSRS1--Annual Suicide Risk Screen)

Performance Requirement: Contractor shall complete all behavioral health screening and follow-up evaluations.

Standard: Contractor shall maintain a behavioral health composite score of at least 96%.

Acceptable Quality Level: Contractor shall maintain a behavioral health composite score of at least 96%.

Surveillance Frequency and Method: VA will monitor progress COR will monitor Quarterly by Quality CBOC Report. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Composite CVRM90 EC – Cardiovascular/Diabetes Risk Management (EQM) (Composite of dm27h_ec (DM: BP LT 140/90 (eMeasure)) ihd53h_ec (Controlling High Blood Pressure (eMeasure)) statn1_ec (Statin Therapy for patients with cardiovascular disease (eMeasure)) statn7_ec (Statin therapy for patients with diabetes (eMeasure))

Performance Requirement: Contractor shall manage patients with diabetes and cardiovascular disease in accordance with VA standards.

Standard: Contractor shall maintain a cardiovascular/diabetes risk management composite score of at least 73%.

Acceptable Quality Level: Contractor shall maintain a cardiovascular/diabetes risk management composite score of at least 73%.

Surveillance Frequency and Method: VA will monitor progress: COR will monitor Quarterly by Quality CBOC Report. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Composite PRV90 EC – Prevention (Composite of bcs01_ec (Breast Cancer Screening for Women 50-74y (Electronic)) ccs01_ec (Cervical Cancer Screening (eMeasure)) col01_ec (Colorectal Cancer Screening Ages 45-75 (eMeasure)) psa1_ec (Non-Recommended PSA-Based Screening (eMeasure))

Performance Requirement: Contractor shall provide preventive health care in accordance with VA standards.

Standard: Contractor shall maintain a prevention composite score of at least 63%.

Acceptable Quality Level: Contractor shall maintain a prevention composite score of at least 63%.

Surveillance Frequency and Method: VA will monitor progress: COR will monitor Quarterly by Quality CBOC Report. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Composite SMG90_1 – Tobacco Use And Cessation (Composite of p7 (Tobacco Use - Screened for Use - NEXUS (Outpt)) smg10 (Tobacco Use Cessation - Discussed Cessation Medications (Outpt)) smg8 (Tobacco Use Cessation - Advised to Quit (Outpt)) smg9 (Tobacco Use Cessation - Discussed Cessation Strategies (Outpt))

Performance Requirement: Contractor shall provide tobacco counseling and care in accordance with VA standards.

Standard: Contractor shall maintain a tobacco use and cessation composite score of at least 96%.

Acceptable Quality Level: Contractor shall maintain a tobacco use and cessation composite score of at least 96%.

Surveillance Frequency and Method: VA will monitor progress: COR will monitor progress Quarterly by Quality CBOC report. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Composite FLU90_EC – Outpatient Influenza Immunization (Composite of p28h_ec (Influenza Immunizations 66 years of age and older (eMeasure)) p29h_ec (Influenza Immunization 19-65 years of age (eMeasure))

Performance Requirement: Contractor shall provide flu immunizations in accordance with VA standards.

Standard: Contractor shall maintain an influenza immunization composite score of at least 40%.

Acceptable Quality Level: Contractor shall maintain an influenza immunization composite score of at least 40%.

Surveillance Frequency and Method: VA will monitor progress, COR will monitor progress quarterly using data from the Outpatient Sites of Care Report. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Composite AIS90_EC– Adult Immunization Status (Composite of ais01_ec Pneumococcal Immunization (eMeasure) ais10td_ec Tdap Immunization (eMeasure) ais20zos_ec Zoster Immunization (eMeasure))

Performance Requirement: Contractor shall provide adult immunizations in accordance with VA standards.

Standard: Contractor shall maintain an adult immunization status composite score of at least 58%.

Acceptable Quality Level: Contractor shall maintain an adult immunization status composite score of at least 58 %.

Surveillance Frequency and Method: VA will monitor progress, COR will monitor progress quarterly using data from the Outpatient Sites of Care Report. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Composite DMG91_EC – Diabetes (Composite of dmg23h_ec (DM: HbA1c poor control (eMeasure)) dmg31h_ec (Diabetes Mellitus - Outpatient - Retinal exam, timely by disease (eMeasure)) ked01_ec (Kidney Health Evaluation for Patients with Diabetes (eMeasure))

Performance Requirement: Contractor shall provide diabetes care in accordance with VA standards.

Standard: Contractor shall maintain a diabetes composite score of at least 70%.

Acceptable Quality Level: Contractor shall maintain a diabetes composite score of at least 70%.

Surveillance Frequency and Method: VA will monitor progress, will monitor progress quarterly using data from the Outpatient Sites of Care Report. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Clinical Reminders

Performance Requirement: Contractor shall complete all clinical reminders as indicated in the VA EHR.

Standard: Contractor shall complete no less than 90% of all clinical reminders as indicated in the VA EHR.

Acceptable Quality Level: Contractor shall complete no less than 90% of all clinical reminders as indicated in the VA EHR.

Surveillance Frequency and Method: VA will monitor progress, COR will monitor progress weekly using VA EHR reports. COR will send reports weekly to the contractor and CO to notify them of current performance. COR will send reports to the Contractor and CO quarterly to notify them of current performance.

Measure: Progress Notes And Patient Care Encounter (PCE) Module Documentation

Performance Requirement: Contractor shall complete workload and encounter documentation as defined in the PWS within 2 business days. Progress notes shall be connected to the correct visit/encounter and location, entered into the VA EHR on the same day as the visit/encounter, meet CMS guidelines, and include content as defined in PWS.

Standard: Contractor shall properly complete documentation for each patient encounter within 2 business days 100% of the time.

Acceptable Quality Level: Contractor shall properly complete documentation for each patient encounter within 2 business days 100 % of the time.

Surveillance Frequency and Method: VA COR will monitor progress weekly using VA EHR reports. COR will send reports quarterly to the Contractor and CO to notify them of current performance.

Domain: Panel Management

Measure: Percent Of Teams With Core Teamlet Staffing Ratio 3 or More (PACT 22) (The total Core Primary Care Support Staff (administrative associate, RN care manager, and clinical associate) FTE divided by the total Primary Care Provider FTE entered in the PCMM Web Application)

Performance Requirement: Contractor shall provide core teamlet staffing at a ratio of 3 or more. FTE core teamlet members (RN care manager, clinical associate, administrative associate) for each PCP FTE

Standard: 100% of contractor's primary care teams shall have a core teamlet staffing ratio of 3 or more.

Acceptable Quality Level: 99% of contractor's primary care teams shall have a core teamlet staffing ratio of 3 or more.

Surveillance Frequency and Method: VA will monitor progress, COR will monitor progress monthly using data from the PACT Compass. COR will send reports monthly to the contractor and CO to notify them of current performance. COR will send reports quarterly to the Contractor and CO to notify them of current performance.

Measure: Percent Of Women Assigned To Women's Health PACT Or Designated Women's Health Provider (WH1) (The total number of female patients assigned to a Women's Health PACT (identified by having *WH* in the team name or a primary or secondary team focus of Women's Health) OR to a provider with a team role of designated WH provider (WH-PCP) divided by the total number of unique female patients assigned to Primary Care Teams)

Performance Requirement: Contractor shall ensure that an appropriate number of WH-PACTs or WH-PCPs are available at site of care to ensure that all VHA access goals are met for women patients who choose to be seen by a designated WH provider.

Standard: Contractor's percent of women assigned to WH-PACT or designated WH provider shall meet or exceed 88%.

Acceptable Quality Level: Contractor's percent of women assigned to WH-PACT or designated WH provider shall meet or exceed 88%.

Surveillance Frequency and Method: VA will monitor progress, COR will monitor progress quarterly using data from the PACT Compass. COR will send reports to the contractor and CO quarterly to notify them of current performance.

Standard: Contractor shall exceed 53% ratio of non-traditional encounters.

Acceptable Quality Level: Contractor shall exceed 53% ratio of non-traditional encounters.

Surveillance Frequency and Method: VA will monitor progress, COR will monitor progress quarterly using data from the PACT Compass. COR will send reports to the contractor and CO quarterly to notify them of current performance.

Measure: Ratio Of Non-Traditional Encounters (This is the sum of all PC Telephone encounters added to the sum of all PC Group Encounters (including Shared Medical Appointments for Primary Care and Shared Medical Appointments for Primary Care-Mental Health Integration) added to the sum of all Virtual Video Connect encounters added to the sum of all incoming and outgoing secure messages as the numerator. The denominator is the total number of team assignments for the reporting period.)

Performance Requirement: Contractor shall provide telephone encounters, group encounters, and secure messaging.

Standard: Contractor shall exceed 53% ratio of non-traditional encounters.

Acceptable Quality Level: Contractor shall exceed 53% ratio of non-traditional encounters.

Surveillance Frequency and Method: VA will monitor progress, COR will monitor progress quarterly using data from the PACT Compass. COR will send reports to the contractor and CO quarterly to notify them of current performance.

Domain: Veteran Satisfaction

Measure: Veteran Satisfaction (Overall rating of Primary Care Provider on question 32 of the SHEP/PCMH survey)

Performance Requirement: The contractor's PCPs shall provide care that patients rate as a 9 or 10 on question 32 of the SHEP/PCMH survey.

Standard: Contractor's SHEP question 32 score shall meet or exceed 73%.

Acceptable Quality Level: Contractor's SHEP question 32 score shall meet or exceed 73%.

Surveillance Frequency and Method: VA will monitor progress, COR will monitor progress quarterly using data from SHEP/PCMH report. COR will send reports to the contractor and CO quarterly to notify them of current performance.

Domain: Coordination of Care

Measure: Post Discharge Contact By PACT Teamlet (The percent of assigned primary care patients discharged from any VA facility who have been contacted by PACT teamlet within two business days)

Performance Requirement: The contractor's PACT teamlets shall contact their assigned patients within 2 business days of discharge from any VA facility.

Standard: Contractor's rate of post discharge contact by PACT teamlet within 2 business days shall meet or exceed 61%.

Acceptable Quality Level: Contractor's rate of post discharge contact by PACT teamlet within 2 business days shall meet or exceed 61 %

Surveillance Frequency and Method: VA will monitor progress, COR will monitor progress quarterly using data from the PACT Compass. COR will send reports to the contractor and CO quarterly to notify them of current performance.

Domain: Environment of Care (EOC)

Measure: Percentage Of Deficiencies Identified During EOC Rounds That Are Closed Timely (e5eoc1) (Percentage of deficiencies identified during EOC rounds that are closed within 14-business days or have a documented Plan for Action (PFA))

Performance Requirement: Contractor shall address deficiencies identified in EOC rounds within 14 business days or have a documented PFA.

Standard: Contractor's rate of EOC deficiencies that are closed within 14 business days or have a PFA shall meet or exceed 93%.

Acceptable Quality Level: Contractor's rate of EOC deficiencies that are closed within 14 business days or have a PFA shall meet or exceed 93 %

Surveillance Frequency and Method: VA will monitor progress , COR will monitor progress quarterly using data from the Performance Measure Report. COR will send reports to the contractor and CO quarterly to notify them of current performance.

Domain: Pharmacy

Measure: Medication Storage Area Inspections

Performance Requirement: Contractor shall review medication storage areas monthly.

Standard: The contractor shall ensure that 100% of medication storage areas are inspected monthly and documented on VA Form 10-0053 (or VA parent facility form). Inspection documentation will be sent to the COR within 10 calendar days of the end of each month. Any issues discovered shall be resolved by the contractor and reported to the COR within 15 calendar days of the inspection date.

Acceptable Quality Level: The contractor shall ensure that 99% of medication storage areas are inspected monthly and documented on VA Form 10-0053 (or VA parent facility form). Inspection documentation will be sent to the COR within 10 calendar days of the end of each month. Any issues discovered are resolved by the contractor and reported to the COR within 15 calendar days of the inspection date.

Surveillance Frequency and Method: VA will monitor progress, COR will monitor performance via receipt of VA Form 10-0053 and documented resolution of any issues discovered during inspections. COR will send reports to the contractor and CO quarterly to notify them of current performance.

17. REQUIRED REGISTRATION WITH CONTRACTOR PERFORMANCE ASSESSMENT REPORTING SYSTEM (CPARS)

As prescribed in Federal Acquisition Regulation (FAR) Part 42.15, the Department of Veterans Affairs (VA) evaluates contractor past performance on all contracts that exceed the simplified acquisition threshold (SAT) as defined in FAR 2.101 and shares those evaluations with other Federal Government contract specialists and procurement officials. The FAR requires that the contractor be provided an opportunity to comment on past performance evaluations prior to each report closing. To fulfill this requirement VA uses an online database, CPARS, which is maintained by the Naval Sea Logistics Center in Portsmouth, New Hampshire. CPARS has connectivity with the Past Performance Information Retrieval System (PPIRS) database, which is available to all Federal agencies. PPIRS is the system used to collect and retrieve performance assessment reports used in source selection determinations and completed CPARS report cards transferred to PPIRS. CPARS also includes access to the federal awardee performance and integrity information system (FAPIIS). FAPIIS is a web-enabled application accessed via CPARS for contractor responsibility determination information.

Each contractor whose contract award is estimated to exceed the SAT is required to register with CPARS database at the following web address: <https://www.cpars.gov/>. Help in registering can be obtained by contacting customer support desk @ DSN: 684-1690 or COMM: 207-438-1690. Registration should occur no later than thirty calendar days after contract award and shall be kept current should there be any change to the contractor's registered representative.

For contracts with a period of one year or less, the CO will perform a single evaluation when the contract is complete. For contracts exceeding one year, the CO will evaluate the contractor's performance annually. Interim reports will be filed each year until the last year of the contract, when the final report will be completed. The report shall be assigned in CPARS to the contractor's designated representative for comment. The contractor representative will have thirty calendar days to submit any comments and re-assign the report to the VA CO.

Failure to have a current registration with the CPARS database, or to re-assign the report to the VA CO within those thirty calendar days, will result in the Government's evaluation being placed on file in the database with a statement that the contractor failed to respond.

18. GOVERNMENT RESPONSIBILITIES:

Oversight of Service/Performance Monitoring:

CO Responsibilities: The CO is the only person authorized to approve changes or modify any of the requirements of this contract. The contractor shall communicate with the CO on all matters pertaining to contract administration. Only the CO is authorized to make commitments or issue any modification to include (but not limited to) terms affecting price, quantity, or quality of performance of this contract. The CO shall resolve complaints concerning contractor's provider relations with the Government employees or patients. The CO is final authority on validating complaints. In the event the contractor effects any such change at the direction of any person other than the CO without authority, no adjustment shall be made in the contract price to cover an increase in costs incurred as a result thereof. If contracted services do not meet quality and/or safety expectations, the best remedy will be implemented, to include but not limited to a targeted and time limited performance improvement plan; increased monitoring of the contracted services; consultation or training for the contract staff to be provided by the VA parent facility; replacement of the contract staff or termination of the contract.

COR Responsibilities: The COR shall be the VA parent facility official responsible for verifying contract compliance. After contract award, any incidents of contractor or contractor's provider noncompliance as evidenced by the monitoring procedures shall be forwarded immediately to the CO. The COR will be responsible for monitoring the contractor staff performance to ensure all specifications and requirements are fulfilled. Quality improvement data that will be collected for ongoing monitoring is outlined in the QASP. The COR will maintain a record-keeping system of services by reviewing the QASP and invoices submitted by the contractor. The COR will review this data monthly, cross reference with the invoice when received and certify all invoices for payment. Any evidence of the contractor's non-compliance shall be forwarded immediately to the CO.

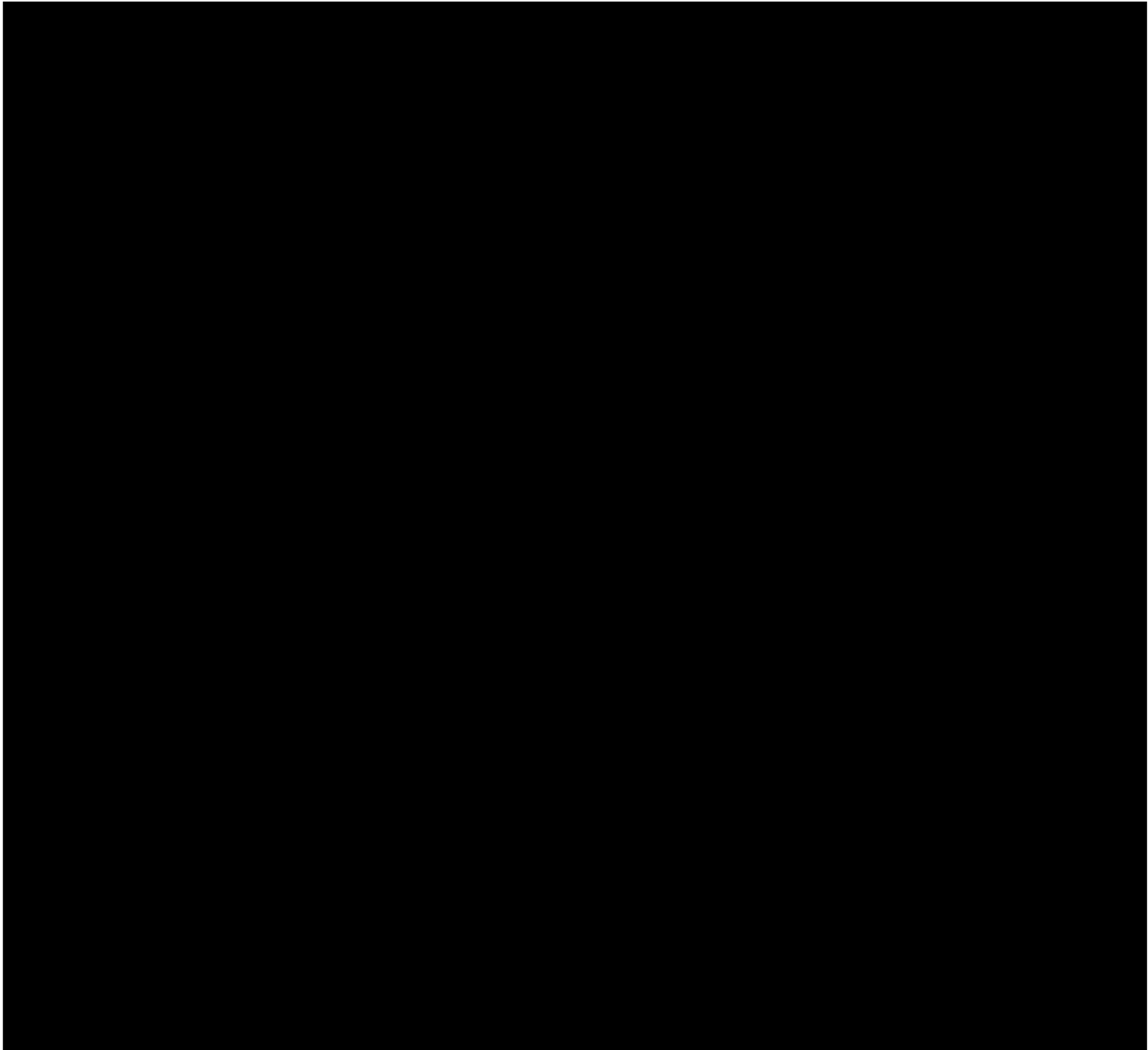
Contract Administration: All contract administration functions will be retained by the VA. After award of contract, all inquiries and correspondence relative to the administration of the contract shall be addressed to:

Contracting Officer (CO) will be identified at time of award.

Contracting Officer's Representative (COR) will be identified at time of award

Liaison persons: While the liaison persons identified and other VA parent facility staff may be contacted for questions/information and/or may visit the contractor's sites to oversee policy compliance, **only the CO is authorized to make commitments or issue changes which will affect the price, quantity, quality, or delivery terms of this contract.** Any guidance provided, which the contractor feels is beyond the scope of this contract, shall be communicated to the CO, via the COR, for possible contract modification.

The VA parent facility has designated the following liaison personnel for this contract:



The contractor shall identify and provide contact information for a contact person(s), who shall serve as liaison between the contractor and the VA parent facility. This individual will also ensure the functionality of the clinic per the contract PWS. The contact person(s) will be available during the administrative tour of duty from 8:00 a.m. - 4:30 p.m. Monday through Friday. The contractor shall also provide contact information for an after-hours point of contact who shall be reachable by phone at all times.

19. SPECIAL CONTRACT REQUIREMENTS:

Contract Start-up Requirements:

- The contractor's start-up requirements shall be completed prior to the commencement of the contractor's treatment of VA patients. Upon approval by the VA parent facility of the contractor's completion of the start-up requirements, the VA parent facility will issue a written notice to proceed to the contractor.
- The contractor shall have 90 calendar days from contract award to commencement of the provision of medical care to VA patients. However, the contractor shall have all start-up requirements in place and ready to commence operation NLT 83 calendar days from contract award. The final seven (7) calendar days will be used for training and resolution of any last minute or unexpected technical or personnel related challenges. The contractor shall comply with the following contract requirements prior to commencement of clinical operations:
 - The contractor shall hire, train, and ensure licensure of all necessary personnel.
 - The contractor shall furnish evidence of insurability of the offeror and/or of all health-care providers, who will perform under this contract (see VAAR 852.237-70, Indemnification and Medical Liability Insurance).
 - All contractor-provided health care services shall be available.
 - The contractor's case management program with primary care providers as case managers for all health care services provided to VA parent facility patients shall be operational.
 - The contractor's VA parent facility approved performance improvement program shall be operational.
 - The contractor's facility shall be compliant with the requirements of this contract.

The VA parent facility will provide training to the contractor at the VA parent facility relative to data reporting needs, the VA EHR, eligibility issues, billing procedures and medical referral procedures within 89 calendar days of contract award. The contractor is responsible to provide future training to their personnel after the initial 90 calendar days of the contract award. The contractor shall provide documentation of training prior to pathology and laboratory medicine providing access to the VA EHR laboratory software options. The contractor shall be responsible for attendance and performance regarding training sessions. Training will be coordinated by the COR and the contractor's designee. After contract performance begins, VA parent facility staff is readily available by telephone and e-mail to answer questions and provide guidance.

Upon receipt of notice of award, contractor shall immediately commence the credentialing and privileging process for all clinical staff through the VA parent facility. A minimum of six (6) calendar weeks is required for VA parent facility credentialing after the package has been completed and received from the contractor's provider.

Signage: The contractor shall furnish and install clearly visible signage in accordance with the VA Signage Design Manual which is attached under Section D. The signage shall include the name of the clinic which is Lincoln County Oregon VA Clinic.

20. BILLING AND INVOICING PROCEDURES:

Billable Roster: The billable roster is utilized to track Veterans who meet the billing criteria for reimbursement under the PMPM methodology based on qualifying encounters. The roster is applicable to both PC and MH LINS.

Additions to the Billable Roster:

VA parent facility has the sole authority to assign patients who are treated by the contractor into the PCMM enterprise application. The contractor is responsible for notifying the VA parent facility of newly seen patients at the contractor's site that are not already assigned in PCMM. The VA parent facility will then verify that the patient was seen through the VA EHR documentation and enter the patient into PCMM as credited to the contractor's site and associated clinic roster upon verification with the patient of their intent to establish care at the contractor's site.

If the contractor seeks to request placement on the billable roster a patient at the contractor's site who is already assigned to another primary care team or provider in the VHA, the VA parent facility will have final authority to

assign the primary care site for the patient. The main basis for this decision will be patient preference. Patients shall not be allowed to be assigned to more than one VA Outpatient Site of Care. In addition, patients will not be allowed to be assigned simultaneously at the contractor's site and in any of the primary care teams at the VA parent facility. If the patient has an active PCMM assignment at another location, care provided will fall under the requirements in the episodic care for unassigned/unenrolled patients section of this contract. If the patient is requesting transfer of care to the contractor's site, VA parent facility will review the request in accordance with the VHA Directive 1406 Patient Centered Management Module (PCMM).

Note: Newly assigned patients who have not had a qualifying encounter with the contractor's PCP and 12 months have passed since the time of assignment to that PCP will be automatically inactivated in PCMM due to inactivity. Established/active patients that have not had a qualifying encounter with the contractor's PCP in the past 36 months will be considered inactive and the assignment will be automatically inactivated in PCMM. Billable rosters and PCMM assignments are different processes. Patients shall not be included on the monthly billable roster if they have not had a qualifying encounter with the contractor's PCP in the previous 12 months but shall not be unassigned in PCMM from the contractor's site of care just because they have not had a qualifying encounter with the contractor's PCP in the previous 12 months. The contractor is still responsible for providing care to these patients until the patients are automatically inactivated due to inactivity or there is a reason (such as death of the patient) to unassign them.

Primary Care LIN:

The billable roster for the PC LIN will include all patients assigned to the contractor's site of care in the PCMM with at least one qualifying encounter by the contractor's PCP within the previous 12 months. For the purposes of this contract, a qualifying encounter is a completed encounter by a PCP and shall have one of the following primary stop codes and **ANYTHING** in secondary credit code:

- 323 (Primary Care Medicine)
- 338 (Telephone Primary Care) Note: The use of this stop code is only appropriate for established patients.
- 348 (Primary Care Shared Appointment)
- 322 (Comprehensive Women's and Gender Diverse Primary Care Clinic)
- 350 (GeriPACT)
- 326 (Telephone/Geriatrics) Note: The use of this stop code is only appropriate for established patients.
- 32A (Primary Care Cerner)

OR ANYTHING in primary stop code that is not on the list above and credit stop is:

- 179 (Real Time Clinical Video Telehealth to Home Provider Site)
- 322 (Comprehensive Women's and Gender Diverse Primary Care Clinic)
- 323 (Primary Care Medicine)
- 350 (GeriPACT)
- 32A (Primary Care Cerner)

Mental Health LIN:

The billable roster for the MH LIN (if there is a MH LIN) will include all patients provided at least one primary care-mental health integration (PCMHI) and/or general/specialty MH service by the contractor's professional MH staff within the previous 12 months under a 500 series stop coded clinic with a separate CPT coded encounter.

The ONLY time the contractor may bill against the MH LIN is when PCMHI, and/or general/specialty MH services are provided by the contractor's professional MH staff under the 500 series (MH) stop coded clinics with separate CPT coded encounters. MH screening and services provided as part of primary care in a 300 series stop coded clinic are NOT billed against the MH LIN.

"Professional MH Staff" eligible to provide services that will result in billing against the MH LIN are psychologist, psychiatrist, licensed social worker practicing at the independent level, MH nurse practitioner (NP), mental health physician assistant (PA), licensed professional mental health counselor (LPMHC) and marriage and family therapist.

Removal from Billable Roster:

Removal of patients from the billable roster may include, but not be limited to, the following reasons:

- The patient loses eligibility for VA care.
- The VA parent facility decides that removal from the billable roster is in the best interest of the patient.
- The patient was found to have falsified the application for VA services, and approval was based on false information.
- When it is determined that a patient has abused the VA system by allowing an ineligible person to utilize the patient's identification card to obtain services.
- When it is determined that the patient has willfully and repeatedly refused to comply with the contractor's requirements or VA parent facility requirements, subject to federal laws and regulations.
- When it is determined that the patient has abused the VA program by using VA identification card to seek or obtain drugs or supplies illegally or for resale, subject to state and federal laws and regulations.
- When it is determined by VA's Disruptive Behavior Committee that it is not safe for the patient to receive care at contractor's site of care.
- The contractor gives written notification to the VA parent facility that the contractor cannot provide the necessary services to the patient or establish an appropriate provider patient relationship.
- If a patient has a legitimate complaint and requests to be unassigned from the contractor's site of care.
- Death of the patient.
- When a patient moves to another area.
- When a patient receives their primary care elsewhere.
- When a patient assigned to the contractor's site of care in the PCMM does not receive at least one qualifying encounter by the contractor's PCP within the previous 12 months.

NOTE: *The contractor shall notify VA parent facility of any of the above reasons. These circumstances may become known after the fact. Upon discovery of these situations, the contractor shall credit or reimburse the VA parent facility back to the original date of the removal criteria being met for reasons above.*

The VA parent facility has ultimate authority to remove a patient from the billable roster and from the responsibility of the contractor at any time. The VA parent facility will notify the patient and the contractor of the effective date of removal from the billable roster. If the contractor disagrees with a removal from the billable roster, the issue will be referred to the CO for resolution.

Monthly Billable Roster and Invoice Reconciliation: Monthly billable roster and invoice reconciliation shall take place as follows:

- The VA parent facility shall present to the contractor the VA parent facility billable roster for the applicable month to be invoiced.
- The contractor shall reconcile the VA parent facility billable roster with its records, negotiate any differences between its records and the VA parent facility billable roster, and invoice the VA parent facility.
- The VA parent facility shall certify the contractor's invoice.

No later than the seventh (7th) business day of each month, the VA parent facility coordinator or the COR (or their designee) will submit to the contractor a list of patient names who properly meet the billing criteria. This list is the VA parent facility "billable roster" for the applicable month to be invoiced. This list will include the names of all patients who are assigned to the contractor's site of care in PCMM and have had at least one qualifying encounter with contractor's PCP within the previous 12 months. (Example: A list sent to the contractor on October 7, 2023, will cover the time frame of October 1, 2022, through September 30, 2023.) This billable roster represents all patients assigned to contractor's site of care in PCMM and have had at least one qualifying encounter with the contractor's PCP within the previous 12 months minus any patients who may have been seen in that timeframe but have, in the meantime, been removed from the billable roster. (See the Removal from Billable Roster section in this PWS).

The VA parent facility will also provide the contractor with an alphabetically arranged lists of names of patients who were removed that month from the billable roster. The list shall also include the reason the patient was removed from the billable roster.

Patient names that come to either the VA parent facility or the contractor's attention "after the fact" will not only be removed from the current billable roster, but the contractor shall also credit or reimburse the VA parent facility for any previous months the VA parent facility paid for the care of patients who should have been removed from previous billable rosters.

The contractor shall reconcile the VA parent facility billable roster with its records. Any perceived discrepancies identified by the contractor, regarding the VA parent facility provided billable roster, will be required to be discussed between the contractor and the VA parent facility Coordinator/COR or the CO or their designee. The final arbitrator regarding any discrepancies between the contractor and the VA parent facility regarding the billable roster is CO. CO decisions in this regard are final.

Upon receipt of an electronic invoice from the contractor, based on the billable roster agreed upon and including supporting data, the VA parent facility will certify the invoice for payment. The contractor shall have 30 calendar days from the date of invoice to justify any additions to the billable roster for the applicable month of invoice.

Electronic Invoice Submission: Invoices will be electronically submitted to the Tungsten Automation website at <https://www.tungstenautomation.com/>. Tungsten Automation direct vendor support number is 877-489-6135 for VA contracts. The VA-FSC pays all associated transaction fees for VA orders. During implementation (technical set-up) Tungsten Automation will confirm your Taxpayer ID Number with the VA-FSC. This process can take up to 5 business days to complete to ensure your invoice is automatically routed to your certifying official for approval and payment. To successfully submit an invoice to VA-FSC please review "How to Create an Invoice" within the how to guides. All invoices submitted through Tungsten Automation to the VA-FSC should mirror the current submission of invoice, with the following items required. Clarification of additional requirements should be confirmed with your certifying official (your CO or buyer). Payments will only be made for actual services rendered. Payments shall be made monthly, in arrears. The contractor shall be reimbursed at the capitation rate specified in the schedule of services section. The VA-FSC requires specific information in compliance with the Prompt Pay Act and business requirements. The contractor shall be reimbursed upon receipt of a proper invoice. Invoices shall contain the following information:

- Total number of listed patients from the previous month's invoice.
- New patients added to the billable roster since the previous month's invoice.
- Patients removed from the billable roster since the previous month's invoice.
- Number of patients (if any) whose removal from the billable roster generate a credit, the amount of the credit, and the calculation(s) used to arrive at the credit.
- Firm's Taxpayer ID Number (TIN)
- Firm's "Remit Address" information
- The VA purchase order (PO) number
- Firm's contact information: (Personal name, email, and phone)
- VA parent facility point of contact information: (Personal name, email, and phone)
- The period of performance dates (beginning and ending)
- All discount information if applicable (percent and date terms)

For additional information, please contact:



Patients determined to be ineligible for VA medical care will be billed by VA parent facility for the care rendered in accordance with VA regulations. VA parent facility shall reimburse the contractor for one visit for patient or Veteran subsequently deemed ineligible by VA. Reimbursement will be at the Medicare rate in effect

on date of service for the state of Oregon for the CPT codes utilized during the initial visit. In accordance with the PWS, the VA is required to verify Veteran eligibility within twenty-four (24) hours from the time the contractor requests an eligibility determination for each applicant.

The VA parent facility may deny payment for emergency medical services performed locally outside the contractor's facility if the VA physician reviewing the patient's medical record determines that no emergency existed. The contractor can appeal this determination in writing to the CO by submitting supporting documentation. If a dispute still exists after contractor's documentation is reviewed, the contractor may file a claim in accordance with the procedures set forth in FAR 52.212-4(d).

21. PAYMENT IN FULL/NO BILLING VA BENEFICIARIES:

The contractor shall accept payment for services rendered under this contract as payment in full. VA parent facility beneficiaries shall not under any circumstances be charged nor their insurance companies charged for services rendered by the contractor, even if VA parent facility does not pay for those services. This provision shall survive the termination or ending of the contract. To the extent that the patient desires services which are not a VA benefit or covered under the terms of this contract, the contractor shall notify the patient that there will be a charge for such service and that the VA parent facility will not be responsible for payment. The contractor shall not bill, charge, collect a deposit from, seek compensation, remuneration, or reimbursement from, or have any recourse against, any person or entity other than VA parent facility for services provided pursuant to this contract. It shall be considered fraudulent for the contractor to bill other third-party insurance sources (including Medicare) for services rendered to patients under this contract.

22. PROCEDURE REGARDING THIRD PARTY RESOURCES:

The VA parent facility shall be entitled to and shall exercise full subrogation rights and shall be responsible for making every reasonable effort to determine the legal liability of third parties to pay for services rendered to patients under this contract and recover any such liability from the third party. If the contractor has determined that third party liability exists for part or all the services provided directly by the contractor to an enrolled patient, the contractor shall make reasonable efforts to notify VA for recovery from third party liable sources the value of services rendered. All such cases will be referred to the MCCR section at VA parent facility.

VA has the authority to bill insurance carriers for treatment provided to patients for non-service-related conditions. Patients presenting for care will be asked by the contractor's staff to provide their insurance and/or Medicare card(s). Per the national mandate, the contractor's staff will then scan the insurance cards (front and back) into the DSS program for processing. In the event the card is not able to be scanned, a photocopy of the front and back should be made and faxed to the MCCR Section at the VA. The copy of the card shall be faxed no later than the end of the second business day the patient is seen. The system automatically requires update of this data every six months (180 days) unless the patient identifies a change in his insurance status. Contractor is not liable for data older than 6 months if patient has not visited. The contractor shall review the health insurance information at the time of each clinic visit. The contractor shall provide the VA parent facility with patient treatment information daily to facilitate third party billing. The contractor shall also provide copies of medical records, at no charge, when requested by the VA to support billing.

23. CONTRACTOR SECURITY REQUIREMENTS: : (HANDBOOK 6500.6):

The contractor, their personnel, and their subcontractors shall be subject to the Federal laws, regulations, standard, and VA Directives and Handbooks regarding information and information system security as delineated in this contract.

GENERAL: Contractors, contractor personnel, subcontractor, and subcontractor personnel shall be subject to the same Federal laws, regulations, standards, and VA Directives and Handbooks as VA and VA personnel regarding information and information system security.

1. ACCESS TO VA INFORMATION AND VA INFORMATION SYSTEMS

- A contractor/subcontract shall request logical (technical) or physical access to VA information and VA information systems for their employees, subcontractors, and affiliates only to the extent necessary to perform the services specified in the contract, agreement, or task order.
- All contractors, subcontractors, and third-party servicers and associates working with VA information are subject to the same investigative requirements as those of VA appointees or

employees who have access to the same types of information. The level and process of background security investigations for contractors must be in accordance with VA Directive and Handbook 0710, Personnel Suitability and Security Program. The Office for Operations, Security, and Preparedness is responsible for these policies and procedures.

- Contract personnel who require access to national security programs must have a valid security clearance. National Industrial Security Program (NISP) was established by Executive Order 12829 to ensure that cleared U.S. defense industry contract personnel safeguard the classified information in their possession while performing work on contracts, programs, bids, or research and development efforts. The Department of Veterans Affairs does not have a Memorandum of Agreement with Defense Security Services (DSS). Verification of a Security Clearance must be processed through the Special Security Officer located in the Planning and National Security Service within the Office of Operations, Security, and Preparedness.
- Custom software development and outsourced operations must be located in the U.S. to the maximum extent practical. If such services are proposed to be performed abroad and are not disallowed by other VA policy or mandates, the contractor/subcontractor must state where all non-U.S. services are provided and detail a security plan, deemed to be acceptable by VA, specifically to address mitigation of the resulting problems of communication, control, data protection, and so forth. Location within the U.S. may be an evaluation factor.
- The contractor or subcontractor must notify the Contracting Officer immediately when an employee working on a VA system or with access to VA information is reassigned or leaves the contractor or subcontractor's employ. The Contracting Officer must also be notified immediately by the contractor or subcontractor prior to an unfriendly termination.

2. VA INFORMATION CUSTODIAL LANGUAGE

- Information made available to the contractor or subcontractor by VA for the performance or administration of this contract or information developed by the contractor/subcontractor in performance or administration of the contract shall be used only for those purposes and shall not be used in any other way without the prior written agreement of the VA. This clause expressly limits the contractor/subcontractor's rights to use data as described in Rights in Data - General, FAR 52.227-14(d) (1).
- VA information should not be co-mingled, if possible, with any other data on the contractors/subcontractor's information systems or media storage systems to ensure VA requirements related to data protection and media sanitization can be met. If co-mingling must be allowed to meet the requirements of the business need, the contractor must ensure that VA's information is returned to the VA or destroyed in accordance with VA's sanitization requirements. VA reserves the right to conduct on-site inspections of contractor and subcontractor IT resources to ensure data security controls, separation of data and job duties, and destruction/media sanitization procedures are in compliance with VA directive requirements.
- Prior to termination or completion of this contract, contractor/subcontractor must not destroy information received from VA, or gathered/created by the contractor in the course of performing this contract without prior written approval by the VA. Any data destruction done on behalf of VA by a contractor/subcontractor must be done in accordance with National Archives and Records Administration (NARA) requirements as outlined in VA Directive 6300, Records and Information Management and its Handbook 6300.1 Records Management Procedures, applicable VA Records Control Schedules, and VA Handbook 6500.1, Electronic Media Sanitization. Self-certification by the contractor that the data destruction requirements above have been met must be sent to the VA Contracting Officer within 30 days of termination of the contract.
- The contractor/subcontractor must receive, gather store, back up, maintain, use, disclose and dispose of VA information only in compliance with the terms of the contract and applicable Federal and VA information confidentiality and security laws, regulations and policies. If Federal or VA information confidentiality and security laws, regulations and policies become applicable to the VA information or information systems after execution of the contract, or if NIST issues or updates applicable FIPS or Special Publications (SP) after execution of this contract, the parties agree to negotiate in good faith to implement the information confidentiality and security laws, regulation and policies in this contract.
- The contractor/subcontractor shall not make copies of VA information except as authorized and necessary to perform the terms of the agreement or to preserve electronic information stored on

contractor/subcontractor electronic storage media for restoration in case any electronic equipment or data used by the contractor/subcontractor needs to be restored to an operating state. If copies are made for restoration purposes, after the restoration is complete, the copies must be appropriately destroyed.

- If VA determines that the contractor has violated any of the information confidentiality, privacy, and security provisions of the contract, it shall be sufficient grounds for VA to withhold payment to the contractor or third party or terminate the contract for default or terminate for cause under Federal Acquisition Regulation (FAR) part 12.
- If a VHA contract is terminated for cause, the associated BAA must also be terminated and appropriate actions taken in accordance with VHA Handbook 1600.01, Business Associate Agreements. Absent an agreement to use or disclose protected health information, there is no business associate relationship.
- The contractor/subcontractor must store, transport, or transmit VA sensitive information in an encrypted form, using VA-approved encryption tools that are, at a minimum, FIPS 140-2 validated.
- The contractor/subcontractor's firewall and Web services security controls, if applicable, shall meet or exceed VA's minimum requirements. VA Configuration Guidelines are available upon request.
- Except for uses and disclosures of VA information authorized by this contract for performance of the contract, the contractor/subcontractor may use and disclose VA information only in two other situations: (i) in response to a qualifying order of a court of competent jurisdiction, or (ii) with VA's prior written approval. The contractor/subcontractor must refer all requests for, demands for production of, or inquiries about, VA information and information systems to the VA contracting officer for response.
- Notwithstanding the provision above, the contractor/subcontractor shall not release VA records protected by Title 38 U.S.C. 5705, confidentiality of medical quality assurance records and/or Title 38 U.S.C. 7332, confidentiality of certain health records pertaining to drug addiction, sickle cell anemia, alcoholism or alcohol abuse, or infection with human immunodeficiency virus. If the contractor/subcontractor is in receipt of a court order or other requests for the above-mentioned information, that contractor/subcontractor shall immediately refer such court orders or other requests to the VA contracting officer for response.
- For service that involves the storage, generating, transmitting, or exchanging of VA sensitive information but does not required C&A or an MOU-ISA for system interconnection, the contractor/subcontractor must complete a Contractor Security Control Assessment (CSCA) on a yearly basis and provide it to the COR.

3. INFORMATION SYSTEM DESIGN AND DEVELOPMENT

a.) Information systems that are designed or developed for or on behalf of VA at non-VA facilities shall comply with all VA directives developed in accordance with FISMA, HIPAA, NIST, and related VA security and privacy control requirements for Federal information systems. This includes standards for the protection of electronic PHI, outlined in 45 C.F.R. Part 164, Subpart C, information and system security categorization level designations in accordance with FIPS 199 and FIPS 200 with implementation of all baseline security controls commensurate with the FIPS 199 system security categorization (reference Appendix D of VA Handbook 6500, VA Information Security Program). During the development cycle a Privacy Impact Assessment (PIA) must be completed, provided to the COR, and approved by the VA Privacy Service in accordance with Directive 6507, VA Privacy Impact Assessment.

b.) The contractor/subcontractor shall certify to the COR that applications are fully functional and operate correctly as intended on systems using the VA Federal Desktop Core Configuration (FDCC), and the common security configuration guidelines provided by NIST or the VA. This includes Internet Explorer 7 configured to operate on Windows XP and Vista (in Protected Mode on Vista) and future versions, as required.

c.) The standard installation, operation, maintenance, updating, and patching of software shall not alter the configuration settings from the VA approved and FDCC configuration. Information technology staff must also use the Windows Installer Service for installation to the default "program files" directory and silently install and uninstall.

d.) Applications designed for normal end users shall run in the standard user context without elevated system administration privileges.

e.) The security controls must be designed, developed, approved by VA, and implemented in accordance with the provisions of VA security system development life cycle as outlined in NIST Special Publication 800-37, Guide for Applying the Risk Management Framework to Federal Information Systems, VA Handbook 6500, Information Security Program and VA Handbook 6500.5, Incorporating Security and Privacy in System Development Lifecycle.

f.) The contractor/subcontractor is required to design, develop, or operate a System of Records Notice (SOR) on individuals to accomplish an agency function subject to the Privacy Act of 1974 (as amended), Public Law 93-579, December 31, 1974 (5 U.S.C. 552a) and applicable agency regulations. Violation of the Privacy Act may involve the imposition of criminal and civil penalties.

4. The contractor/subcontractor agrees to:

- a.) Comply with the Privacy Act of 1974 (the Act) and the agency rules and regulations issued under the Act in the design, development, or operation of any system of records on individuals to accomplish an agency function when the contract specifically identifies:
- b.) The Systems of Records (SOR); and
- c.) The design, development, or operation work that the contractor/subcontractor is to perform;
- d.) Include the Privacy Act notification contained in this contract in every solicitation and resulting subcontract and in every subcontract awarded without a solicitation, when the work statement in the proposed subcontract requires the redesign, development, or operation of a SOR on individuals that is subject to the Privacy Act; and
- e.) Include this Privacy Act clause, including this subparagraph (3), in all subcontracts awarded under this contract which requires the design, development, or operation of such a SOR.
- f.) In the event of violations of the Act, a civil action may be brought against the agency involved when the violation concerns the design, development, or operation of a SOR on individuals to accomplish an agency function, and criminal penalties may be imposed upon the officers or employees of the agency when the violation concerns the operation of a SOR on individuals to accomplish an agency function. For purposes of the Act, when the contract is for the operation of a SOR on individuals to accomplish an agency function, the contractor/subcontractor is considered to be an employee of the agency.

7. “Operation of a System of Records” means performance of any of the activities associated with maintaining the SOR, including the collection, use, maintenance, and dissemination of records.

8. “Record” means any item, collection, or grouping of information about an individual that is maintained by an agency, including, but not limited to, education, financial transactions, medical history, and criminal or employment history and contains the person’s name, or identifying number, symbol, or any other identifying particular assigned to the individual, such as a fingerprint or voiceprint, or a photograph.

9. “System of Records” means a group of any records under the control of any agency from which information is retrieved by the name of the individual or by some identifying number, symbol, or other identifying particular assigned to the individual.

10. The vendor shall ensure the security of all procured or developed systems and technologies, including their subcomponents (hereinafter referred to as “Systems”), throughout the life of this contract and any extension, warranty, or maintenance periods. This includes, but is not limited to workarounds, patches, hotfixes, upgrades, and any physical components (hereinafter referred to as Security Fixes) which may be necessary to fix all security vulnerabilities published or known to the vendor anywhere in the Systems, including Operating Systems and firmware. The vendor shall ensure that Security Fixes shall not negatively impact the Systems.

11. The vendor shall notify VA within 24 hours of the discovery or disclosure of successful exploits of the vulnerability which can compromise the security of the Systems (including the confidentiality or integrity of its data and operations, or the availability of the system). Such issues shall be remediated as quickly as is practical, but in no event longer than 10 calendar days.

12. When the Security Fixes involve installing third party patches (such as Microsoft OS patches or Adobe Acrobat), the vendor will provide written notice to the VA that the patch has been validated as not affecting the Systems within 10 working days. When the vendor is responsible for operations or maintenance of the Systems, they shall apply the Security Fixes within 10 calendar days.

13. All other vulnerabilities shall be remediated as specified in this paragraph in a timely manner based on risk, but within 60 days of discovery or disclosure. Exceptions to this paragraph (e.g. for the convenience of VA) shall only be granted with approval of the contracting officer and the VA Assistant Secretary for Office of Information and Technology.

14. INFORMATION SYSTEM HOSTING, OPERATION, MAINTENANCE, OR USE

- a.) For information systems that are hosted, operated, maintained, or used on behalf of VA at non-VA facilities, contractors/subcontractors are fully responsible and accountable for ensuring compliance with all HIPAA, Privacy Act, FISMA, NIST, FIPS, and VA security and privacy directives and handbooks. This includes conducting compliant risk assessments, routine vulnerability scanning, system patching and change management procedures, and the completion of an acceptable contingency plan for each system. The contractor's security control procedures must be equivalent, to those procedures used to secure VA systems. A Privacy Impact Assessment (PIA) must also be provided to the COR and approved by VA Privacy Service prior to operational approval. All external Internet connections to VA's network involving VA information must be reviewed and approved by VA prior to implementation.
- b.) Adequate security controls for collecting, processing, transmitting, and storing of Personally Identifiable Information (PII), as determined by the VA Privacy Service, must be in place, tested, and approved by VA prior to hosting, operation, maintenance, or use of the information system, or systems by or on behalf of VA. These security controls are to be assessed and stated within the PIA and if these controls are determined not to be in place, or inadequate, a Plan of Action and Milestones (POA&M) must be submitted and approved prior to the collection of PII.
- c.) Outsourcing (contractor facility, contractor equipment or contractor staff) of systems or network operations, telecommunications services, or other managed services requires certification and accreditation (authorization) (C&A) of the contractor's systems in accordance with VA Handbook 6500.3, Certification and Accreditation and/or the VA OCS Certification Program Office. Government- owned (government facility or government equipment) contractor-operated systems, third party or business partner networks require memorandums of understanding and interconnection agreements (MOU-ISA) which detail what data types are shared, who has access, and the appropriate level of security controls for all systems connected to VA networks.
- d.) The contractor/subcontractor's system must adhere to all FISMA, FIPS, and NIST standards related to the annual FISMA security controls assessment and review and update the PIA. Any deficiencies noted during this assessment must be provided to the VA contracting officer and the ISO for entry into VA's POA&M management process. The contractor/subcontractor must use VA's POA&M process to document planned remedial actions to address any deficiencies in information security policies, procedures, and practices, and the completion of those activities. Security deficiencies must be corrected within the timeframes approved by the government. Contractor/subcontractor procedures are subject to periodic, unannounced assessments by VA officials, including the VA Office of Inspector General. The physical security aspects associated with contractor/subcontractor activities must also be subject to such assessments. If major changes to the system occur that may affect the privacy or security of the data or the system, the C&A of the system may need to be reviewed, retested and re-authorized per VA Handbook 6500.3. This may require reviewing and updating all the documentation (PIA, System Security Plan, Contingency Plan). The Certification Program Office can provide guidance on whether a new C&A would be necessary.
- e.) The contractor/subcontractor must conduct an annual self-assessment on all systems and outsourced services as required. Both hard copy and electronic copies of the assessment must be provided to the

COTR. The government reserves the right to conduct such an assessment using government personnel or another contractor/subcontractor. The contractor/subcontractor must take appropriate and timely action (this can be specified in the contract) to correct or mitigate any weaknesses discovered during such testing, generally at no additional cost.

- f.) VA prohibits the installation and use of personally-owned or contractor/subcontractor-owned equipment or software on VA's network. If non-VA owned equipment must be used to fulfill the requirements of a contract, it must be stated in the service agreement, SOW or contract. All the security controls required for government furnished equipment (GFE) must be utilized in approved other equipment (OE) and must be funded by the owner of the equipment. All remote systems must be equipped with, and use, a VA-approved antivirus (AV) software and a personal (host-based or enclave based) firewall that is configured with a VA-approved configuration. Software must be kept current, including all critical updates and patches. Owners of approved OE are responsible for providing and maintaining the anti-viral software and the firewall on the non-VA owned OE.
- g.) All electronic storage media used on non-VA leased or non-VA owned IT equipment that is used to store, process, or access VA information must be handled in adherence with VA Handbook 6500.1, Electronic Media Sanitization upon: (i) completion or termination of the contract or (ii) disposal or return of the IT equipment by the contractor/subcontractor or any person acting on behalf of the contractor/subcontractor, whichever is earlier. Media (hard drives, optical disks, CDs, back-up tapes, etc.) used by the contractors/subcontractors that contain VA information must be returned to the VA for sanitization or destruction or the contractor/subcontractor must self-certify that the media has been disposed of per 6500.1 requirements. This must be completed within 30 days of termination of the contract.
- h.) Bio-Medical devices and other equipment or systems containing media (hard drives, optical disks, etc.) with VA sensitive information must not be returned to the vendor at the end of lease, for trade-in, or other purposes. The options are:
 - a.) Vendor must accept the system without the drive;
 - b.) VA's initial medical device purchase includes a spare drive which must be installed in place of the original drive at time of turn-in; or
 - c.) VA must reimburse the company for media at a reasonable open market replacement cost at time of purchase.
 - d.) Due to the highly specialized and sometimes proprietary hardware and software associated with medical equipment/systems, if it is not possible for the VA to retain the hard drive, then:
 - e.) The equipment vendor must have an existing BAA if the device being traded in has sensitive information stored on it and hard drive(s) from the system are being returned physically intact; and
 - f.) Any fixed hard drive on the device must be non-destructively sanitized to the greatest extent possible without negatively impacting system operation. Selective clearing down to patient data folder level is recommended using VA approved and validated overwriting technologies/methods/tools. Applicable media sanitization specifications need to be pre-approved and described in the purchase order or contract.
 - g.) A statement needs to be signed by the Director (System Owner) that states that the drive could not be removed and that (a) and (b) controls above are in place and completed. The ISO needs to maintain the documentation.

15. SECURITY INCIDENT INVESTIGATION

- a.) The term "security incident" means an event that has, or could have, resulted in unauthorized access to, loss or damage to VA assets, or sensitive information, or an action that breaches VA security procedures. The contractor/subcontractor shall immediately notify the COR and simultaneously, the designated ISO and Privacy Officer for the contract of any known or suspected security/privacy incidents, or any unauthorized disclosure of sensitive information, including that contained in system(s) to which the contractor/subcontractor has access.
- b.) To the extent known by the contractor/subcontractor, the contractor/subcontractor's notice to VA shall identify the information involved, the circumstances surrounding the incident (including to whom, how, when, and where the VA information or assets were placed at risk or compromised), and any other information that the contractor/subcontractor considers relevant.

- c.) With respect to unsecured protected health information, the business associate is deemed to have discovered a data breach when the business associate knew or should have known of a breach of such information. Upon discovery, the business associate must notify the covered entity of the breach. Notifications need to be made in accordance with the executed business associate agreement.
- d.) In instances of theft or break-in or other criminal activity, the contractor/subcontractor must concurrently report the incident to the appropriate law enforcement entity (or entities) of jurisdiction, including the VA OIG and Security and Law Enforcement. The contractor, its employees, and its subcontractors and their employees shall cooperate with VA and any law enforcement authority responsible for the investigation and prosecution of any possible criminal law violation(s) associated with any incident. The contractor/subcontractor shall cooperate with VA in any civil litigation to recover VA information, obtain monetary or other compensation from a third party for damages arising from any incident, or obtain injunctive relief against any third party arising from, or related to, the incident.

16. LIQUIDATED DAMAGES FOR DATA BREACH

- a.) Consistent with the requirements of 38 U.S.C. §5725, a contract may require access to sensitive personal information. If so, the contractor is liable to VA for liquidated damages in the event of a data breach or privacy incident involving any SPI the contractor/subcontractor processes or maintains under this contract. However, it is the policy of VA to forgo collection of liquidated damages in the event the contractor provides payment of actual damages in an amount determined to be adequate by the agency.
- b.) The contractor/subcontractor shall provide notice to VA of a “security incident” as set forth in the Security Incident Investigation section above. Upon such notification, VA must secure from a non- Department entity or the VA Office of Inspector General an independent risk analysis of the data breach to determine the level of risk associated with the data breach for the potential misuse of any sensitive personal information involved in the data breach. The term ‘data breach’ means the loss, theft, or other unauthorized access, or any access other than that incidental to the scope of employment, to data containing sensitive personal information, in electronic or printed form, that results in the potential compromise of the confidentiality or integrity of the data. Contractor shall fully cooperate with the entity performing the risk analysis. Failure to cooperate may be deemed a material breach and grounds for contract termination.
- c.) Each risk analysis shall address all relevant information concerning the data breach, including the following:
 - d.) Nature of the even (loss, theft, unauthorized access);
 - e.) Description of the event, including:
 - f.) Date of occurrence;
 - g.) Data elements involved, including any PII, such as full name, social security number, date of birth, home address, account number, disability code;
 - h.) Number of individuals affected or potentially affected;
 - i.) Names of individuals or groups affected or potentially affected;
 - j.) Ease of logical data access to the lost, stolen or improperly accessed data considering the degree of protection for the data, e.g., unencrypted, plain text;
 - k.) Amount of time the data has been out of VA control;
 - l.) The likelihood that the sensitive personal information will or has been compromised (made accessible to and usable by unauthorized persons);
 - m.) Known misuses of data containing sensitive personal information, if any;
 - n.) Assessment of the potential harm to the affected individuals;

- o.) Data breach analysis as outlined in 6500.2 Handbook, Management of Security and Privacy Incidents, as appropriate; and
- p.) Whether credit protection services may assist record subjects in avoiding or mitigating the results of identity theft based on the sensitive personal information that may have been compromised.
- q.) Based on the determinations of the independent risk analysis, the contractor shall be responsible for paying to the VA liquidated damages in the amount of \$37.50 per affected individual to cover the cost of providing credit protection services to affected individuals consisting of the following:

Notification;

- 1. One year of credit monitoring services consisting of automatic daily monitoring of at least 3 relevant credit bureau reports;
- 2. Data breach analysis;
- 3. Fraud resolution services, including writing dispute letters, initiating fraud alerts and credit freezes, to assist affected individuals to bring matters to resolution;
- 4. One year of identity theft insurance with \$20,000.00 coverage at \$0 deductible; and
- 5. Necessary legal expenses the subjects may incur to repair falsified or damaged credit records, histories, or financial affairs.

17. SECURITY CONTROLS COMPLIANCE TESTING

On a periodic basis, VA, including the Office of Inspector General, reserves the right to evaluate any or all the security controls and privacy practices implemented by the contractor under the clauses contained within the contract. With 10 working-days' notice, at the request of the government, the contractor must fully cooperate and assist in a government-sponsored security controls assessment at each location wherein VA information is processed or stored, or information systems are developed, operated, maintained, or used on behalf of VA, including those initiated by the Office of Inspector General. The government may conduct a security control assessment on shorter notice (to include unannounced assessments) as determined by VA in the event of a security incident or at any other time.

18. TRAINING

- a.) All contractor employees and subcontractor employees requiring access to VA information and VA information systems shall complete the following before being granted access to VA information and its systems:
 - Sign and acknowledge (either manually or electronically) understanding of and responsibilities for compliance with the Contractor Rules of Behavior, Appendix E relating to access to VA information systems;
 - Successfully complete the VA Cyber Security Awareness and Rules of Behavior training and annually complete required security training;
 - Successfully complete the appropriate VA privacy training and annually complete required privacy training; and
 - Successfully complete any additional cyber security or privacy training, as required for VA personnel with equivalent information system access [to be defined by the VA program official and provided to the contracting officer for inclusion in the solicitation document – e.g., any role-based information security training required in accordance with NIST Special Publication 800-16, Information Technology Security Training Requirements.]
- b.) The contractor shall provide to the contracting officer and/or the COTR a copy of the training certificates and certification of signing the Contractor Rules of Behavior for each applicable employee within 1 week of the initiation of the contract and annually thereafter, as required.
- c.) Failure to complete the mandatory annual training and sign the Rules of Behavior annually, within the timeframe required, is grounds for suspension or termination of all physical or electronic access privileges and removal from work on the contract until the training and documents are complete.

(End of Performance Work Statement)

SOURCES SOUGHT QUESTIONNAIRE

Company Name _____

POC _____

Phone Number _____

Email Address _____

SAM UEI _____

1. Is your firm considered a Small Business? The North American Industry Classification System (NAICS) code is tentatively 621498, All Other Outpatient Care Centers; the small business size standard for this NAICS code is \$25.5 million.

A size standard, which is usually stated in number of employees or average annual receipts, represents the largest size that a business (including its subsidiaries and affiliates) may be to remain classified as a small business for Small Business Administration and federal contracting programs. The definition of "small" varies by industry. Is your business considered any of the following?

☐ yes ☐ no Service Disabled Veteran Owned Small Business (SDVOSB)**

☐ yes ☐ no Veteran Owned Small Business (VOSB)**

☐ yes ☐ no Historic Underutilized Business Zone (HUBZONE)

☐ yes ☐ no Small Disadvantaged Business (SDB)

☐ yes ☐ no Woman Owned Small Business (WOSB)

☐ yes ☐ no Small Business (SB)

***If you responded 'yes' to SDVOSB or VOSB, submit a copy of your current Vendor Information Pages (VIP) Certificate.*

2. Does your firm have an active registration in the System for Award Management (SAM)?

☐ yes ☐ no

3. Have you provided CBOC services under NAICS 621498 to a Federal Agency in the past? If so, please briefly list experience.

4. Is your firm capable and interested in performing these services? If so, what is the estimated number of days your firm would require to stand up operations?

5. In your opinion, what are the risks associated with this effort? Comment on any current or potential technical, cost, schedule, or performance risks that you would like to make the Government aware of that concern CBOC Services.

6. Provide any additional feedback that you feel is relevant (i.e., problems or any other issues experienced with similar contracts).

Note: Do not include proprietary, classified, confidential, or sensitive information in responses.